



CAREMETRIC BREATHE

INTELLIGENT DME & PAP CARE PLATFORM

User Manual

The complete guide to running your PAP department

Organised by role: Administrator · Biller · CSR · Respiratory Therapist

STOREFRONT · RESUPPLY · BILLING · CLINICAL · VOICE AI

September 2026 · Platform edition

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Introduction

Welcome to **CareMetric Breathe** — the single platform that runs a CPAP/DME resupply department end to end: the patient storefront, the customer database, billing and the revenue cycle, clinical therapy monitoring, and all the documentation in between. This manual is your guide to using it.

Platform vs. tenant. CareMetric Breathe is the software platform — every practice that runs on it is a separate **tenant** with its own brand, web address, phone numbers, patients, and billing. A new tenant starts with CareMetric branding and makes it their own from System Configuration; **Penn Home Medical Supply** is simply one example of a tenant operating on the platform. The screenshots in this manual come from the CareMetric demo environment, so they show the platform's own branding — your live console shows whatever brand you configure.

Getting your workspace — self-serve sign-up

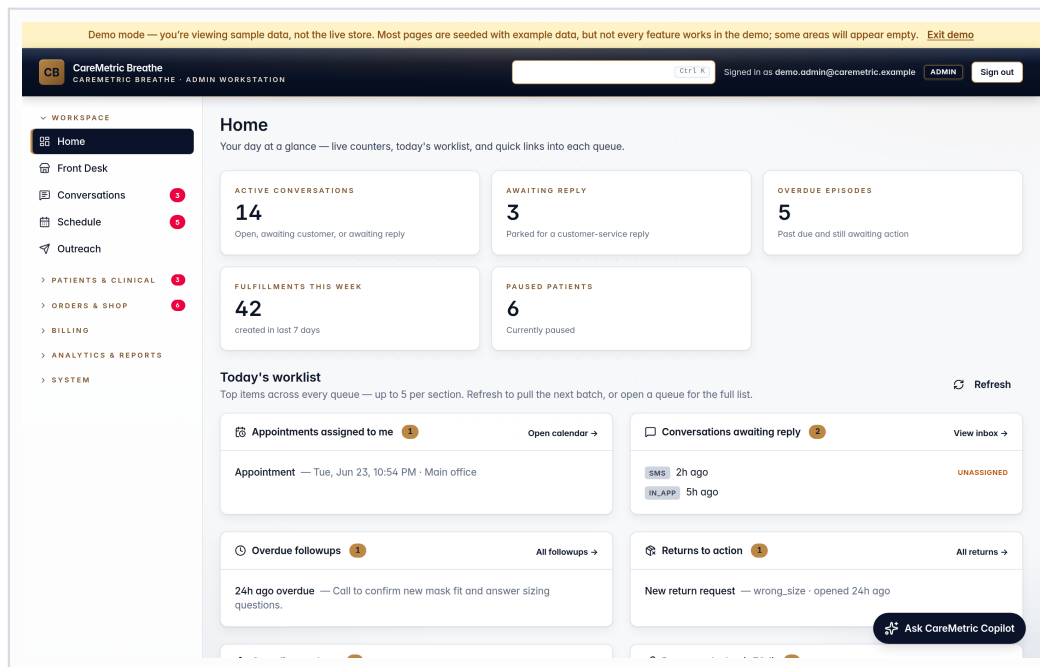
A new practice creates its own workspace from the public CareMetric Breathe site: open **/breathe** and choose **Create your account (/breathe/signup)**. Enter your company name, a work email, a password (at least 12 characters), and pick a plan — Virtual Mask Fitter, Launch, Growth, or Scale; Enterprise is custom-quoted, so it routes to a sales conversation rather than self-signup. Submitting provisions your organization, copies in the feature-flag bundle for your plan, and creates your first **Owner** login.

For security the account starts unverified: we email a verification link to the address you entered. Click it to activate the account, then sign in to your new workspace at **/admin/sign-in** — you confirm payment as you finish setting up. From there the Owner invites the rest of the team from the **Team** page (no one else needs to self-sign-up). Operators can still pre-provision a workspace from the command line, but self-serve sign-up is the normal path.

Create your account on the public CareMetric Breathe site — company, work email, password, and a plan spin up your workspace and first Owner login.

Signing in

Staff sign in at **/admin/sign-in** with the email your administrator invited and a password you set from the invite link. Forgot it? Use **/admin/forgot-password**; a link to verify a new email address arrives the same way. To protect against account enumeration, these pages always confirm “check your email” whether or not the address was on file. Every admin should enroll multi-factor authentication under **System** → **Account Security**. After signing in you land on the **Home** dashboard.



The Home dashboard: today's worklist, live counters, and the left-hand navigation grouped by area.

The four roles

Every staff account has a role that decides what they can see and do. This manual is organised around the four operating roles of a DME practice:

Administrator	Owner/admin of the practice — setup, team, controls, integrations, and full oversight of every area. The Owner is the top tier (team management + system secrets); Admin is broad management below that.
Billor	Revenue-cycle staff — the entire Billing area (eligibility, claims, A/R, ERA, clearinghouse) plus patient billing context.
Customer Service Rep	Front-line patient service — the inbox, scheduling, patients, fit requests, orders, and outreach.
Respiratory Therapist	Clinical staff — therapy monitoring, clinical documentation, interventions, and provider-ready reports.

Access is enforced everywhere: if a page is above your role, its link simply doesn't appear in your navigation. An Administrator assigns roles on the **Team** page — see the appendix for a role/permission matrix.

Getting help inside the app

Two AI assistants are built in. **CareMetric Copilot** (the admin assistant; “PennPilot” for the Penn tenant) floats on every admin page — ask it how a feature works or where to find a page and it answers with clickable links. **CareMetric Assistant** (the storefront chatbot; “PennBot” for the Penn tenant) answers patients on the storefront and can hand off to a CSR. Both are tenant-renamable and can be toggled in the Control Center.

What Sets CareMetric Breathe Apart

Most DME operations run on a patchwork: a billing system here, a resupply vendor there, an e-ordering tool, and spreadsheets between them. CareMetric Breathe replaces the patchwork with one AI-native platform built specifically for CPAP/PAP resupply — so the handoffs that used to mean re-keying, portal-hopping, and phone tag simply disappear. These are the capabilities that set it apart.

Capabilities a spreadsheet stack can't match

AI mask fitting, no appointment

Staff send a patient an invite link by text or email; the patient's phone camera measures their face right in the browser and scores every available mask for fit. Images never leave the device — only numeric measurements are sent — and none of the leading DME platforms offer anything like it. The completed fitting attaches itself to the right chart. Fewer fitting appointments, fewer wrong-size exchanges.

A voice agent that takes reorders

Patients call and a natural AI voice confirms identity, takes the resupply order, and writes a structured summary with sentiment and clinical flags — 24/7, with no hold queue. It hands off to a human the moment the caller asks.

Resupply that runs itself

Device-reported usage and payer replacement schedules trigger SMS and email reminders with signed one-tap confirm links; confirmed orders flow straight to fulfillment and billing. The reorder cycle turns without a CSR dialing a phone.

Eligibility and claims that work ahead of you

Real-time 270/271 eligibility runs on a schedule and again right before a claim goes out; claims arrive pre-scrubbed; the AI flags the fix before submission. Billers stop chasing avoidable rejections.

Denial recovery ranked by dollars

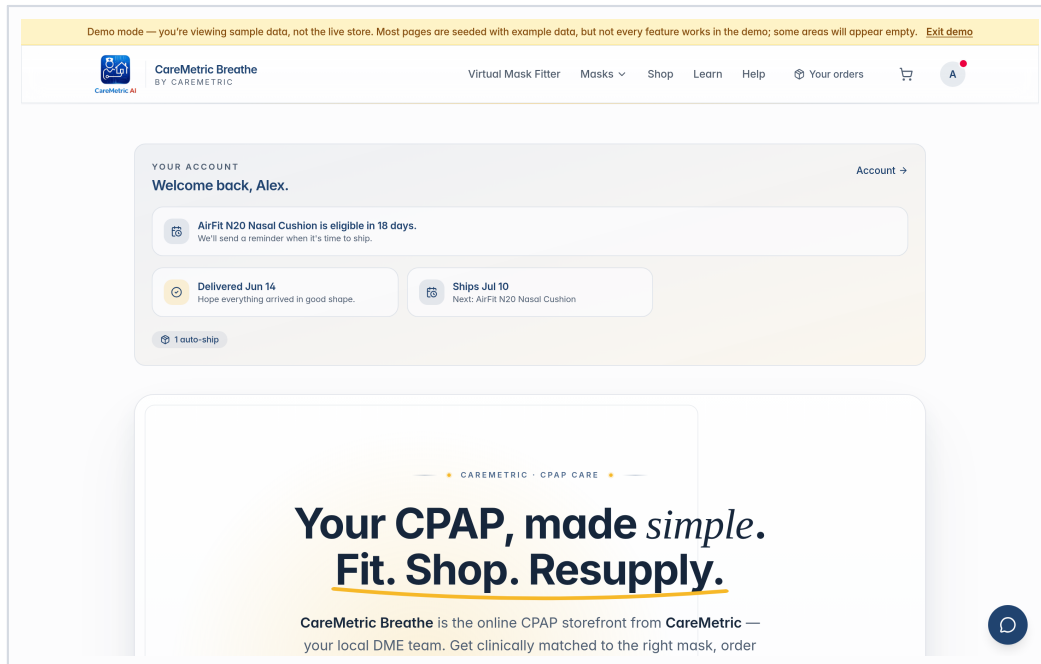
When a denial does land, the worklist ranks every one by recoverable value weighted by win-probability, and the AI denial analyzer reads the CARC/RARC codes, explains the root cause, drafts the fix or appeal, and — when it is safe — offers a one-click resubmit.

Every channel, one inbox

SMS, MMS, email, chat, and AI call summaries land in a single triage queue with assignment, priorities, and consent and quiet-hours enforced automatically. One inbox replaces five tools.

Therapy data from all three clouds, one PAP department

Nightly sync from ResMed AirView, Philips Care Orchestrator, and 3B React Health drives CMS 90-day compliance tracking, clinical worklists, and resupply timing — and it lives in the SAME system as the storefront, the customer database, billing, and documentation. No swivel-chair between systems, no per-module pricing, no integration projects.



The patient storefront — one front door for fitting, shopping, and resupply, with the AI assistant one tap away.

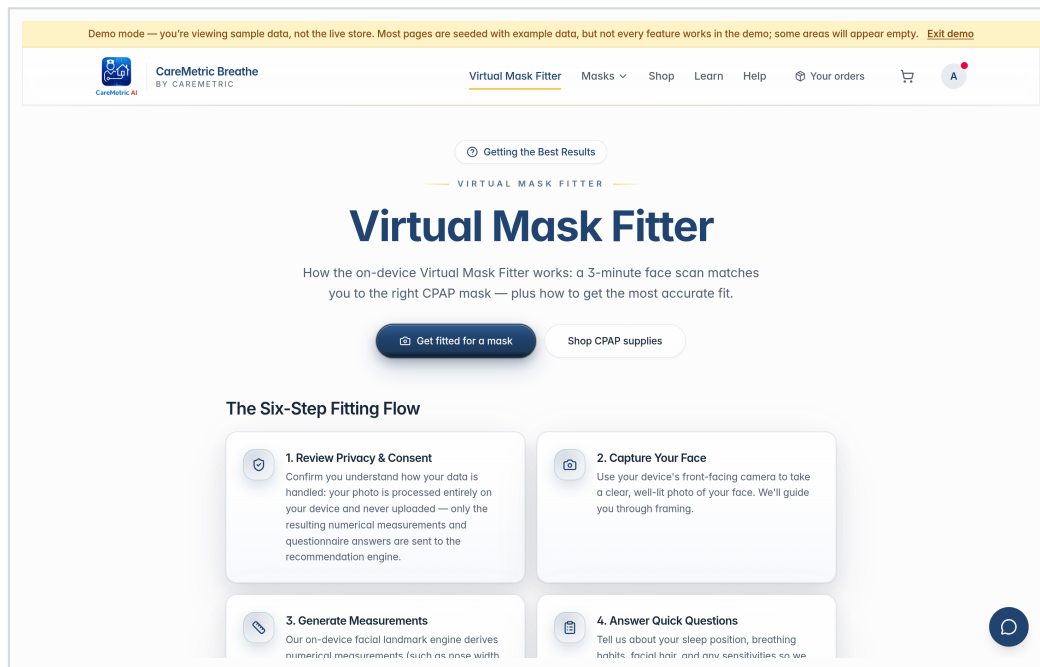
The platform underneath every role

Beneath the four role workspaces sits a shared platform layer. These capabilities serve patients directly or protect the business as a whole, and every role benefits from them.

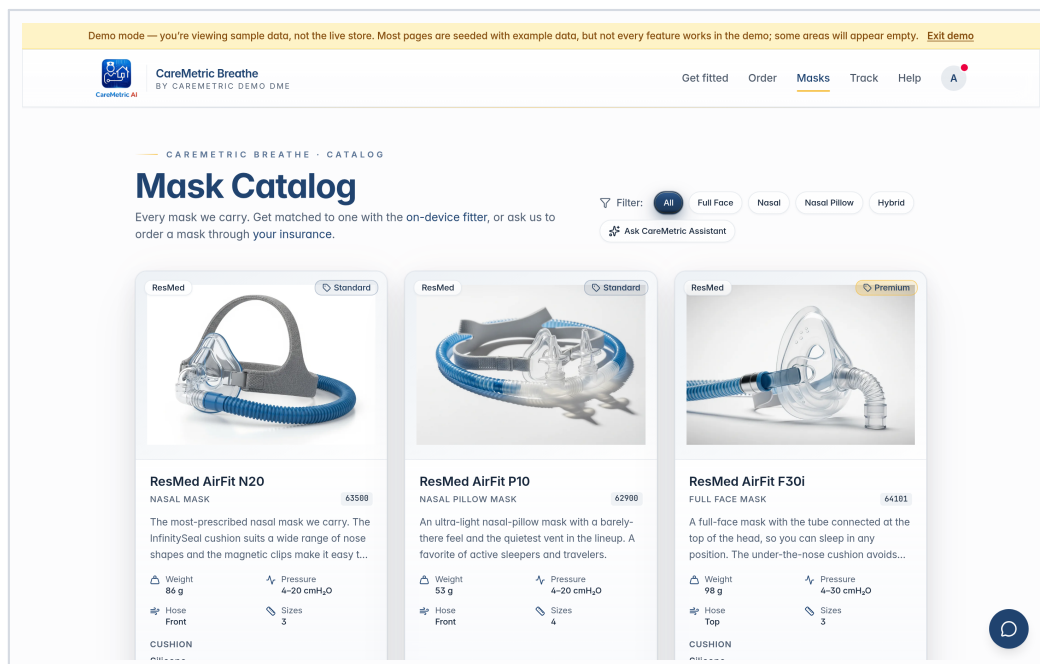
Patient storefront & portal	A patient-facing site and self-service account portal: what they are due for, order tracking, statements and billing history, document access and e-signature, insurance details, caregiver access, and an in-app message thread with your team. It is an insurance storefront, not a retail one — patients are supplied against their plan and are never charged a card, so there is no cart and no checkout.
AI mask fitter	Camera-based facial measurement in the patient's browser scores every available mask for fit. Images never leave the device — only numeric measurements are transmitted. The fitter is invitation-only : staff send a signed invite link by SMS or email from Orders & Leads → Fitter Invites , and the completed fitting attaches back to the patient's chart.
Resupply reminder engine	Automated SMS and email reminders with signed one-tap confirm/decline links, quiet-hours awareness, and unsubscribe handling.
AI voice agent	A natural-voice phone agent that takes reorders, runs reminder and check-in calls, hands off to staff on request, and writes a structured summary of every call.
Chatbot & sleep coach	Patient-facing AI chat for shopping help and therapy coaching, with optional high-confidence email auto-reply; anything uncertain hands off to staff.
CareMetric Copilot (admin assistant)	An in-app AI helper on every admin page that answers “how do I” questions about the console and forwards staff feature ideas to ownership — always confirming before anything is sent.
Security & privacy	Hardened sign-in with optional MFA, role-based access with granular permissions, CSRF and rate-limit protection, and strict PHI discipline: camera images and order payloads are never logged.
Always-on by design	Feature flags flip capabilities instantly, vendor outages degrade gracefully instead of taking the site down, and background jobs handle syncs, reminders, and campaigns around the clock.

The Storefront & End-to-End Fulfillment

The patient storefront supports fitting requests, insurance intake, documents and service. Staff verify the patient, benefits and intended supplies before creating the insurance order. Internal self-pay pricing models do not add a cart, patient card checkout or subscription purchase. The journey continues through reviewed signatures, fulfillment, recorded delivery evidence and a later resupply conversation.



The storefront walks a patient from mask fitting to a request your team works — one front door for fit and resupply.



The mask catalog — every mask the tenant carries. Fitting still ends in a request, not a cart.

1 - Fitting by invitation. Staff send a signed fitting link from **Fitter Invites**. The patient consents and completes the camera-based fitting and comfort questions; camera images stay on the device. The result attaches to the matching chart or awaits staff matching. A recommendation is a request for staff follow-up, not a purchase.

2 - Confirm the requested supply. The CSR reviews the fit request, patient record and requested model/size. Read prior supply orders, available replacement-rule quantities and prescription expiry. Confirm that supplies are wanted; a calendar entry alone is not authorization.

3 - Verify benefits and pricing. Verify coverage and required documentation. Review exact items, quantity, supplier, destination and insurance collection assumptions. If pricing review is required, attach a matching approved insurance review. Billed charges and expected collections remain distinct; no patient charge is invented to repair a margin shortfall.

4 - Sign, release and ship. Send the authorized order for signature. Priced signatures queue exact lines together and preserve accepted amounts. Resolve prescription, stock and delivery holds before release. Obtain the appropriate label or supplier shipment and record tracking. A rate estimate or queued fulfillment is not evidence of shipment or delivery.

5 - Record delivery evidence. Record the actual shipping/delivery dates and required proof in the appropriate order workflow. Do not mark delivered simply because a label was printed or work was assumed shipped. Separate factual delivery evidence from a patient's signature approving an order.

6 - Review the next resupply. Use the patient's **Supplies & next eligibility** and the **Resupply calendar** for the next review. Send individual or selected-batch reminders through configured channels, inspect results and obtain the patient's response. Each future order requires its own current clinical, coverage and pricing checks.

Demo mode — you're viewing sample data, not the live store. Most pages are seeded with example data, but not every feature works in the demo; some areas will appear empty. [Exit demo](#)

CareMetric Breathe
CAREMETRIC BREATHE · ADMIN WORKSTATION

Ctrl, K Signed in as demo.admin@caremetric.example ADMIN Sign out

WORKSPACE PATIENTS & CLINICAL ORDERS & LEADS Catalog Backorders Orders Leads BILLING ANALYTICS & REPORTS SYSTEM

Filter requests Shipping labels

Orders

All orders submitted through your storefront. Click a row to view full patient details.

Signature orders

Build an order for a customer and send them a secure link to review and e-sign paperwork. Billed to their insurance — nothing is charged on the link. Draft-backed resupply orders queue fulfillment on sign; ad-hoc orders stay Signed here for staff to attach a patient and SKU.

+ Create order

REFERENCE	CUSTOMER	BILLED TO INSURANCE	PAPERWORK	STATUS	SENT	ACTIONS
CMB-DEMO-1001	Avery Sample avery.sample@example.com	\$80.00	1 doc	Signed	8/23/2026, 2:08:00 AM	Resend X Cancel
CMB-DEMO-1002	Demo Patient demo.patient@example.com	\$45.00	—	Sent	8/25/2026, 2:08:00 AM	Resend X Cancel
CMB-DEMO-1003	Quinn Mockton quinn.mockton@example.com	\$47.00	—	Viewed	8/24/2026, 2:08:00 AM	Resend X Cancel

Search by name, email, or reference All statuses

REFERENCE	PATIENT	EMAIL	MASK	SHIP TO	STATUS	SUBMITTED
CMB-DEMO-2000	Jordan Sample	jordan@caremetric.example	ResMed AirFit N20 ResMed	Philadelphia, PA	Delivered	Ask CareMetric Copilot
CMB-DEMO-			ResMed AirFit N20	Philadelphia		

The order queue — approved orders ready to fulfill, ship, and track, all in the same system that bills them.

Demo mode — you're viewing sample data, not the live store. Most pages are seeded with example data, but not every feature works in the demo; some areas will appear empty. [Exit demo](#)

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WORKSPACE PATIENTS & CLINICAL ORDERS & LEADS Catalog Backorders Orders Leads BILLING ANALYTICS & REPORTS SYSTEM

Product catalog

The SKUs you dispense and how many are on the shelf. Stock moves are recorded with a reason, so every count can be explained.

+ Add a SKU

2 SKUs at or below the reorder point — show only these

Search by SKU or name All categories Low stock only

SKU	NAME	CATEGORY	ON HAND	
63550	AirFit N20 Nasal Cushion · ResMed	cushion	Not tracked	Record movement
62932	AirFit P10 Nasal Pillows · ResMed	cushion	2 eachs low	Record movement
64162	AirFit F30i Full Face Cushion · ResMed	cushion	Not tracked	Record movement
63508	AirFit N20 Complete Mask · ResMed	mask	Not tracked	Record movement
62980	AirFit P10 Complete Mask · ResMed	mask	Not tracked	Record movement
64181	AirFit F30i Complete Mask · ResMed	mask	Not tracked	Record movement
RT02-H	React Health Rio II Nasal Mask · React Health	mask	Not tracked	Record movement
NUMA-FF	React Health Numa Full Face Mask · React Health	mask	Not tracked	Record movement
63470	AirFit N20 Headgear · ResMed	headgear	Not tracked	Record movement
77796	Climotal InaAir Hantari Trichlor · DasMed	trichlor	Not tracked	Record movement

Ask CareMetric Copilot

The product catalog — SKUs you dispense and on-hand counts. A blank count means untracked, not zero.

Paperless Paperwork — Referrals, eFax & E-Signature

The paperwork around DME — referrals coming in, prescriptions and CMNs going out, signatures chased by fax — is where days disappear and claims stall waiting on a page that never came back. CareMetric Breathe turns that paper chase into a tracked, largely self-driving pipeline: AI reads an incoming referral and stages a ready-to-accept patient; any document goes out by eFax carrying a barcode that files itself the moment it returns signed; and every signature out with a provider is tracked until it comes home. Most of it is opt-in — turn each piece on per tenant in the Control Center.

The referral, read and ready in one pass

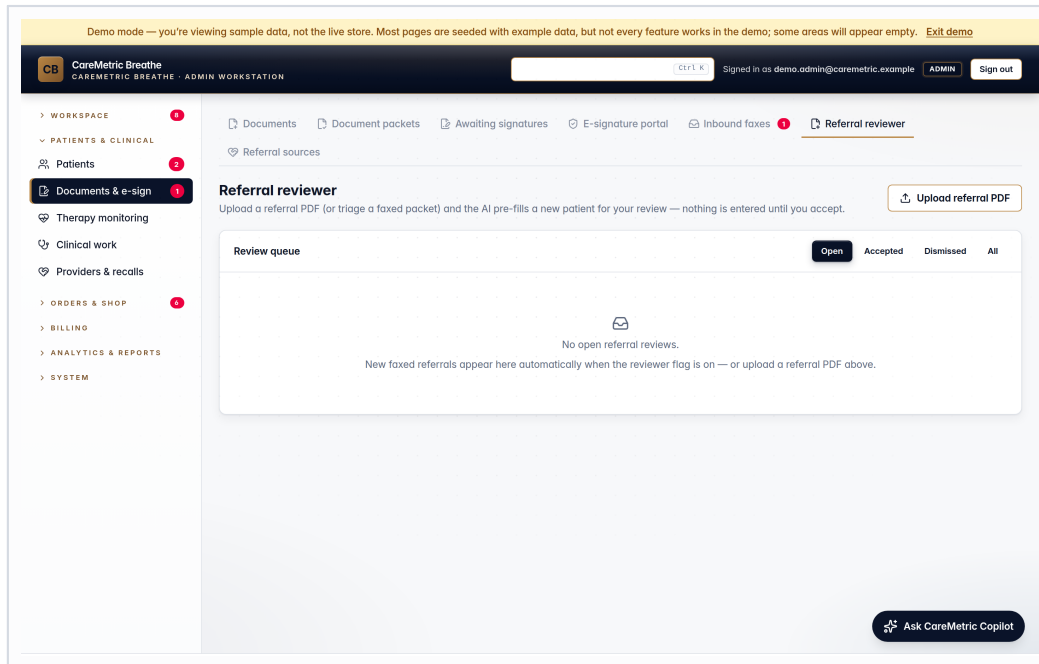
AI reads the whole packet. Upload a referral PDF — or let an inbound fax open one automatically — and the AI extracts the entire packet in a single pass: patient demographics, primary and secondary insurance, the ordered equipment with HCPCS codes, the sleep-study results, the referring physician (name, NPI, clinic, fax), and the diagnosis codes. Everything lands in an editable form beside the original page so you can check any field against the source.

It tells you what's missing. A completeness check flags exactly what a clean claim will need but the packet doesn't have — no signed physician order, a missing prescriber NPI, no face-to-face note, missing diagnosis codes, or a sleep study that doesn't qualify — so gaps surface at intake instead of at denial.

Verify coverage before you commit. One click runs a live 270/271 eligibility check on the extracted payer and member ID and shows active/inactive status, deductible and out-of-pocket remaining, and any prior-auth requirement — a coverage preview before the patient is ever created.

Accept once — patient, insurance, and chart, all at once. When it looks right, accept it: the system creates the new patient, saves the insurance coverages, splits the referral into named documents (sleep study, physician order, insurance card, chart notes...) filed straight to the chart, and generates a Referral Review summary. A duplicate-patient check warns you before you ever create a second record for someone already on file.

Send gaps back to the provider. If the referral is short something, one action drafts a letter to the referring office listing precisely what's needed; review it on the Documents page and fax it. Find it at **Patients & Clinical** → **Documents & e-sign** → **Referral reviewer**.



The Referral Reviewer — AI-extracted intake beside the original packet, with a completeness check and one-click insurance verification.

eFax with a barcode — send once, file itself

Fax anything, from right where you're working. Prescriptions, CMNs, orders, renewal requests, appeal letters — send any of them out by fax straight from the document, with no fax machine and no separate eFax login. The platform renders the PDF and dispatches it over its fax carrier; a failed fax can be retried in a click, and routine outreach (like a batch of renewal requests) can go automatically.

Every signature document carries a barcode. Any document you send out to be signed is stamped with a scannable barcode and a short tracking code, with plain-text filing instructions printed right on it. That barcode becomes the document's identity for the rest of its life.

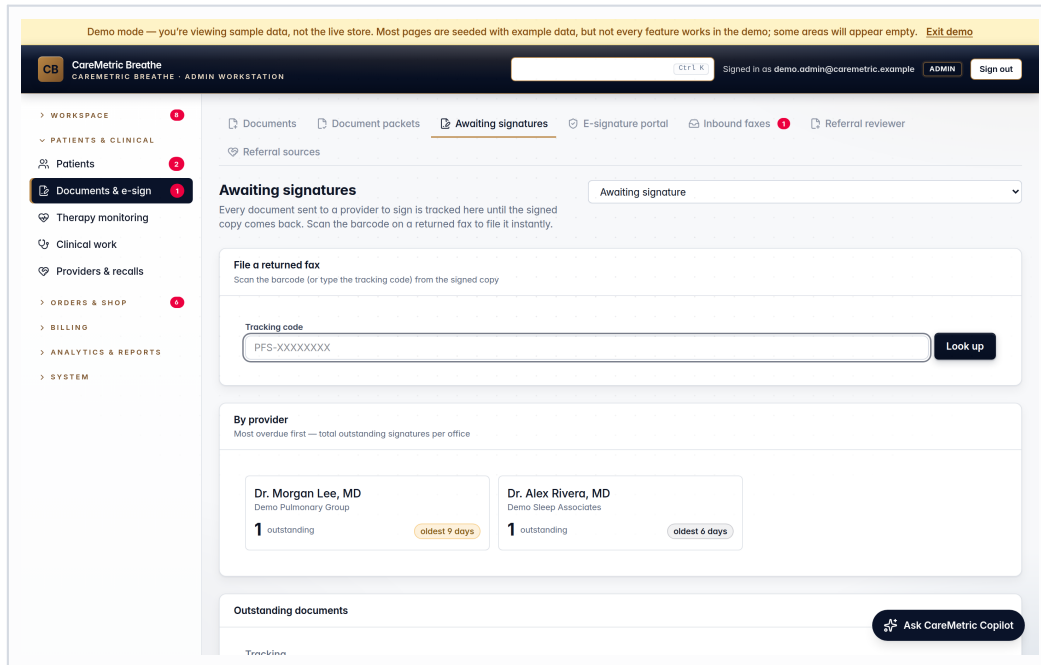
It files itself when it comes back. When the signed copy is faxed back, the system reads the barcode off the page, matches it to the document it came from, copies the signed PDF into that patient's chart, marks the item returned and signed, and releases any bill-hold that was waiting on the paperwork — no manual filing and no "whose page is this?" Anything it can't match still lands in a triage queue for a human. (Automatic filing is an opt-in toggle; staff can also run it on demand.)

Never lose a signature again

One worklist for everything out for signature. The Awaiting Signatures dashboard lists every packet and document you've sent a provider to sign, who it went to, and how long it has been outstanding — so nothing vanishes into a fax-machine black hole. Scan or type a returned document's barcode to file it instantly; mark items returned, hand-delivered, or canceled; and re-send the ones that have gone quiet.

Or skip the fax entirely — providers e-sign online. Invite a referring provider (by NPI, verified against the national registry) into a secure, MFA-protected portal and stage their CMNs, orders, and packets for signature. They sign on their own device instead of printing and faxing back. Every signature is captured in a

tamper-evident, hash-chained audit trail with a printable, E-SIGN-compliant certificate you can hand a payer.



Awaiting Signatures — every document out for a provider's signature, tracked until the barcode comes back.

The Business Case — More Revenue, Less Labor

CareMetric Breathe earns its keep two ways at once: it collects more of the revenue you've already earned, and it does it with less staff time. The same automation that confirms reorders and scrubs claims also removes the manual work behind them — so the practice grows without growing headcount.

Where the new revenue comes from

More reorders confirmed	Automated reminders with one-tap confirm — and the AI voice agent for calls — lift the share of eligible patients who actually reorder on time, instead of supplies (and revenue) slipping.
More clean claims paid	AI scrubbing plus the pre-bill eligibility gate raise your first-pass acceptance rate, so more of what you bill is actually paid and less is written off.
Denials recovered, not lost	The AI denial analyzer fixes and resubmits denials ranked by recoverable dollars — recapturing money that would otherwise be abandoned.
Faster cash, lower DSO	Auto-submit and ERA auto-posting compress the claim-to-cash cycle, so the same revenue arrives sooner and working capital improves.
Coverage you didn't know about	Insurance Discovery finds active coverage from a patient's demographics — turning would-be self-pay or uncollected balances into billable claims.
Every billable cycle captured	Capped-rental month tracking and automatic secondary / COB drafting make sure no rental month or leftover balance is missed.
More new patients	The in-browser AI mask fitter and storefront convert visitors without an appointment, widening the top of the funnel.

The biller's job, transformed

Nowhere is the impact bigger than the biller's seat. The platform moves the work upstream — verify, scrub, prevent — and automates the cleanup, so a single biller can carry far more claims, cleanly.

Insurance verified before you bill. Coverage is confirmed by real-time 270/271 on a schedule and again in the seconds before a claim transmits. Inactive or prior-auth-required coverage is held back automatically — so claims don't bounce for eligibility and you stop billing into dead coverage.

Only clean claims go out. Every claim is scrubbed — structurally and by AI — before submission: wrong modifier for the rental month, quantity over the LCD limit, HCPCS/diagnosis mismatch, fee-schedule drift. Problems are flagged or fixed first, so your first-pass acceptance rate climbs and rejections fall.

Denials research and fix themselves. When a denial lands, the AI reads the CARC/RARC codes, researches the root cause, populates the corrective patch, and — when it's safe and high-confidence — resubmits in one click. The worklist is ranked by recoverable dollars, so the biggest recoveries happen first instead of claims aging into write-offs.

Paid faster, and audit-ready. Clean claims plus automatic payment posting mean cash arrives sooner (a lower DSO). And because required paperwork is enforced by bill hold, signed on file via e-signature, and every claim and signature carries a tamper-evident audit trail, you go into any payer audit far less exposed.

Demo mode — you're viewing sample data, not the live store. Most pages are seeded with example data, but not every feature works in the demo; some areas will appear empty. [Exit demo](#)

CareMetric Breathe
ADMIN WORKSTATION

Signed in as demo.admin@caremetric.example ADMIN Sign out

WORKSPACE PATIENTS & CLINICAL ORDERS & SHOP BILLING Dashboards Worklists A/R & collections Tools ANALYTICS & REPORTS SYSTEM

Verify insurance Insurance discovery **Eligibility** Re-verification Prior auths CMN / DIF worklist Bill hold
ADR / audit response Audit readiness Collections Auto-submit AI queue Denials worklist

Eligibility worklist

Every 270/271 round-trip in the selected window. Rejected and transport-failed checks are where the team's time pays off — those are coverage rows about to bite us.

Run a verification

Filters

Window: 7d 30d 90d Status: All

TOTAL IN WINDOW: 4 ACTIVE COVERAGE: 1 INACTIVE / REJECTED: 2 PRIOR-AUTH REQUIRED: 1

Recent eligibility checks
Newest first — 4 rows

STATUS	PAYER	HCPCS	ACTIVE / IN-NETWORK	DEDUCTIBLE	OOP MAX	COINSURANCE	PA?	REQUESTED	PATIENT
PARSED	Sample Medicare	E0601	active - in-network	\$250.00 met \$185.00	\$3,000.00 met \$450.00	20% copay \$0.00	NO	6/23/2026, 7:56:09 PM	Details
PARSED	Acme Health Demo	A7030	inactive	—	—	—	—	6/23/2026, 7:56:09 PM	Details
REJECTED									
REJECTED									

Ask CareMetric Copilot

Coverage is verified before billing and re-checked the moment before a claim goes out — claims stop bouncing for eligibility.

Hours back, every day — and what they cost

Because CareMetric Breathe is one platform — inventory, billing, the customer database, clinical care, and documentation together — the handoffs that used to mean re-keying, portal-hopping, and phone tag disappear. The estimates below are illustrative for a typical single-location PAP operation (about 1,500 active resupply patients; a team of one owner, one biller, two CSRs, and one respiratory therapist). Tune them to your own volumes.

Where the time goes — by feature

Feature	Replaces this manual work	Time saved
AI mask fitter	In-person fitting appointments and trial-and-error exchanges	15–30 min / new setup
Reminder engine + voice agent	Outbound reorder calls and voicemail loops	6–10 min / resupply order
Real-time eligibility	Payer phone calls and portal checks	10–15 min / verification
AI scrubbing + ranked denials	Hunting denial causes claim by claim	~15 min / denial worked
Therapy-cloud sync	Pulling three vendor portals for compliance data	~10 min / patient / month
Unified inbox + patient 360	Cross-referencing phone logs, email, fax, and billing	30–60 min / rep / day
Fax OCR + e-signature packets	Manual filing, printing, and signature chasing	5–10 min / document
PacWare sync	Re-keying patients and orders into the billing system	3–5 min / record

Rolled up by role

Role	What runs itself	Per day
Administrator (Owner)	Live dashboards and KPI alerts replace hand-built reports and spreadsheet checks	~45 min / day
Biller	Eligibility runs itself; claims arrive scrubbed and denials arrive ranked	~2.0 hrs / day
Customer Service Rep (each of 2)	Reorders confirm themselves by text, link, or the AI phone agent; one inbox replaces five tools	~2.5 hrs / day
Respiratory Therapist	Compliance data lands nightly and worklists build themselves	~1.5 hrs / day

~9.25 staff-hours back per day	~200 hours back per month	~\$5,000 per month (~\$60K/yr) at \$25/hr
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And that is labor alone. Consolidating onto one platform also retires the rest of the stack — the business-management system, the resupply add-on, the e-ordering tool, and the spreadsheets between them — along with the subscriptions and integration upkeep they carry.

What it adds up to

The labor estimates below total roughly 200 staff-hours and about \$5,000 a month for a typical single-location practice. The revenue side compounds on top: even a few points of improvement on first-pass rate, denial recovery, and reorder confirmation — on the same patient base — adds materially more collected revenue every month. Together that is the return: more dollars in, fewer hours spent, on one platform.

Illustrative planning estimates, not a guarantee. Actual savings depend on patient volume, payer mix, and current workflows. Per-task figures reflect the manual workflows each feature replaces; the roll-up assumes the team mix above, 21.7 workdays per month, and a \$25/hour fully loaded labor rate ($9.25 \text{ hrs/day} \times 21.7 \text{ days} \approx 200 \text{ hrs}$; $200 \text{ hrs} \times \$25 \approx \$5,000/\text{month}$, $\approx \$60,000/\text{year}$).

CareMetric Copilot & the AI Assistants

Two AI assistants work alongside the business around the clock — one on the storefront answering patients, one inside the admin console guiding staff. Both are built in, both fall back gracefully (Claude first, then OpenAI, then a safe offline reply) if a vendor key is missing, and both are renamable per tenant — the Penn Home Medical Supply tenant calls them PennBot and PennPilot.

CareMetric Assistant — the patient's 24/7 answer

The storefront chatbot (PennBot for the Penn tenant) is the customer-facing assistant.

Answers patients 24/7. A chat assistant on the storefront answers questions about CPAP therapy, masks, resupply, insurance, and your company — instantly, day or night, with no hold queue and no callback. It is the front line for the routine questions that used to ring the phone.

Knows the patient's account. For a signed-in patient it is account-aware: it can pull recent shipments, order status and tracking, what they are due for next, and device details, and help with returns and account changes. So “where's my order?” and “when's my next refill?” answer themselves.

Knows when to hand off. Anything it can't resolve — order-specific, clinical, or an action that needs a person — it escalates to a CSR by opening a message thread, with the context attached. It never accepts sensitive data (SSN, card, member ID), it is not a clinician, and it gives no medical advice.

A sleep coach, too. In sleep-coach mode it offers supportive, plain-language coaching and troubleshooting for patients adjusting to therapy — the kind of reassurance that keeps new patients adherent.

The same brain by email. The optional email auto-reply uses the same knowledge base to answer inbound patient emails automatically — but only when it is highly confident. Anything below the confidence bar (or order/clinical-specific) falls through to a human, exactly like a hand-off in chat.

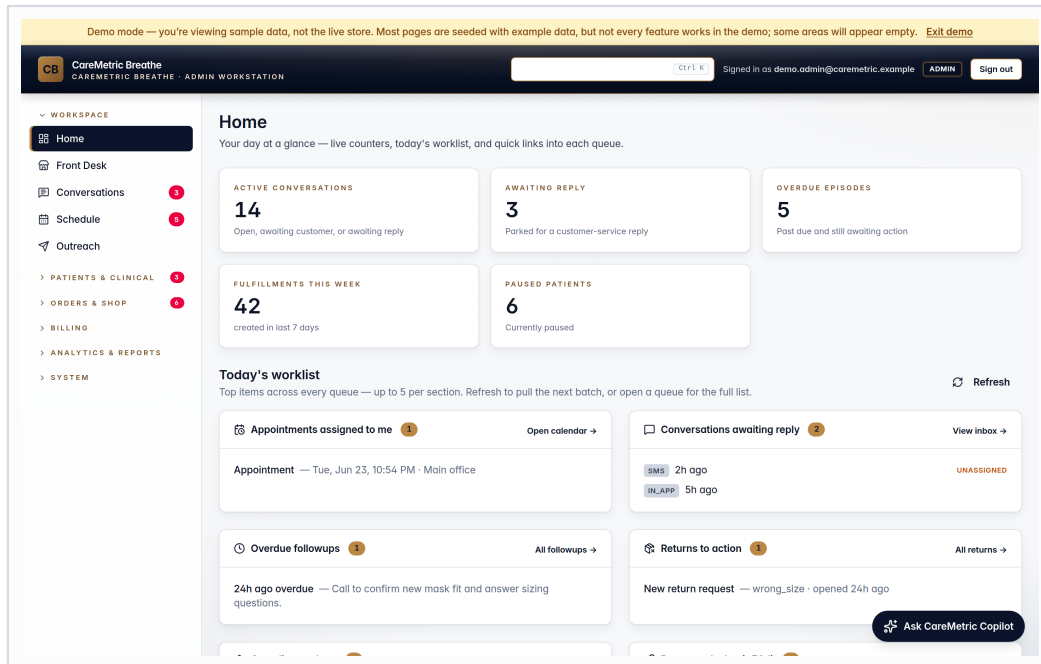
CareMetric Copilot — every employee's guide to the app

The in-app admin assistant (PennPilot for the Penn tenant) is the staff-facing helper, on every admin page.

Every employee's guide to the console. CareMetric Copilot floats on every admin page. Ask it “how does this work?” or “where's the page that does X?” and it answers from a complete map of the console and returns one-click links straight to the right screen. New hires get productive without interrupting a supervisor, and veterans find the rarely-used corner fast.

Walks staff through the work. Beyond finding pages, it explains workflows in plain language — “walk me through processing a claim,” “how do I turn a feature on?” — so the answer to a how-do-I question is a chat away instead of a training ticket.

Turns ideas into action — safely. Its one action, after the employee confirms, emails a structured feature suggestion to the owners. It takes no other actions: it never changes data, never sends anything silently, and never echoes patient PHI. It's an on/off toggle in the Control Center.



“Ask CareMetric Copilot” sits in the corner of every admin page — staff get an answer without leaving the work.

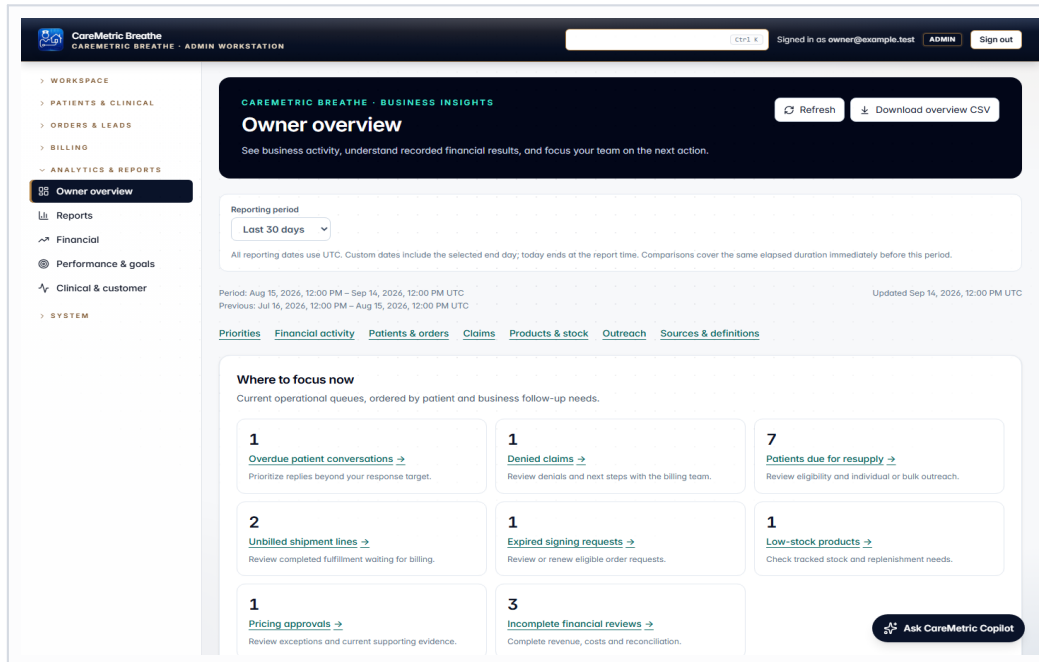
What a deflected call is worth

Every routine question a patient resolves in chat is a phone call your team never has to staff. An inbound call ties up a CSR for several minutes of handle time plus the queue around it; a chat the assistant answers costs essentially nothing and happens 24/7 — including the nights and weekends when no one is at the desk. Across a busy resupply line that is hours of phone time removed every week, shorter holds for the calls that do need a person, and patients who get an instant answer instead of waiting for a callback. The Copilot side compounds it: less time lost to “how do I...?” and faster onboarding for every new hire.

Tip *Illustrative: at a \$25/hour blended rate, a 6-minute call is about \$2.50 of labor. Deflecting even 20–30 routine questions a day to chat is roughly \$50–75/day — about \$1,000–1,500 a month — before counting the after-hours coverage you'd otherwise have to pay for. Directional planning figures, not a guarantee; tune to your call volume and rate.*

Running the Business — the Owner's Playbook

Start with Owner overview for recorded activity and current priorities, then open the detailed workload that needs attention. Use Pricing & Profitability to review current costs, accepted-order results and explicit future scenarios. Keep period activity, current queues and lifetime completed-review contribution separate. The platform cannot infer missing invoices, future demand or the business's approved targets.



Owner overview with illustrative sample data: select a period, review priorities, and download the current report.

What to watch, and when

Every day (5 minutes)

- **Owner overview** - refresh the UTC report; check activity, current priorities, source availability and financial date quality.
- **Home dashboard** — conversations awaiting reply, overdue follow-ups, returns to action, fulfillments this week.
- **Resupply calendar** - review due/overdue patients, their exact order history and any skipped outreach before choosing the next contact.
- **Pricing Follow-up** - assign expiring evidence, delivery holds, cost overruns or collection gaps to an owner and review date.
- **Operations** — the background worker and nightly sync ran clean; nothing stuck.
- **Delivery Failures / Outbound Messages** — clear any bounced texts/emails or shipping exceptions.
- **KPI Alerts** — anything that crossed a threshold overnight (revenue dip, denial spike, churn).
- **Live Staffing** — open-conversation load per agent so nobody is buried.

Every week (20 minutes)

- **Reviews & actuals** - reconcile recorded invoices and collections, preserving original quote amounts and documenting completeness.
- **Supplier costs / Collection evidence** - verify changed or expiring sources before they block new patient reviews or scheduled prices.
- **Billing Hub + Denials & DSO** — money in flight, denial rate, and days-to-pay trend per payer.
- **A/R Aging + Filing Deadlines** — work the oldest buckets and anything near timely-filing.
- **Therapy Fleet + Setup Adherence** — who is at risk of failing their CMS 90-day window.
- **Team Throughput** — per-agent close/approve/resolve counts.
- **Reorder Reminders funnel** — due → reminded → confirmed → shipped conversion.

Every month (45 minutes)

- **Owner overview** - compare equal-duration activity; assess separate lifetime completed-review contribution and excluded/incomplete coverage.
- **Owner models** - explicitly test break-even, cost changes, order volume, repeat-order acquisition and funding assumptions. Download the current scenario report when a planning record is needed.
- **Bulk prices** - preview future changes with current evidence and retained contexts. Activate or schedule only after reviewing the complete snapshot.
- **Reports** — export the revenue summary, patient payments, and insurance-claims reports (CSV / PDF / QuickBooks) for the books.
- **Goals & Targets** — review pace-to-goal and reset targets for the next period.
- **Customer NPS** — post-delivery satisfaction and the comment tail.
- **Package & Usage** — your plan, add-ons, and usage for the month.

Monitor every corner from one screen

Because it is one platform, you watch the whole business without leaving it. Where to look, by area:

Owner overview	One report for patient/order activity, current queues, claims, supply scheduling, stock, shipping and outreach; tracked pricing financial activity and lifetime completed-review contribution stay separate.
Pricing & planning	Pricing & Profitability - costs and evidence, item/patient reviews, new-item proposals, owner models, bulk versions, actuals and assigned follow-up.
Patients & service	Home, Conversations, Cases, Follow-ups, Live Staffing — every patient touch in one inbox with assignment and SLA visibility.
Revenue cycle	Billing Hub, Denials & DSO, A/R Aging, Collections Forecast and Payer Profitability - use each page's own claims, dates and collection definitions.
Clinical & therapy	RT Overview, Therapy Fleet, Setup Adherence — adherence and clinical risk across every device cloud.
Storefront & growth	Storefront Analytics, Acquisition Funnel, Reorder Reminders, Fitter Prospects — where patients come from and convert.
Operations & delivery	Operations, Integrations, Outbound Messages, Delivery Failures, Webhook Deliveries — the plumbing, at a glance.
Goals & exceptions	Goals & Targets and KPI Alerts — set the number you want; the platform flags the moment you drift from it.

Reports you can run

From **Analytics & Reports** → **Reports**, run any of these over a date range and export them:

Revenue summary	Per-day revenue, refunds, and net rollup.
Orders	Orders created in the date range.
Patient payments	Patient-responsibility amounts collected against a balance (deductible, coinsurance, non-covered items), kept separate from insurance receipts to avoid double-counting. Patients are not charged at a storefront checkout — this is what the billing team collects.
Insurance claims	Billing-side claims and payer receipts.
Refunds journal	A chronological refund ledger.
Returns	Comfort-guarantee returns and RMAs.
Customer activity	Aggregated storefront activity per day — signups, returning orders, active count (count-only, no PHI).
All-financial	One-click bundle: orders + refunds + payer receipts + patient payments, unioned chronologically.

Tip The legacy Reports catalog provides CSV, printable PDF, or QuickBooks (Desktop .iif and Online .qbo.csv). Date range defaults to the last 30 days (up to 90). PHI is minimized — storefront reports hash customer IDs and the customer-activity report is counts only. Owner overview is a separate CSV report with up to 366 days and explicit source coverage; Owner models has its own scenario CSV. Neither inherits the legacy report dates, source definitions or accounting export formats.

Benchmarks — and staying ahead

CareMetric Breathe tracks the metrics a DME operation is judged on and lets you set a target for each, so you always know how you stack up. The “typical target” column lists commonly-cited industry rules of thumb — directional, not a guarantee — and the right column is how the platform helps you beat them.

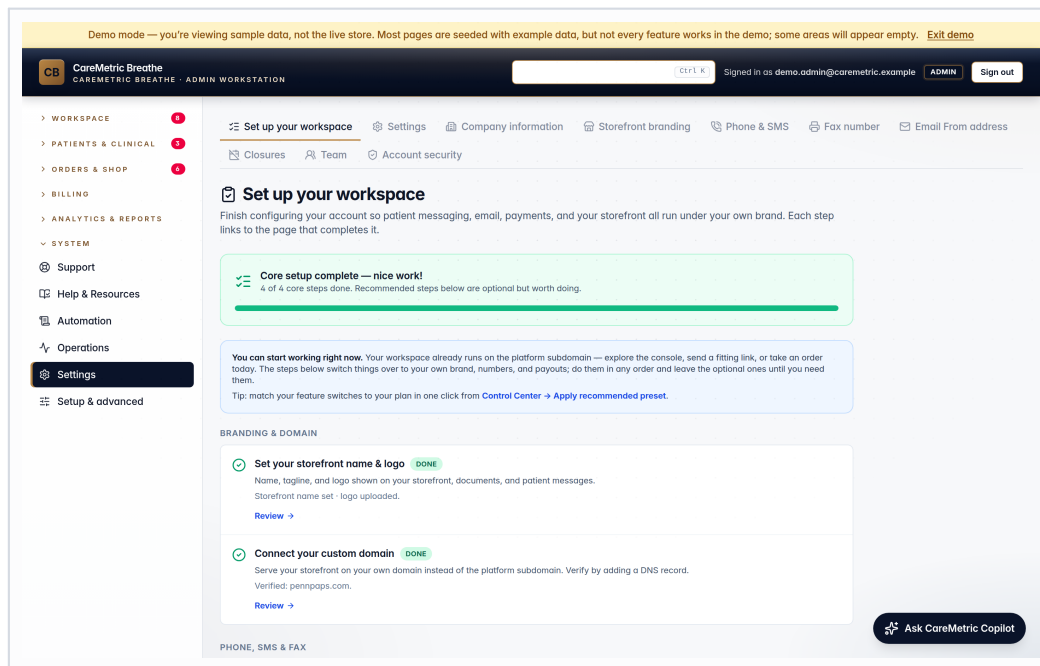
Metric	Typical target	How CareMetric Breathe keeps you ahead
Clean-claim / first-pass rate	≥ 95%	AI scrubbing + the pre-submit eligibility gate stop avoidable rejections before the 837P leaves.
Denial rate	< 5–10%	Denials & DSO benchmarking by payer plus the AI denial analyzer surface and fix the root cause fast.
Days sales outstanding (DSO)	< 30–40 days	Auto-submit, ERA auto-posting, and the recoverable-dollars denials worklist shorten claim-to-cash.
Resupply reorder rate	~30–40% of eligible	Automated reminders with one-tap confirm + the AI voice agent lift confirmations without staff dialing.
CMS 90-day adherence (new setups)	≥ 70% of nights ≥ 4 hrs	Setup Adherence tracks every new patient's rolling 30-day window and flags at-risk early for coaching.
Patient satisfaction (NPS)	Higher is better	Post-delivery NPS, fast omnichannel response, and the self-service portal keep scores up.

Target ranges are widely-cited DME/RCM rules of thumb for orientation only; they are not commitments and vary by payer mix and specialty. What the platform guarantees is the visibility and automation to measure and improve each one.

Setup Guide

Before your practice goes live, a handful of things must be configured so the platform runs under your own identity. Work the checklist below (the Home dashboard also shows a “finish setting up” banner until the essentials are done), then review the Control Center toggles so you know exactly what is on and off.

Before you go live



Set Up Your Workspace — the guided checklist that walks a new practice through the essentials.

Accounts & access

Create your workspace

Sign up at the public site (/breathe → Create your account): enter your company name, work email, a password (12+ characters), and a plan. That provisions your organization and your first Owner login; verify the emailed link, then sign in at /admin/sign-in. (Operators can also pre-provision a workspace from the command line, but self-serve sign-up is the normal path.)

Enroll multi-factor authentication

Each admin should add an authenticator app under System → Account Security before handling patient data.

Invite your team

Add staff on the Team page and give each the right role — Owner, Admin, CSR, Biller, or Respiratory Therapist — so they see only what they need.

Apply your plan's recommended preset

In the Control Center, one click sets your feature flags to the recommended bundle for your plan. New tenants already start on it; re-apply it after switching plans, then fine-tune individual flags.

Brand & identity

Company information	Enter your legal name, addresses, and identifiers — they print on documents, statements, and the storefront.
Storefront branding	Set the storefront name, tagline, logo, and theme. No logo yet? Generate a one-click starter monogram from your storefront name so nothing shows blank while you source artwork.
Custom domain	Point your domain at the platform so patients see your brand (the platform fronts custom domains through Cloudflare).

Communication channels

Phone & SMS numbers	You can start messaging immediately: until you provision your own number, texts and calls go out on a shared platform number, and a patient's reply is routed back to the tenant that owns them. Provision your own dedicated voice and SMS numbers (through the platform's carrier, Twilio/Telnyx — you don't supply or port a line) when you want messages to come from your own number, typically as volume grows. Set this under Phone & SMS.
Fax number	Faxes work on the shared platform number too; provision your own for inbound referrals and outbound signed paperwork when you're ready.
Email From address	The platform default sender works out of the box; to send from your own address, set it AND authenticate the sending domain (SPF/DKIM) in the email provider — an unauthenticated address still sends but lands in spam.

Money & integrations

Platform subscription billing	Complete the practice's CareMetric subscription setup separately from patient insurance ordering. Platform billing credentials do not activate patient card checkout or establish patient-order processing costs.
Pricing readiness	Enter the business-approved margin policy, canonical supplier costs, pack/fee/delivery evidence and insurance collection assumptions before making approved reviews mandatory. Test normal and exception cases first; blank values are not verified zero.
Clearinghouse & billing config	Enter Office Ally (and, if used, Da Vinci PAS) credentials under System Configuration, then set up payer profiles and fee schedules under Billing → Config before submitting claims.
Therapy-cloud integrations	Add ResMed AirView / Philips Care Orchestrator / 3B React Health credentials so adherence data flows into the RT boards.
Slack (optional)	To bring alerts and digests into Slack, add a Slack bot token + channel under System Configuration and link each teammate's Slack handle on the Team page; the Slack toggles ship on but stay inert until the credentials are set.

Tip Most of these live under **System** → **Settings and System** → **Setup & Advanced**. You can launch without the optional integrations — the platform degrades gracefully when a credential is unset; the matching feature simply stays idle.

Ready to bill on day one — the pre-loaded payers

You don't start with an empty payer table. CareMetric Breathe ships with **100+ fully-configured payer profiles** — the complete Pennsylvania DME market plus the national carriers, Medicare, Medicaid, federal, and workers'-comp / auto programs a PAP business actually bills. Each one arrives with the electronic IDs, claim format, timely-filing window, prior-auth rules, and claims address already filled in, so you can submit clean claims on day one instead of researching payer setup for weeks. Billing for a different state? Adding a payer takes a minute on the billing config page — the pre-loaded set is a head start, not a fence.

What's in the box

Pennsylvania Blues & major commercial	Highmark BCBS (Western & Central PA), Independence Blue Cross, Capital BlueCross, UPMC Health Plan, Geisinger Health Plan.
National commercial & employer TPAs	Aetna, Cigna, UnitedHealthcare, Humana, AmeriHealth, Ambetter, Oscar, Surest — plus self-funded administrators (Meritain, UMR, Allied, HealthSmart, Luminare, WebTPA, Imagine360, Nova, MagnaCare, EBMS, Independence Administrators).
Medicare (Part B, DME MAC, Railroad)	Novitas Solutions (PA Part B), Noridian (DME MAC Jurisdiction A), and Palmetto GBA Railroad Medicare.
Medicare Advantage & D-SNP	Highmark Freedom Blue, UPMC for Life, Geisinger Gold, Aetna Medicare, Capital Senior Blue, Keystone 65, Personal Choice 65, Cigna, Wellcare, Devoted, Clover, Humana Gold Plus, UnitedHealthcare Dual Complete, and more.
Medicare Supplement (Medigap)	AARP/UnitedHealthcare, Mutual of Omaha, Cigna, and Aetna Medigap plans.
Medicaid, HealthChoices MCOs, CHIP & CHC	PA Medical Assistance (FFS), Keystone First, UPMC for You, AmeriHealth Caritas PA, Highmark Wholecare, Geisinger Family, PA Health & Wellness, UHC Community Plan, Health Partners Plans; CHIP and Community HealthChoices (LTSS) plans.
Federal programs	BCBS Federal Employee Program, GEHA, MHBP, NALC, APWU, Compass Rose, SAMBA (FEHB); TRICARE East, TRICARE For Life, VA Community Care Network, CHAMPVA, and Federal Black Lung.
Workers' comp & auto no-fault	SWIF, PMA, Erie, Liberty Mutual, Travelers, The Hartford, plus WC TPAs (Sedgwick, Gallagher Bassett, Broadspire, ESIS) and auto MedPay/PIP (Progressive, Allstate, Nationwide, GEICO, USAA, State Farm). Routed to the right specialty channel, not the standard clearinghouse.
Rental PPO networks (router rows)	Anthem/Elevance BlueCard, First Health, and MultiPlan/PHCS — so you bill the plan on the member's card, not the network.

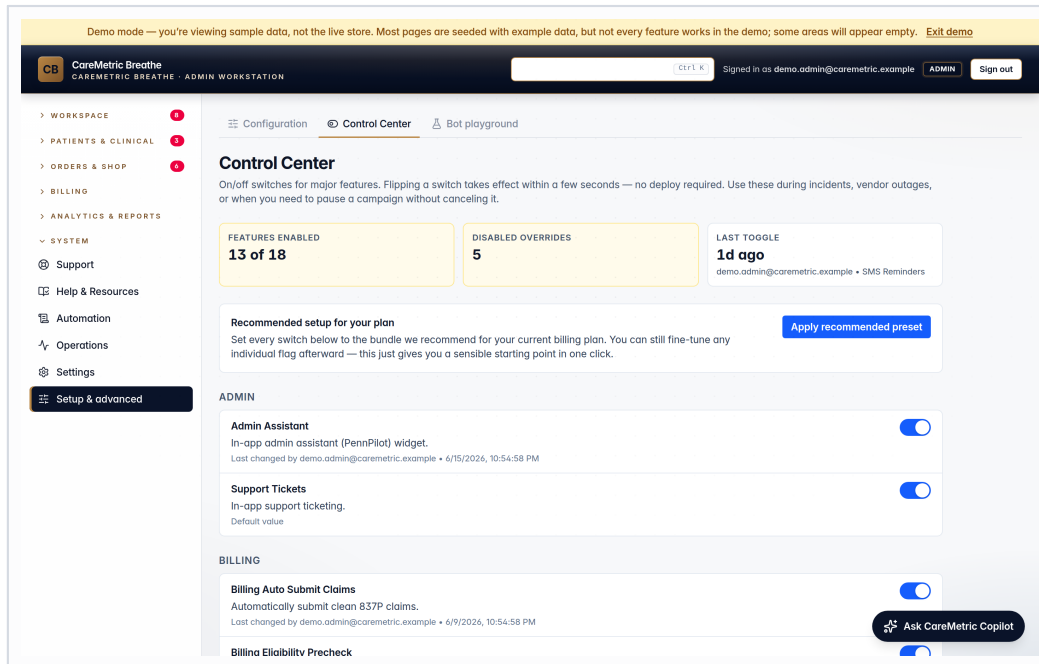
What every payer profile already knows

Identity	CSR-friendly display name, the legal name as it appears on EDI enrollment, the parent organization, and a stable internal code.
Electronic IDs & format	Office Ally payer ID, 5010 payer ID, and ERA payer ID; the claim format (837P, 837I, or paper CMS-1500); and EDI/ERA enrollment status.
Submission rules	Timely-filing window, any required claim modifiers (e.g. Medicare's KX), whether electronic secondary/COB is accepted, whether a referring-provider NPI is required, and a member-ID format hint/pattern.
Prior authorization	Whether capped-rental DME needs prior auth, the submission method (portal, fax, phone, electronic 278, or paper), the PA phone/fax, and the expected turnaround.
Claims routing	Claims mailing address (and a separate appeals address when different), claims phone/fax, the provider-portal URL, and the fee-schedule source.
Bookkeeping	Free-form payer-level notes (PHI-free), an active/inactive flag, and a last-verified date and author so the team can spot stale entries at a glance.

***Tip** Payer profiles are platform-wide and operator-maintained, so every tenant inherits the same vetted catalog. Manage them under **Billing** → **Config** → **Payers**: filter by region or line of business, add or edit a payer in a drawer (no deploy needed), and export the catalog in Office Ally's enrollment-review format for quarterly payer-ID updates.*

The Control Center — every toggle explained

The Control Center (**System** → **Setup & Advanced** → **Control Center**) is the master switch panel. Every major capability has a boolean on/off toggle that takes effect immediately — no deploy needed. Toggles are Owner-controlled and every change is recorded. The tables below list every toggle, its default, and what it does. A feature whose key shows after a dot (for example `billing.auto_submit_claims`) is the exact key you will see in the panel. Many automations also require a matching credential or schedule to actually act — turning the toggle on without it is safe (it simply stays idle).



The Control Center — master on/off switches for every major feature, grouped by area.

■ Messaging

SMS — reminders

`sms.reminders`

● On by default

Outbound SMS resupply reminders (90/30-day reorder nudges). Disabling stops the reminder dispatcher from sending new SMS.

Email — reminders

`email.reminders`

● On by default

Outbound email resupply reminders. Disabling stops the reminder dispatcher from sending new email.

Bulk campaigns — send

`bulk_campaigns.send`

● On by default

Sending of bulk-messaging campaigns to opted-in patients. Disabling stops the bulk-campaign worker from sending new messages.

Outreach playbooks — dispatcher

`outreach_playbooks.dispatcher`

● On by default

Outreach playbooks — scheduled SMS/email touches and call tasks for patient outreach runs started by staff. Turning this OFF pauses all pending playbook touches without cancelling the runs.

Smart triggers — dispatcher

`smart_triggers.dispatcher`

● On by default

Smart-trigger reorder dispatcher (therapy-night patterns). Disabling stops the dispatcher from queueing new reminders.

Patient onboarding — dispatcher

`patient_onboarding.dispatcher`

● On by default

First-90-day adherence-coaching nudges. Disabling stops the onboarding dispatcher from sending day-1/7/30/90 messages.

Fitter supply campaign — dispatcher

`fitter_supply_campaign.dispatcher`

● On by default

Multi-touch nurture campaign for patients who completed the at-home fitter but have not placed an order. Disabling stops the dispatcher from sending new touchpoints; in-flight conversions still attribute normally.

Reminder escalation — dispatcher

`reminder_escalation.dispatcher`

● Off by default

Daily multi-channel escalation for unanswered resupply reminders: follow up on the next channel (SMS then email) ~3 days after the first touch, then raise a CSR call alert once both channels are exhausted. Disabled keeps reminders to the single first-touch channel.

Alerts — auto dispatch

`alerts.auto_dispatch`

● Off by default

Allow server-side events to automatically send alert-library messages to patients (currently: the `payment_failed` alert on a Stripe invoice.`payment_failed` event). Disabled keeps the alert library admin-send only. Review patient-consent and wording before enabling.

Therapy fleet — auto outreach

`therapy_fleet.auto_outreach`

● Off by default

Therapy-fleet adherence auto-outreach: gentle SMS nudges to consented at-risk patients from the nightly fleet-alerts scan and the setup-deadline job. Also requires `sms.reminders` ON and Twilio configured; per-patient consent / DND / frequency caps still apply. SEEDER DISABLED.

Clinical outreach — dispatcher`clinical_outreach.dispatcher`

● On by default

Proactive clinical outreach: templated, consent/DND/frequency-cap-gated nudges to patients with an open non-adherence intervention. Scheduling is controlled by CLINICAL_OUTREACH_CRON; turning this OFF pauses the sends without changing the schedule. The admin "Run now" trigger is unaffected.

Fitter first day nudge — dispatcher`fitter_first_day_nudge.dispatcher`

● On by default

Fitter lead first-day nudge: a one-time email/SMS to new fitter leads on day one. Registration is gated by RESUPPLY_FITTER_FIRST_DAY_NUDGE_ENABLED; turning this OFF pauses the sweep without changing that env gate.

Fitter reengage — dispatcher`fitter_reengage.dispatcher`

● On by default

Fitter lead re-engagement: follow-up outreach to fitter leads who have gone quiet. Registration is gated by RESUPPLY_FITTER_REENGAGE_ENABLED; turning this OFF pauses the sweep without changing that env gate.

Patient packets — autosend on delivery`patient_packets.autosend_on_delivery`

● Off by default

Automatically email a new-patient document packet for e-signature the first time an order is delivered to a customer linked to a patient record. One-time per patient; the admin "Send packet" action is always available regardless of this toggle. OFF by default.

Patient packets — autoremind`patient_packets.autoremind`

● Off by default

Automatically re-send the signing link to patients who have not finished their document packet, on a fixed cadence with a maximum number of nudges. The sweep runs daily (override with PATIENT_PACKET_REMINDER_CRON); turning this OFF pauses the sends without changing the schedule. OFF by default.

■ Voice & AI

Email — auto reply

`email.auto_reply`

● Off by default

Chatbot email auto-replies. When ON, inbound patient emails are answered automatically by the storefront chat assistant (Claude Sonnet / GPT fallback); it hands off order/account/clinical or low-confidence messages to a human. When OFF, every inbound email is routed to the admin inbox for a manual reply.

Voice — agent

`voice.agent`

● On by default

Inbound voice agent (OpenAI Realtime). Disabling returns a 503 / hangup TwiML to any inbound call.

Telehealth — video

`telehealth.video`

● On by default

Telehealth video visits — staff-initiated browser video calls with patients for setups, troubleshooting, and follow-ups. Disabling blocks new visit creation and dead-ends outstanding join links.

Storefront — chatbot

`storefront.chatbot`

● On by default

Storefront chat assistant (Claude Sonnet / GPT fallback). Disabling makes the chat widget return a friendly "currently offline" message.

Admin — assistant

`admin.assistant`

● On by default

CareMetric Copilot — the in-app admin tech-support and program-manager assistant (renamable per tenant; "PennPilot" for the Penn tenant). When ON, signed-in staff can ask how the app works and where to find features, and the assistant can email feature suggestions to the super-admins (after confirming). Claude Sonnet / GPT fallback; degrades to an offline reply when no AI provider key is set.

Reminder escalation — voice

`reminder_escalation.voice`

● Off by default

Automated phone-call tier for the resupply-reminder escalation ladder. When ON (and the voice path is configured), an episode still unanswered after the SMS and email reminders gets an automated AI resupply check-in call before the ladder raises a CSR "call them" alert. Placed inside the patient's local 9am–8pm window. When OFF, the ladder is SMS → email → CSR alert.

Voice — breathe sales

`voice.breathe_sales`

● Off by default

CareMetric Breathe B2B platform sales voice agent. When ON (and a sales phone number is configured), inbound calls to the dedicated platform line reach an AI sales rep that pitches the platform to prospective DME businesses, emails them information, captures leads for follow-up, and can start a tenant sign-up on the call. Seeded OFF — it answers regulated calls and costs real voice spend, so enable it once the sales number is wired up.

Support — tickets

`support.tickets`

● On by default

Support tickets — tenants file support requests to the platform operator; an AI bot answers from the admin-console knowledge base on intake (Claude / GPT fallback) and escalates anything it cannot confidently answer to a human. Degrades to a hand-off when no AI provider key is set.

■ Storefront

Storefront — checkout (retired)

`storefront.checkout`

● On by default

Retired. Patients are billed to insurance and are never charged a card, so there is no storefront checkout for this toggle to gate. The row still exists in Control Center; changing it has no effect.

Storefront — pickup (retired)

`storefront.pickup`

● Off by default

Retired. It offered in-store pickup at a checkout that no longer exists. Arrange a pickup with the patient directly instead; changing this toggle has no effect.

Storefront — reviews collection

`storefront.reviews_collection`

● On by default

Post-delivery review-request emails. Disabling stops new review-request emails; previously-sent ones still accept submissions.

Storefront — NPS

`storefront.nps`

● On by default

Post-delivery NPS survey emails. Disabling stops new NPS sends; previously-sent surveys still record responses.

Cart abandonment — dispatcher (retired)

`cart_abandonment.dispatcher`

● On by default

Retired. There is no cart to abandon — the storefront does not sell. Changing this toggle has no effect; use Bulk Campaigns for outreach to patients who dropped out of a fitting.

Storefront — auto reminder enrollment

`storefront.auto_reminder_enrollment`

● Off by default

On a paid storefront order, auto-enroll the buyer in replacement reminders for the consumables they purchased (mask/cushion/tubing/filter/etc.) at the standard cadence. Email-only, with a manage/unsubscribe token; never re-enrolls a prior unsubscribe. Disabled keeps reminder enrollment to the explicit /reminders opt-in. Review consent before enabling.

Billing

AI billing — suggestions

`ai_billing.suggestions`

● On by default

AI-assisted billing claim suggestions in the billing AI queue. Disabling returns suggestions as "AI offline"; manual workflow is unaffected.

Billing — auto submit claims

`billing.auto_submit_claims`

● Off by default

Unattended auto-submission of submission-ready claims to Office Ally by the `billing.auto-submit-batch` worker. When ON (and `CLAIMS_AUTOSUBMIT_CRON` is set) the cron batches draft claims that pass preflight with no blocking errors AND have active eligibility on file, then transmits them per payer. When OFF, the cron is a no-op; the operator-driven staged-approval submit (Billing → Auto-submit) still works regardless. SEEDED DISABLED.

Billing — auto submit prior auths

`billing.auto_submit_prior_auths`

● Off by default

Unattended automatic Da Vinci PAS submission of draft prior authorizations by the `billing.prior-auth-auto-submit` worker. When ON (and `PRIOR_AUTH_AUTOSUBMIT_CRON` is set) the cron front-loads draft PAs that carry a coverage through the same submit core as the manual button; PAs missing a diagnosis, payer PAS endpoint, credentials, or address are skipped and retried. When OFF, the cron is a no-op; the operator-driven manual submit still works regardless. SEEDED DISABLED.

Billing — eligibility precheck

`billing.eligibility_precheck`

● Off by default

Before batch-submitting 837P claims to Office Ally, consult each claim's most recent parsed 270/271. A claim whose coverage is explicitly inactive or requires prior auth is held back and the batch returns `eligibility_blocked` with the offending claim ids. Fail-open: no/stale eligibility result allows the claim through. Pairs with `resupply.eligibility_enforcement` (the order-confirm coverage guard).

Billing — eligibility precheck refresh

`billing.eligibility_precheck_refresh`

● Off by default

When `billing.eligibility_precheck` is on AND real-time eligibility is configured, the claim-submit precheck additionally runs a fresh real-time 270 for any coverage in the batch with no recent parsed 271 (deduped per coverage, capped per batch). Off → the precheck only consults cached results. Auto-firing 270s has a per-transaction cost, so this is opt-in separately.

Insurance — discovery

`insurance.discovery`

● Off by default

Insurance discovery add-on. When ON (and the Office Ally discovery endpoint is configured), staff can search the payer network from a patient's demographics to find active coverage when their insurance is unknown or a coverage on file came back inactive. Seeded OFF — it is a paid add-on that triggers real clearinghouse spend, so enable it for tenants that have purchased it.

Billing — line ordering provider

`billing.line_ordering_provider`

● Off by default

Emit the line-level ordering-provider loop (837P 2420E NM1*DK) on each claim line, sourced from the claim's referring (prescribing) provider. DMEPOS-strict placement so Medicare's PECOS edit binds at the line; additive to the claim-level 2310D referring loop. SEEDED OFF — turning it on changes the live 837P, so validate against a live 277CA in the Office Ally test (T) cycle first.

Billing — payment plan autocharge

`billing.payment_plan_autocharge`

● Off by default

Auto-charge due patient payment-plan installments against the patient's authorized card. Off by default.

Billing — patient autopay
billing.patient_autopay

● Off by default

Auto-charge a patient's outstanding balance against the card they saved and authorized in the patient portal. Off by default; each patient also controls their own autopay toggle.

Billing — auto secondary claims

billing.auto_secondary_claims

● Off by default

Auto-draft a secondary / coordination-of-benefits claim when a primary payer pays a claim that carries a secondary coverage and leaves a patient-responsibility balance. The secondary lands in draft for a biller to review and submit — never auto-submitted. Off by default; when off, use the manual COB worklist at /admin/billing/secondary-eligible.

Eligibility — auto reverify
eligibility.auto_reverify

● On by default

Scheduled eligibility re-verification: fires fresh 270s for the most-urgent active coverages (throttled + capped per run). Scheduling is controlled by ELIGIBILITY_REVERIFY_CRON; turning this OFF pauses the unattended 270s without changing the schedule. The admin "Run batch now" trigger is unaffected.

Billing — bill hold
billing.bill_hold

● On by default

Bill hold. When ON, a claim cannot be submitted to the clearinghouse while it has any outstanding REQUIRED signed-paperwork requirement; the hold lifts automatically when the last one comes back (manual mark, portal e-sign, upload, or an inbound fax auto-matched to it). A claim with no tracked requirements is never held. When OFF, the gate is skipped entirely.

Billing — bill hold auto remind

billing.bill_hold_auto_remind

● Off by default

Bill-hold auto-reminders. When ON (and the opt-in BILL_HOLD_SWEEP_CRON is scheduled), the sweep job sends a reminder for paperwork that has been outstanding past the reminder threshold. When OFF, the sweep still recomputes holds and seeds requirements but sends nothing — reminders are sent only from the per-requirement "remind" action.

Billing — ADR / audit-response queue
billing.adr_queue

● Off by default

Medicare ADR / audit-response queue + audit-packet builder. When ON, staff can log payer/contractor Additional Documentation Requests against their response deadline, assemble the requested chart documents + generated summaries into one audit-packet PDF, and record the outcome. A nightly SLA sweep surfaces at-risk/overdue deadlines. When OFF, the queue and builder are hidden and the sweep no-ops.

Collections — dunning ladder

collections.dunning

● Off by default

Patient A/R dunning ladder. When ON, unpaid patient-responsibility balances escalate on a cadence (statement → reminder → second notice → final notice → agency), pausing automatically the moment the balance is paid or the patient goes onto a payment plan / autopay. Consent + quiet-hours are always enforced. When OFF, the open-scan and tick jobs no-op.

Collections — agency export

collections.agency_export

● Off by default

Collections-agency hand-off export. When ON, balances that reach the agency step can be exported (formula-injection-guarded CSV) for a collections agency. Nothing is ever sent to an agency automatically — the export is a reviewed, deliberate action. When OFF, the agency step simply parks the balance for review.

■ Resupply

Resupply — entitlement enforcement

`resupply.entitlement_enforcement`

● Off by default

Block a resupply confirmation when the item is not yet payable under the Medicare/payer replacement schedule (too soon since last dispense, or over the per-period quantity cap). When ON, a blocked reorder is routed to a CSR via a `resupply_too_soon` alert instead of shipping. When OFF, confirmations ship as before. Fails open on any eligibility-lookup error.

Resupply — eligibility enforcement

`resupply.eligibility_enforcement`

● Off by default

Consult the cached 270/271 eligibility result at order-confirm time. When ON and the patient confirms a resupply, an explicitly inactive plan or a prior-auth-required flag raises a `resupply_coverage_blocked` CSR alert and routes the order to the work queue instead of auto-shipping. Fail-open: no/stale result allows the order. When OFF, the coverage check is skipped (cadence/quantity entitlement is gated separately by `resupply.entitlement_enforcement`).

Resupply — usage compliance check

`resupply.usage_compliance_check`

● Off by default

Continued-use check at resupply order-confirm time. When ON and the patient confirms a resupply, recent therapy data (`patient_therapy_nights`, last 30 days) showing the device is effectively unused raises a `resupply_usage_review` CSR alert and routes the order to the work queue instead of auto-shipping — protecting against continued-use claim denials. Fail-open: no data, sparse data, or any lookup error allows the order. When OFF, the check is skipped.

Resupply — refill affirmation capture

`resupply.refill_affirmation_capture`

● On by default

Record the beneficiary's Medicare/payer refill attestation (still using the device + current supply running low) as an audit-grade `refill_confirmations` row each time a resupply order is confirmed (SMS YES, email-link click, voice, or admin). Captured AFTER a successful confirm; a write failure never blocks the order. ON by default — this is a compliance documentation baseline, not a change to the ship decision. When OFF, no attestation row is written.

Resupply — refill window enforcement

`resupply.refill_window_enforcement`

● Off by default

Enforce the CMS DMEPOS refill SHIP window at order-confirm time: block a resupply that would ship earlier than 10 days before the current supply's expected depletion (last dispense + the HCPCS supply duration), routing it to a CSR via a `resupply_refill_too_early` alert instead of shipping. When OFF, the confirm-time interval/quantity entitlement guard is the only timing gate. Fail-open: a first fill, an unmapped SKU, or any lookup error allows the confirmation through.

Resupply — auto order drafts

`resupply.auto_order_drafts`

● Off by default

Auto-stage resupply order drafts from device-eligible supplies (daily worker). Drafts are proposals a CSR reviews and approves into an order — nothing is created or charged automatically. Seeded OFF; CSRs can still stage drafts manually from the resupply opportunities page. Enable per tenant in the Control Center.

Asset recovery — auto populate

`asset_recovery.auto_populate`

● Off by default

Nightly auto-population of asset-recovery cases from discontinuation signals (undismissed usage-dropping smart triggers). When off, cases are opened only manually from `/admin/asset-recovery`.

■ Orders

Orders — require signed paperwork

`orders.require_signed_paperwork`

● Off by default

Require required intake paperwork (HIPAA NPP, Assignment of Benefits, Supplier Standards) to be signed before a patient-linked order can be marked shipped. Off by default — turning it on blocks shipment until paperwork is on file. A per-payer requirement (`payer_profiles.requires_signed_paperwork`) can also impose this for specific payers while this stays off.

■ Documents

Patient packets — autofile signed pdf

`patient_packets.autofile_signed_pdf`

● On by default

Automatically file the signed e-sign packet PDF (documents + signature certificate) onto the patient's chart when the patient completes signing. Internal record-keeping only — no messages are sent. ON by default.

■ Provider Portal

Provider — portal enabled

`provider.portal_enabled`

● Off by default

Secure provider e-signature portal: physicians/NPs sign in (MFA-protected) to e-sign outstanding orders/prescriptions/claims; employees track + release signed items and print the signature audit log.

■ Integrations

Domains — TLS automation

`domains.tls_automation`

● Off by default

NOT YET ACTIVE in this build — the TLS automation has not shipped, so this toggle currently has no effect. When it lands: automatically provision + renew TLS for tenant custom domains via Cloudflare for SaaS (Custom Hostnames) when a domain is verified. Requires `CLOUDFLARE_API_TOKEN` + `CLOUDFLARE_ZONE_ID`. Off = manual operator edge binding.

Slack — team notifications

`slack.notifications`

● On by default

Post real-time CS alerts (a patient reply needing a human, a voice post-call hand-off, an SLA breach) into the operator's Slack channel. Inert until a Slack bot token + alerts channel are configured in System Configuration. Messages are non-PHI (reference + status + a deep link into the admin console).

Slack — interactivity

`slack.interactivity`

● On by default

Accept inbound Slack button clicks (Escalate) and the slash command. Every request is verified against the Slack signing secret; inert until that secret is configured in System Configuration.

Slack — operator digests

`slack.digests`

● On by default

Also post the operator digests (owner weekly KPIs, metric alerts, stuck-job monitor, low-stock inventory) into Slack. Routes to the optional digests channel, else the alerts channel. Inert until Slack credentials are configured. Non-PHI (counts / KPI headlines / SKU + queue names). The email path is unchanged.

■ Operations

Failed email digest — dispatcher

`failed_email_digest.dispatcher`

● On by default

Daily failed-order email digest: a single summary email to the ops alerts address listing orders whose confirmation email failed in the last 24h (order reference + timestamp only — no patient PHI). Scheduling is gated by `RESUPPLY_FAILED_EMAIL_DIGEST_ENABLED`; turning this OFF pauses the digest without changing that env gate.

Multi location — enabled

`multi_location.enabled`

● Off by default

Multi-branch company. When ON, the admin console shows the Locations page plus per-patient and per-staff branch assignment, a branch filter on the patients list, and per-branch counts. When OFF (the default), the company is treated as a single branch and none of the branch UI appears. Billing identity is shared at the org level in both modes.

Fax — auto file signed

`fax.auto_file_signed`

● Off by default

Inbound-fax barcode auto-filing. When ON, every received fax is scanned (via the existing Claude vision path) for the platform's signature-tracking code (PFS-XXXXXXX). On an exact unique match to an outstanding signature, the fax is filed into the patient's chart, the signature is marked returned & signed (advancing the source prescription packet), and any claim paperwork requirement it was sent to clear is satisfied (releasing the bill hold). When OFF, faxes are triaged by hand exactly as before. Degrades to hand-triage whenever no AI key is configured or the scan finds no code.

Fax — referral review

`fax.referral_review`

● Off by default

AI referral reviewer for inbound faxes. When ON, every received fax that is not barcode auto-filed is queued for one AI extraction pass (Claude document path) that pulls the referral's patient demographics, insurance, ordered items, sleep-study results, and referring physician into a review the staff can edit and explicitly accept — accepting creates the patient record, the insurance coverage, and files the split per-section PDFs to the chart. Nothing is entered into the system without that explicit accept. When OFF, faxes triage exactly as before; the manual "Upload referral PDF" reviewer remains available either way. Degrades to hand-triage when no AI key is configured.

Frontdesk — counter orders (retired)

`frontdesk.counter_orders`

● On by default

Retired with the Front Desk page. A walk-in is handled like any other patient: verify the benefit and raise the order against their plan. Changing this toggle has no effect.

■ Referrals

Referrals — adherence report

`referrals.adherence_report`

● Off by default

Automatically send a patient's 90-day CPAP adherence attestation (Medicare LCD L33718) to their REFERRING PROVIDER, by fax (preferred) or email, once the patient reaches the 90-day therapy mark. This is a PHI disclosure to the treating/referring provider — a permitted treatment/care-coordination disclosure that you opt into deliberately. Seeded OFF: turn it on only once you have decided the disclosure is appropriate and your providers' fax/email is set up to receive it. Each patient's 90-day report is sent at most once. When OFF, nothing is sent.

■ Clinical

Fitter — multiframe capture

`fitter.multiframe_capture`

● Off by default

Guided multi-angle scan capture: a live coach walks the patient through three angles (front, then a slight turn each way) with real-time quality checks (lighting, distance, head position, obstruction, movement), and the measurements are cross-checked across frames for agreement — producing a measurement confidence score instead of a single unverified snapshot. Falls back to single-frame capture automatically on devices that cannot run the live face tracker. OFF keeps the one-photo capture.

Part 1 — Feature Summary by Role

A quick reference: every feature with a one-line description, grouped by the role that uses it most. Part 2 describes each in depth.

ADMINISTRATOR (OWNER & ADMIN)

Administrator

Run the whole practice — setup, team, money, and the controls that govern every other role.

Command center

Home dashboard	Today's work, live KPI counters, and quick links into every queue.
CareMetric Copilot	Floating in-app assistant that answers “how do I... / where is...” and can email feature ideas to the owner.
Support	File a support request and get instant how-to answers from the assistant.

Setup & identity

Set Up Your Workspace	Guided checklist: brand, domain, phone/SMS/fax, email sender, and payments.
Company Information	Legal name, addresses, and contacts printed on documents, the storefront, and messages.
Storefront Branding	Storefront name, tagline, logo (or a one-click starter monogram), and custom-domain wiring.
Phone & SMS / Fax / Email	Provision dedicated voice, SMS, and fax numbers for the practice and set the From email address.
Locations	Define service branches (multi-branch practices) and assign them to patients.
Account Security	Enroll your own multi-factor authentication (authenticator app).

Team & control

Team	Invite staff, set their role (Owner, Admin, CSR, Biller, RT), revoke access, and link each teammate's Slack handle.
Control Center	On/off switches for major features — voice agent, campaigns, auto-submit, AI billing, chatbots.
Recommended preset	One-click “set my feature flags to the bundle for my plan” — adopt or re-baseline the recommended set after picking or switching a plan.
System Configuration	Owner-only vault for integration credentials and platform secrets.
Automation Rules / Rule Tester	“When X happens, do Y” automations, with a dry-run tester before you go live.
Compliance Rules	Per-payer CPAP adherence thresholds (minimum hours / nights).
Bot Playground	Rehearse the chat and voice bots against scripted situations to tune them.

Operations & integrations

Operations	Health of background jobs and pipelines (worker, nightly sync).
Integrations	Therapy-cloud connections (ResMed AirView, Philips Care Orchestrator, 3B React Health) and sync status.
Slack notifications	Post real-time CS alerts and operator digests into your team's Slack channel; act on them with Escalate buttons and a slash command.
PacWare	CSV import/export bridge to the PacWare billing/warehouse system.
Outbound Messages / Delivery Failures	Every SMS & email with its delivery result; retry or resolve bounces.
Webhook Deliveries	Outbound event deliveries to partner endpoints; re-queue failed sends.

Analytics & goals

Owner overview	Recorded business activity, current queues, claims, stock, outreach, and completed-review contribution with explicit reporting periods.
Reports	Exportable CSV/PDF/QuickBooks reports for ops and finance.
Audit Trail	Who accessed which patient's information, and when — filter by employee, patient, and time frame (admins only).
Financial analytics	Margin & COGS, LTV:CAC, inventory turnover, revenue by source, outreach attribution.
Performance & Goals	Team throughput, live staffing load, KPI targets, and threshold alerts.
Clinical & customer analytics	Resupply funnel, reorder reminders, NPS, and storefront traffic.

Pricing & Profitability

Supplier costs & collection evidence	Versioned, dated canonical-unit supplier costs, whole-pack rules, freight coverage and verified insurance collection assumptions.
Pricing policy	Set your own targets, hard floors, minimum dollars and scoped rules; deliberately enable required patient reviews.
Item review & New items	Evaluate exact items and fulfillment costs; review provisional supplier comparisons before mapping a new item.
Bulk prices	Select published item or bundle scenarios, compare old/new economics, retain other prices, then activate or schedule a complete version.
Owner models	Six assumption-driven planning models: pricing strategies, break-even, sensitivity, price/volume, repeat orders and working capital.
Reviews & actuals / Follow-up	Approve eligible exceptions, reconcile dated financial events, and assign cost or collection follow-up.

 BILLER

Biller

Own the revenue cycle — eligibility, claims, A/R, and getting every dollar collected.

Dashboards

Billing Hub	A/R director view — money in flight, top payers, and an aging summary.
Denials & DSO	90-day denial rate and days-to-pay per payer, with trend lines.
Collections Forecast	Projected cash from claims in flight, bucketed by expected landing date.
Chargeback Disputes	Card chargebacks against storefront charges, ordered by evidence deadline.
Payer Profitability	Net yield per payer: billed → allowed → collected, net of cost.

Claim worklists

Verify Insurance	Run a one-off 270/271 eligibility check for any patient.
Insurance Discovery	Find active coverage from demographics when the plan is unknown.
Eligibility	System-wide 270/271 worklist; rejected and inactive coverage rises to the top.
Re-verification	Active coverages due for a re-check (stale, terminating, never checked).
Prior Auths	At-risk SLA, auths expiring soon, and drafts to submit (incl. Da Vinci PAS).
CMN / DIF Worklist	Certificates of Medical Necessity awaiting completion.
Bill Hold	Claims held from billing until signed paperwork is back.
Auto-submit	Preflight-clean claims ready to transmit — approve a batch or let the cron send.
AI Queue	Scrubber-blocked + denial-analyzer worklist with auto-resubmit suggestions.
Denials Worklist	Open denials ranked by recoverable dollars × win-probability.

A/R & collections

A/R Aging	Open claims by 0/30/60/90-day bucket and by payer.
Filing Deadlines	Open claims ranked by days left before timely-filing closes.
Secondary Claims	Coordination of benefits — roll the primary's balance to the secondary payer.
Statement Send	Send patient-responsibility statements by email/SMS, consent-aware.
Collections	Patient balances on a dunning ladder (statement → reminder → notices → agency) that auto-pauses the moment a balance is paid or on a plan.
Capped Rentals	13- and 36-month CMS rental cycle tracker with KH/KI/KX modifier rotation.
ADR / Audit Response	Payer/contractor documentation requests ranked by deadline, with a checklist and win-rate outcomes.
Audit Packet	Assemble a complete CPAP/PAP documentation packet (SWO, CMN, sleep study, compliance, POD...) as one PDF.

Tools & configuration

ERA Files	Upload an 835 to auto-post payer adjudications.
Office Ally	Clearinghouse 837P submissions, acknowledgements, and transmission status.
Manual Claim	Key a corrected / void-replacement / paper-backup claim by hand.
Billing Config	Payer profiles, fee schedules, modifier rules, denial codes, claim templates.
Package & Usage	Your CareMetric subscription plan, add-ons, and monthly usage.

 CUSTOMER SERVICE REP (CSR)

Customer Service Rep

The patient's first point of contact — messages, orders, scheduling, and day-to-day service.

Daily workspace

Home	Today's worklist and live counters across every queue.
Conversations	Unified SMS/MMS/email inbox — triage, reply, assign, and escalate.
Email Inbox	Inbound patient email split into needs-response vs. answered, with AI draft replies.
Cases	Multi-channel tickets that link conversations, orders, faxes, and documents.
Episodes	Dated follow-up promises and open service episodes.

Schedule & outreach

Resupply calendar	Month and due-now views with patient order/eligibility review and individual or selected-batch email, SMS or automated calls.
Company Calendar	Shared team schedule of fittings, setups, follow-ups, and video visits.
Video Visits	Telehealth visits with secure join links sent by SMS/email.
Follow-ups	Today's callback queue across customers and patients.
Bulk Campaigns	Separate campaign sending requires its own permission; CSRs use the resupply calendar for selected due-patient outreach.
Alert Library / Reminders	One-off patient alerts and the resupply reminder schedule.
Playbooks / Canned Replies / Automated Messages	Reusable outreach cadences, saved reply snippets, and system-message copy.

Patients & paperwork

Supplies & next eligibility	See the exact supply, recorded order history, replacement-rule quantities, prescription expiry and scheduled due date together.
Patients	Patient roster and the 360° timeline (orders, messages, documents, therapy, billing).
Duplicate Review	Find and merge likely-duplicate patient records.
Documents & Packets	Draft CMNs/prescriptions/agreements and send e-signature packets.
Awaiting Signatures / Inbound Faxes	Track documents out for signature; triage inbound faxes.
Referral Reviewer / Sources	AI-extracted intake from faxed referrals; referring-physician scorecards.

Orders, catalog & leads

Pricing item review	Evaluate the patient's items, costs and insurance assumptions; save a within-policy review or request manager approval.
Patient batch review	Check approved pricing separately for up to 50 selected resupply drafts without creating orders or contacting patients.
Signed order safeguards	Preserve approved items and amounts; resolve prescription, inventory and changed-address holds before release.
Fit Requests	Finished fittings waiting for someone to verify the benefit and place the order.
Orders	Insurance orders — fulfill, track, and look up.
Shipping Labels	XPS labels for historical cash-pay shop orders. Current insurance resupplies use the PacWare and fulfillment workflow.
Catalog	The SKUs you dispense and what is on the shelf; stock moves as a reasoned movement, never a typed total.
Backorders	Mark a SKU out of stock and set the substitution the fulfillment path should use instead.
Insurance Leads	Work new benefit-verification requests from the storefront.
Fitter Invites & Prospects	Invite patients to the AI mask fitter and track the conversion funnel.

 RESPIRATORY THERAPIST (RT)

Respiratory Therapist

Watch therapy, keep patients adherent, and document the clinical care that backs every claim.

Therapy monitoring

RT Overview	At-a-glance therapy board with alerts, AHI, leak, and usage per patient.
Therapy Fleet	Population compliance cohorts and the clinical outreach worklist.
Setup Adherence	CMS 90-day adherence tracker for new Medicare setups.
Resupply Opportunities	Device-reported supplies due for replacement.
RT Outcomes	Per-therapist activity: encounters, patients, and interventions.

Clinical work

Clinical Encounters	Document and review patient clinical encounters (notes, assessment, plan).
Interventions	Non-adherence intervention worklist — cause, plan, and outcome tracking.
Mask-fit Feedback	Triage patients reporting a leaking or uncomfortable fit.
Clinical Outreach	Send supportive check-ins to patients with an open intervention (consent/DND-aware).
Adherence Coaching	Outreach plans for patients whose CPAP use is slipping.
Video Library	Manage the short-video education library shown on the storefront.

Providers, devices & reports

Providers	Central physician/NP registry (NPPES-backed).
Recalls	Manufacturer recall registry scanned against dispensed serials.
Asset Recovery	Recover machines from discontinued patients to refurbish and redeploy.
Therapy Report	Provider-ready, print-quality adherence snapshot by provider, patient, or device.
Patient clinical timeline	The clinical tab of the patient 360° view — therapy data, prescriptions, history.

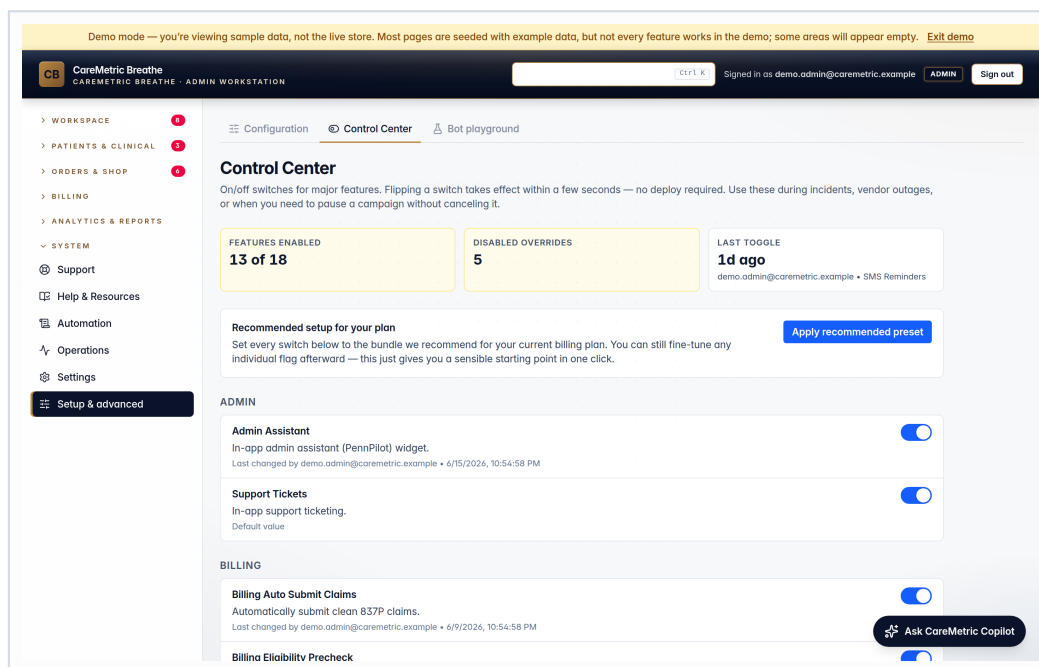
Part 2 — Comprehensive Feature Reference by Role

The full detail on every feature: what it's for and how it fits your day, grouped by role and by the area of the console where you find it.

ADMINISTRATOR (OWNER & ADMIN)

Administrator

Run the whole practice — setup, team, money, and the controls that govern every other role.



Control Center — the master on/off switches for major platform features.

Command center

Home dashboard. The landing page after sign-in. It rolls up today's work — conversations awaiting reply, overdue follow-ups, returns to action, appointments — alongside live KPI counters (active conversations, fulfillments this week, paused patients) and one-click links into each queue. Use it as your morning triage board; every tile deep-links to the full list.

CareMetric Copilot (admin assistant). A floating assistant on every admin page. It does two jobs: (1) tech support — it knows the entire console map and answers “how does X work / where is the page that does Y,” returning clickable links; and (2) product feedback — its one action, after you confirm, emails a structured feature idea to the owner(s). It never takes other actions and never echoes patient PHI. The Penn Home Medical Supply tenant brands it “PennPilot.”

Support. A simple form to file a support request, with the assistant on hand to answer how-to questions instantly before you escalate.

■ Setup & practice identity

Everything that makes the platform “yours.” Most of this lives under System → Settings.

Set Up Your Workspace. A guided checklist that walks a new practice through the core steps: brand and logo, custom domain, phone/SMS/fax numbers, the email From address, and payment processing. The Home dashboard shows a “finish setting up” banner until the essentials are done.

Company Information. Your legal practice name, addresses, NPI/tax identifiers, and contacts. These values are merged onto documents (CMNs, statements), the storefront, and outbound messages, so keep them accurate.

Storefront Branding. The patient-facing storefront's name, tagline, logo, and theme, plus custom-domain wiring (e.g. a tenant domain that routes only to your storefront). No logo yet? One click generates a **starter monogram** tile from your storefront name (entirely in the browser) so the storefront never shows a blank logo while you source artwork.

Phone & SMS, Fax, Email From Address. Messaging works from day one: until you provision your own number, patient texts, calls, and faxes go out on a **shared platform number**, and an inbound reply is routed back to the tenant that owns the patient — so two-way texting works across tenants before anyone has their own line. When you want messages to originate from **your practice's own number**, dedicated voice, SMS, and fax numbers are **provisioned for you through the platform's telephony carrier (Twilio/Telnyx)** — you don't bring or port your own line, and there's no separate carrier account to manage. Email uses the platform default From address until you set your own; deliverability then requires your sending domain to be SPF/DKIM-authenticated, and that status is shown here.

Locations. For multi-branch practices: define each service branch and assign patients to them. Appears only when multi-location is enabled in Control Center.

Account Security. Enroll and manage your own multi-factor authentication (authenticator-app TOTP). Strongly recommended for every admin.

■ Team & platform control

Owner-tier governance. Some items are restricted to the Owner role.

Team. Invite staff by email, assign a role — Owner, Admin, Customer Service Rep, Biller, or Respiratory Therapist — and manage access (resend invite, revoke, delete, change role). Invitees receive a sign-up link and must accept before they can log in. The last active Owner cannot be demoted (a safety lock). You can also link each teammate's **Slack handle** here, so Slack escalations and slash-command actions are attributed to the right person. Owner-only.

Control Center. The master on/off panel for major features: the voice agent, SMS/email campaigns, claim auto-submit, AI billing, the storefront chatbot, the admin assistant, multi-location, and more. Toggles take effect immediately.

Recommended preset. A one-click card in the Control Center that sets your feature flags to the recommended bundle for your billing plan (Virtual Mask Fitter, Launch, Growth, Scale, or Enterprise). New tenants already

land on their plan's preset at onboarding; this lets an existing tenant adopt — or re-baseline to — the recommended set after picking or switching a plan, instead of toggling dozens of flags by hand. It previews the changes first, and you can still fine-tune any individual flag afterward.

System Configuration. The Owner-only vault for integration credentials (therapy-cloud, clearinghouse) and platform secrets. Restricted to the Owner role so secrets can never be viewed or entered by a lower role.

Automation Rules & Rule Tester. Build “when X, do Y” automations — for example, when an inbound message matches a phrase, send a reply or raise a flag. The Rule Tester dry-runs a rule against sample input before you enable it in production.

Compliance Rules. Set per-payer CPAP adherence thresholds (minimum hours per night, minimum nights) that the therapy-monitoring boards measure patients against.

Bot Playground. A safe sandbox to rehearse the chat and voice bots against scripted situations with synthetic data and simulated tools — use it to tune prompts before changing what patients experience.

■ Operations & integrations

Keep the plumbing healthy and the partner connections flowing.

Operations. The health board for background jobs and pipelines — the in-process worker, the nightly therapy sync, reminder dispatch. Shows what ran, what's queued, and what failed.

Integrations. Connect and monitor therapy-cloud vendors (ResMed AirView, Philips Care Orchestrator, 3B Medical React Health). Shows availability and nightly-sync status; credentials are entered under System Configuration.

Slack team notifications. Bring the work into your team's Slack. With a Slack bot token + channel set in System Configuration, the platform posts real-time CS alerts — a patient reply that needs a human, a voice post-call hand-off, an SLA breach — plus the operator digests (owner weekly KPIs, metric alerts, stuck-job monitor, low-stock inventory) into your channel. Staff can act right from Slack: an **Escalate** button and a slash command, every inbound request verified against your Slack signing secret. Messages are deliberately **non-PHI** — a reference, a status, and a deep link back into the admin console — never message bodies, phone numbers, or clinical detail. Three Control Center toggles govern it (`slack.notifications`, `slack.interactivity`, `slack.digests`); all are inert until the Slack credentials are entered.

PacWare. A CSV import/export bridge to the PacWare desktop billing/warehouse system (which has no API). Import fills only blank fields on existing patients (never overwrites); exports (patient roster, resupply-due worklist) include a verify step and formula-injection guards.

Outbound Messages & Delivery Failures. Every outbound SMS and email with its delivery result (sent/delivered/failed/bounced). Delivery Failures collects bounces and shipping exceptions to retry or resolve.

Webhook Deliveries. Outbound event deliveries to partner endpoints, with the ability to re-queue failed or exhausted sends and test-send events.

■ Analytics, reports & goals

The numbers that run the business. Most are finance-gated.

Reports. A catalog of exportable reports (CSV/PDF/QuickBooks) — revenue summary, orders, refunds journal, patient payments, insurance claims, customer activity, and more.

Patient-Access Audit Trail. An admins-only report of who accessed which patient's information, and when. Filter by employee, by patient, and by time frame to answer a “who looked at this chart?” question. It is kept out of the CSR and clinician sidebars (it requires the audit-read permission) and the page itself enforces full-admin access.

Financial analytics. Use **Owner overview** for current business activity and the financial records attached to pricing reviews. Use **Reviews & actuals** for individual order reconciliation. Other specialist reports have their own source populations; do not combine legacy shop totals, claim paid-to-date figures and pricing actuals as though they were separate receipts.

Performance & Goals. Operational management: per-agent Team Throughput, real-time Live Staffing load, KPI Goals & Targets with pace-to-goal, and KPI Alerts that fire when a metric crosses a threshold.

Clinical & customer analytics. Resupply funnel and reorder-reminder conversion, post-delivery NPS with comments, and storefront traffic & revenue.

■ Pricing setup, costs and approval

Open **Orders & Leads - Pricing & Profitability**. Owners and Admins manage pricing; CSRs can evaluate and request review. A financial review never replaces insurance, prescription or clinical checks.

Supplier costs. Record the canonical catalog SKU, supplier SKU and **Goods cost per sellable unit (\$)** for one canonical selling unit, not the total purchase-pack price. Normalize the supplier paperwork before entry. Record units per supplier pack and the allowed quantity range; an order must use whole verified packs within that range. The separate New items comparison accepts whole-pack costs and can show surplus units. Choose stock or dropship fulfillment. Record evidence, verification status, effective date and expiry; a new version preserves history. Itemize freight, dropship, handling, packaging and other applicable charges, including whether each applies per unit, order or parcel. Mark amounts already included elsewhere so they are counted once. Blank means unknown; a verified zero means no charge. CSV imports expose per-row results: review errors and retry the affected rows rather than assuming the whole file succeeded.

Delivery evidence. Compare total delivered cost, not the supplier's goods price alone. Confirm destination coverage, service, availability, lead time, return terms and clinical suitability. Stock rate estimates require measured parcels and the configured warehouse shipping account; selecting a rate binds its service and saved patient address. The app's 30-minute validity is an estimate window, not a carrier price guarantee or label purchase. Dropship freight must come from verified supplier terms; a warehouse rate cannot establish a supplier's shipping charge.

Collection evidence. For insurance, select the patient and exact items/quantities and record dated, verified allowed and expected collectible amounts. Billed charges are separate. Optional insurer, secondary and patient shares plus the collection adjustment must reconcile to the expected total. The allowed amount already includes applicable shares; do not add it to those shares again. An inflated bill amount does not increase expected reimbursement or authorize a new patient charge. CSRs can model estimates, but management verifies the reusable evidence.

Pricing policy. Enter the business-approved target, **Approval floor (%)**, optional **Minimum profit per order (\$)** and profit basis. Contribution is before allocated overhead; the alternative basis includes the explicit

allocation. Add dated item, category or revenue-mode rules with priorities and review the effective rules. Optional unit-price increments, endings, ceilings and allocated item floors also apply. The app does not choose a default business margin. **Save draft policy** preserves a version; **Publish this policy** affects future reviews.

Required review is a separate choice. Publishing and enabling pricing does not by itself mean every patient order is mandatory-review. Inspect the **Patient order review** card. Enable the requirement deliberately only after catalog mappings, supplier evidence, collection assumptions and delivery choices are ready. When Required, new CSR orders need an approved insurance review. When Optional, existing unreviewed workflows remain available. **Pause pricing enforcement** is a management action; neither a simulation nor a supplier import silently changes enforcement.

Item review and its states. Choose exact SKU, quantity, billed unit amount, patient, revenue mode, supplier and delivery assumptions, then calculate. **Meets target** with complete verified evidence can **Save review** as approved. A permissible below-target result uses **Request manager approval**; missing or stale inputs require correction or a permitted documented estimate. **Blocked** includes hard-floor, minimum-dollar and other invalid conditions; an ordinary exception cannot override them. A changed input requires recalculation. Displayed percentages are rounded; approval uses the exact calculation.

Scope and quantity limits. Pricing uses USD cents. Internal scenarios support quantities up to 10,000; the patient signature workflow accepts at most 20 lines and quantities 1-99 per line. The stock-rate form has its own 99-unit limit. A larger internal scenario cannot simply attach to a patient order. These supply-sale calculations are not capped-rental or multi-period reimbursement models. Record actual method-specific processing fees where applicable; the practice's platform subscription fee does not establish a patient-order fee.

New items before catalog pricing. In **New items**, submit manufacturer/model/size, source, terms and quoted costs. Compare up to five suppliers for the same requested quantity and delivery scope. Whole purchase packs and minimums determine the cash outlay and surplus units; fractional unit rounding must not hide that outlay. Missing fee categories or evidence stay unknown, and expired offers remain expired. The comparison is provisional and does not publish a price. Management verifies the offer and links the proposal to the correct canonical SKU before a firm order review.

Reviews, approval and follow-up. In **Reviews & actuals**, filter pending approvals, open the exact revision, read evidence and expiry, and enter the approval reason. A permitted exception needs explicit acknowledgement. Approval applies only to that review; later order creation rechecks current dependencies. Use **Follow-up** for expiring costs, overruns and collection gaps, recording an owner, review date and resolution notes. Supplier or policy changes never rewrite the economics accepted on a bound order.

■ Bulk prices and future versions

Management reviews the full saved result before changing future pricing. A price-list version is not a patient communication or a purchase order.

Choose the portfolio. In **Bulk prices**, use **Find item or SKU**, **Category (exact)**, **Supplier**, revenue mode and published-status filters. Select complete published contexts, including every item in a bundle. A product without a usable context opens item review so quantity, delivery and revenue assumptions can be supplied. Filtered selection is bounded to 1,000 catalog items; narrow the filter if the screen reaches its limit. A list supports at most 100 complete scenarios, including retained contexts.

Compare a frozen preview. Refresh selected catalog assumptions, enter intended unit amounts or explicitly request target recommendations, then create a named preview. The old/new projected margins use the same current costs and delivery assumptions; only the previous published amounts are substituted. This is not a comparison against historical realized profit. Missing or ambiguous old-price comparisons are disclosed. Check the full SKU/quantity, supplier, destination, service and revenue basis; changing a filter afterward cannot alter a saved preview.

Keep unselected prices. A partial update retains other published contexts at their existing amounts. Selected and retained rows are shown separately, and both need current valid evidence. A retained row that fails review blocks the complete snapshot; its SKU/bundle and reason identify what to fix. Previewing an eligible subset can omit a new proposal, but cannot silently delete an already published price. If another publication adds a context or changes a retained amount, an older preview must be rebuilt.

Activate, schedule or return to a version. Review the frozen result, then activate it or choose **Activate later (your local time)**. The scheduler rechecks evidence and applies the full version together; expired or changed dependencies require a new review. Read any blocked schedule in history and follow-up instead of assuming the date changed prices. Re-activating an eligible prior version affects future reviews only and must pass current checks. It does not undo signed orders or recorded financial events.

Volume and discount comparisons. Volume inputs in a frozen preview are explicit expected order counts, not inferred demand. Combined margin is weighted by revenue, and incomplete assumptions prevent a combined result. For an internal self-pay item review, bounded discount analysis finds additional discount within the displayed search interval while preserving the target or floor. A partial search is not a global maximum; costs, quantities and fee assumptions stay fixed. Neither tool automatically applies a discount or charges a patient.

■ Owner models - choose the question first

Open **Owner models**, select a saved review, or choose **Use in owner models** after an item calculation. Each card calculates separately from its own explicit assumptions. These are internal planning tools, not patient checkout, order approval or an accounting income statement.

Pricing strategies. Use this when deciding the unit amount of one selected line in a self-pay simulation. Compare any supplied target contribution margin, markup on all variable costs, fixed whole-order contribution or reference price. The app does not fetch a competitor price. Margin divides contribution by revenue; markup divides it by costs. Other items and quantities stay fixed. The selected item's price grid and ceiling, every other item's ceiling, and the original policy's hard checks still apply. Insurance collections remain fixed, so raising a billed price does not solve a reimbursement shortfall.

Monthly break-even & profit. Enter monthly fixed costs, expected orders and target monthly profit. Every order repeats the source scenario. Break-even orders equal fixed costs divided by contribution per order, rounded upward; target orders include target profit in the numerator. Projected profit equals contribution times expected orders minus fixed costs. Illustrative arithmetic only: \$50 contribution, \$1,000 monthly fixed costs and \$500 target profit require 20 orders to break even and 30 for the target. These numbers are not business defaults. More orders with nonpositive contribution cannot fund a positive profit requirement.

Cost & collection stress. Enter up to 12 named percentage-change cases. Goods changes affect line acquisition unit costs; freight changes affect separately identified shipping/freight charges, counting an included parent once. Freight bundled into another cost needs separate evidence. Collection changes apply to expected insurance collections and remain hypothetical; exceeding recorded allowed amounts is blocked. Each case

shows recalculated contribution and its change from the baseline. It neither verifies a new cost nor changes contracted patient responsibility.

Price versus volume. For up to 12 cases, enter the selected line's unit amount, expected orders and fixed costs for the comparison period. Each order repeats the entire scenario. Projected revenue and contribution multiply that order result by volume; projected profit subtracts the period's fixed costs once. Price and demand are independent assumptions: the app does not predict that a higher or lower price will produce the entered volume. This price-changing model does not apply to fixed insurance collections.

Repeat orders & acquisition. Enter a finite horizon in months, acquired customers, total orders per customer over that whole horizon, acquisition cost per customer, retention cost per order and fixed costs for the horizon. Net per customer is contribution after retention times repeat orders, less acquisition cost. The payback count covers acquisition cost using contribution after retention; the screen compares it with the entered repeat count. Total orders equal customers times orders per customer; projected profit subtracts the horizon's fixed costs from total net per customer. It is not infinite lifetime value, a subscription, a retention forecast, or permission for future resupplies.

Working capital. Enter period days and orders, explicit cash outlay per order, inventory days, days to collect and days to pay the vendor. Funding-gap days are inventory plus collection days minus vendor-payment days, with a minimum of zero. Estimated funding equals cash outlay per order times period orders times gap days, divided by period days and rounded upward to cents. This estimate can use explicit cash assumptions even when profit evidence is incomplete. It does not include opening cash, irregular receipts, interest or other cash flows unless supplied; zero gap does not mean the business needs no cash.

Read model results without double counting. Blank inputs remain missing; explicit zero is accepted where valid. Monthly, price/volume and acquisition models use contribution before allocated overhead and subtract their entered fixed costs once. Do not also include those same expenses as variable costs. Read **More information needed, Review required, Not attainable with these assumptions, Not available for this scenario** and **Uses estimated inputs** with the source-policy result. A calculated loss or below-floor case is not an approved price.

Refresh and download model evidence. Every calculation rechecks its source. **Refresh catalog assumptions** can load current offer versions; it cannot renew expired manual or patient evidence. Starting a refresh clears prior results and export eligibility, and editing a card hides its old answer until recalculated.

Download scenario report includes calculated sections, entered assumptions, source items, selected line, policy/offer versions, deadlines and calculation time. Sections keep their own returned source details. Assumptions survive workspace tabs, but there is no durable saved model record: download a needed report before leaving.

■ Owner overview - recorded results

Open **Analytics & Reports - Owner overview**. This management-only page combines current business activity and tracked pricing-review financial records for your organization. It does not claim complete company profit.

Period and comparison. Choose 7, 30, 90 or 365 days, or **Custom dates** and **Apply dates**, up to 366 days. Dates are UTC; the selected end day is included, except today stops at report time. The previous period is the immediately preceding equal elapsed duration, including across daylight saving changes. Use **Refresh** and read the returned update time. Editing a date selection hides the old report until applied; exports are unavailable

during a refresh.

Activity, stages and priorities. Activity covers new patients, order creation and signatures, opened resupply episodes, fulfillment lines and units prepared, shipment evidence, distinct patients served, fitting requests and messages. Order/resupply/claim stages show the current status of records created in the selected period; later work can change that earlier cohort. Current queues include older open work, signatures, conversations and holds. They do not become historical balances when the date filter changes. Detail links open their own worklists without silently copying the overview's period.

Claims and payers. Period claim figures group claims created during that period and show billed amounts, current stages and payments recorded to date. They are neither cash received during the period nor expected collectible revenue. Current open claims and aging are separate backlog measures; aging uses submission date, or creation when not submitted. Payer detail shows up to ten entries while aggregate cards cover all matching claims. Reconcile payer receipts in the billing workflow rather than adding them again to pricing actuals.

Supply, stock and shipping. Due-now and next-30-day counts use distinct active patients with nonexpired open cycles and current valid active prescriptions. One patient can be in both groups; scheduling does not establish insurance eligibility or patient consent. Top products use recorded shipped units, not prepared units or product profit. Shipment evidence is a recorded ship/delivery date, not proof of receipt; assumed-shipped episodes remain separate. Returning patients also have earlier shipment evidence. Null stock is untracked; low stock uses the configured threshold or five when unset, preserving explicit zero. The low-stock preview shows ten while totals are exact.

Outreach and service. Messages are counted by creation in the selected period with their current delivery status, across inbound and outbound channels. Accepted or sent does not prove delivered; a reply count is not a conversion rate. Review open conversations, follow-ups, signature and address holds in their detailed queues. Prepared supply lines, shipped lines, distinct patients and signed orders count different things and should not be added together as sales.

Period financial activity versus lifetime contribution. Period revenue and cost use each pricing event's economic occurrence date. Refunds reduce revenue; cost credits reduce cost. Revenue and related costs may fall in different periods, so subtracting these period cards does not establish period profit. **Completed-review contribution** is a separate lifetime total: recorded net revenue less recorded net costs for bound reviews with both completeness decisions closed. Multiple lines or events do not count an order twice. Zero and negative results remain visible.

Coverage and date quality. Missing, invalid or future economic dates are disclosed and excluded from period activity; the entry timestamp is never used as a substitute. A review with uncertain dates is also excluded from completed financial totals even when both completeness flags are set. Incomplete-cost, incomplete-collection and uncertain-date categories can overlap; the combined incomplete count counts a review once. Reopening completeness removes it from completed totals. An event never entered cannot be discovered by the report.

CSV and unavailable sources. **Download overview CSV** exports the returned snapshot, exact windows, units and definitions. Negative refunds, credits and losses remain numeric USD amounts for spreadsheet sums; free text is protected against spreadsheet formulas. Business and finance load independently: an unavailable section is labeled, not replaced with zero, including in the CSV. Retry if both fail. A real zero only means no matching recorded activity. Neither this export nor a planning report is a complete bank, remittance or accounting ledger.

■ Reconcile accepted orders and resolve delivery changes

Use **Reviews & actuals** for the bound order review. Private supplier costs and approval notes stay out of patient signature documents.

Record the economic event. Enter the actual source, effect, USD amount, source document reference, unique economic event reference and optional item allocation. Use **Economic occurrence date and time (UTC)** from invoice/payment evidence, then **Record actual event**. Historical dates are supported; blank, impossible or future manual dates are rejected. The form starts at the current minute, which must be changed for older evidence. Cost, revenue, refund and cost-credit entries are separate documented events, not edits to the original quote.

Retry without duplicating money. After a lost response, an unchanged **Retry** resends the exact body, including occurrence time and references. Do not invent another event identifier just to clear an error. Editing a submitted event deliberately creates a changed request; a conflicting reference prompts **Refresh actual event history** so you can check whether the first save committed. Record a correction only after review. A completed save clears the event-specific draft and initializes the next occurrence time; later replies cannot erase a newer edited draft.

Close costs and collections separately. History pages contain 100 events, but totals include all recorded events. Compare the immutable original quote, current forecast and actuals as separate measures. Mark costs and collections complete independently with supporting reason. New actual events reopen completeness; stale revision conflicts require reviewing current history. Completed contribution uses total contribution divided by corresponding revenue for weighted margin, not an average of percentages. Incomplete evidence is always disclosed.

Signed order and delivery holds. Binding uses the approved patient, exact items, amounts, supplier versions and deadline. Signing queues the exact lines together once; stock movements and dropship ownership remain distinct. Missing active prescriptions, inventory bookkeeping or changed-address conditions hold release. A resend cannot extend the pricing deadline; expired requests need a fresh review/request. Cancellation remains possible. For a changed destination on a signed order, a manager reviews the original amounts and fixed collectible revenue against current address, costs and fresh freight evidence, records a reason and approves the exact revision. Clearing an address alert alone does not release priced work.

Release and export. Correct prerequisites and retry the signed order's fulfillment action. A delivery re-review may acknowledge a permissible exception but cannot bypass a hard floor or silently increase accepted prices. Already shipped records remain historical evidence. PacWare preview/export uses signed SKU quantities and withholds priced lines on hold or already submitted; read withheld counts and the disclosed export limit before a manual handoff. Export does not buy a label, place a supplier order, or prove shipment.

BILLER

Biller

Own the revenue cycle — eligibility, claims, A/R, and getting every dollar collected.

Demo mode — you're viewing sample data, not the live store. Most pages are seeded with example data, but not every feature works in the demo; some areas will appear empty. [Exit demo](#)

CareMetric Breathe
ADMIN WORKSTATION

Signed in as demo.admin@caremetric.example ADMIN Sign out

WORKSPACE
PATIENTS & CLINICAL
ORDERS & SHOP
BILLING
Dashboards
Worklists
A/R & collections
Tools
ANALYTICS & REPORTS
SYSTEM

Verify insurance Insurance discovery **Eligibility** Re-verification Prior auths CMN / DIF worklist Bill hold
ADR / audit response Audit readiness Collections Auto-submit AI queue Denials worklist

Eligibility worklist

Every 270/271 round-trip in the selected window. Rejected and transport-failed checks are where the team's time pays off — those are coverage rows about to bite us.

Run a verification

Filters

Window: 7d 30d 90d Status: All

TOTAL IN WINDOW: 4 ACTIVE COVERAGE: 1 INACTIVE / REJECTED: 2 PRIOR-AUTH REQUIRED: 1

Recent eligibility checks
Newest first — 4 rows

STATUS	PAYER	HCPCS	ACTIVE / IN-NETWORK	DEDUCTIBLE	OOP MAX	COINSURANCE	PA?	REQUESTED	PATIENT
PARSED	Sample Medicare	E0601	active - in-network	\$250.00 met \$185.00	\$3,000.00 met \$450.00	20% copay \$0.00	NO	6/22/2026, 7:56:09 PM	Open
PARSED	Acme Health Demo	A7030	inactive	—	—	—	—	6/22/2026, 10:06:00 PM	Open

Ask CareMetric Copilot

The Eligibility worklist — system-wide 270/271 status, worst coverage first.

How the billing engine works, end to end

This is what makes CareMetric Breathe's revenue cycle different: the work happens ahead of you. The sections below explain the machinery; the worklists that follow are where you drive it.

The claim types it handles. Every bill is an insurance claim with a payer sequence. **Primary** claims are built automatically from a fulfillment (the shipped order) or keyed by hand. **Secondary / COB** claims are coordination-of-benefits bills the system can auto-draft once a primary pays and leaves a balance, snapshotting the primary's paid / contractual / patient-responsibility amounts into the 837P COB loop — they always land in *draft* for a biller to review, never auto-submit. **Capped-rental** claims are generated month by month for Medicare CPAP rentals, each seeded with the correct rental-month modifier. **Corrected / void-replacement / paper-backup** claims are keyed from the Manual Claim screen when an automated path doesn't fit.

Modifiers, applied by rule. Modifiers (RR rental, KH/KI/KJ capped-rental months, KX medical-necessity, NU purchase, GA/GZ ABN, LT/RT) are applied automatically from a per-payer rule store. Each rule is keyed to a payer + HCPCS + a condition — “rental month ≤ 3”, “rental month ≥ 4”, “compliant at 90 days”, “initial dispense”, “prior auth approved”, “ABN on file” — and the highest-priority match wins as the claim is built. Capped rentals rotate the month modifier for you: KH for months 1–3, KI from month 4, plus KX once the patient proves CMS compliance (≥4 hours on ≥70% of nights in a 30-day window). Hand-keyed claims pull the same modifiers so manual and automated claims bill identically.

Eligibility, five ways. The platform runs the 270/271 eligibility transaction for you in five places: (1) a real-time check that returns a 271 in seconds (or a deferred SFTP round-trip when real-time isn't configured); (2) a **quick check** you can run with no patient record at all — type a name, DOB, and member ID and read coverage back instantly; (3) the system-wide **Eligibility worklist** that surfaces rejected and inactive coverage first; (4) scheduled **auto-re-verification** that fires fresh 270s for the most-urgent active coverages on a cadence; and (5) an automatic **pre-submit precheck** that re-confirms coverage in the moments before a claim is transmitted and holds back anything inactive or needing prior auth.

One-click insurance verification, everywhere. A Verify-Insurance button rides on the patient chart, the patient's billing tab, and the eligibility worklist, plus a standalone cross-patient verifier — so anyone can confirm coverage in one click from wherever they already are, and the 271 result shows inline. No separate payer portal, no phone tree.

Insurance discovery. When a patient's plan is unknown — or a coverage on file came back inactive — Insurance Discovery searches the payer network from their demographics (name, DOB, ZIP, optional SSN/member-id) and returns the active coverages it finds, ready to attach. It turns “we don't know who covers them” into a billable claim. (A paid clearinghouse add-on; enable per tenant.)

Auto-submit to the clearinghouse. Submission-ready claims flow through a pipeline: a **preflight scrub** (required fields, HCPCS/diagnosis match) drops unsalvageable claims; an **eligibility gate** blocks stale or inactive coverage; the optional **AI scrubber** flags or fixes the rest; clean claims are **batched per payer** into 837P files and transmitted to Office Ally over SFTP, with control numbers tracked and 999 / 277CA acknowledgements reconciled automatically. You can drive it two ways — review and approve a staged batch yourself (Auto-submit), or let the scheduled cron transmit clean, eligible claims unattended. A **bill-hold gate** keeps any claim from going out while required signed paperwork is still outstanding, and lifts the hold the moment the last document is back.

AI claim scrubbing & denial recovery. Before submission the AI scrubber reviews each draft for semantic problems — wrong modifier for the rental month, quantity over the LCD limit, HCPCS/diagnosis mismatch, fee-schedule drift, duplicate risk — and returns a verdict (ready / fixable / blocking) with one-click patches. After a denial, the AI denial analyzer reads the CARC/RARC codes, explains the root cause, drafts the corrective steps and an appeal letter, and recommends the next move (resubmit, appeal, bill the patient, write off). When the fix is safe and confidence is high, it offers a one-click auto-resubmit. The **AI Queue** buckets all of this — blocking, fixable, needs-analysis, and auto-resubmit-ready — so a biller always works the highest-payoff claim first. PHI is minimized before anything reaches the model (names → initials, DOB → year, member IDs fingerprinted).

ERA auto-posting. Upload an 835 remittance and the system matches each claim block to your claims and posts the adjudication automatically — allowed, paid, and patient-responsibility amounts at both the claim and line level, a posted event with the check/EFT reference, and a status flip to paid or denied. Denied lines are handed straight to the denial analyzer. Re-posting the same file is a safe no-op (idempotent), so payer redeliveries never double-count.

■ Billing dashboards

Read-first views that tell you where the money is. Open these to know what to work next.

Billing Hub. The A/R director's home base: money in flight, top payers, aging summary, and the day's billing KPIs. Start here, then drop into a worklist.

Denials & DSO. Benchmark view of the trailing 90-day denial rate and trailing 180-day days-to-pay (DSO) per payer, with trend lines so you can see which payers are slipping.

Collections Forecast. Projected cash from claims already in flight, bucketed by the date you expect each to land — a near-term cash-flow forecast.

Chargeback Disputes. Card chargebacks raised against storefront charges, ordered by the evidence-submission deadline so nothing lapses.

Payer Profitability. Net yield per payer end to end: billed → allowed → collected, denial rate, and the result net of product cost. Finance-gated.

■ Claim worklists — the lifecycle

Worklists are ordered the way a claim moves: verify coverage, secure authorization, gather paperwork, submit, then fix and appeal.

Verify Insurance. Run a one-off 270/271 eligibility transaction for any patient on demand — a quick coverage check that doesn't need an existing patient record.

Insurance Discovery. When you don't know a patient's plan, search the payer network from their demographics to find active coverage.

Eligibility. The system-wide 270/271 worklist. Rejected and inactive coverage rises to the top so you fix the highest-risk claims first.

Re-verification. Active coverages due for a fresh check — never-checked, terminating soon, or stale beyond your interval.

Prior Auths. Authorizations missed or at risk of breaching SLA, auths expiring soon, and drafts ready to submit. Supports electronic Da Vinci PAS submission where the payer accepts it.

CMN / DIF Worklist. Certificates of Medical Necessity / DME Information Forms still awaiting completion before a claim can go out.

Bill Hold. Claims deliberately held from billing until their signed paperwork is back — work this list to release them once documentation lands.

Auto-submit. Claims that are preflight-clean and backed by active eligibility, ready to transmit. Approve a batch yourself or let the scheduled cron send them (the cron only fires when both the schedule and the Control Center flag are on).

AI Queue. Claims the scrubber blocked plus a denial-analyzer worklist; the assistant suggests the code or edit and, where safe, an auto-resubmit.

Denials Worklist. Open denials ranked by recoverable dollars × win-probability so you spend your time where it pays off most.

■ A/R & collections

Work claims and balances to get paid.

A/R Aging. Open claims bucketed 0/30/60/90+ days and broken out by payer — the classic aging worklist.

Filing Deadlines. Open claims ranked by days remaining before the payer's timely-filing window closes, so you never lose a claim to the clock.

Secondary Claims. Coordination of benefits: roll the primary payer's leftover balance onto the secondary payer's claim.

Statement Send. Send patient-responsibility statements by email or SMS. Consent- and quiet-hours-aware so you stay compliant.

Collections. Patient balances that have gone unpaid move automatically up a dunning ladder — statement (day 0) → reminder (day 7) → second notice (day 21) → final notice (day 35) → collection agency (day 60). The ladder **auto-pauses the instant a balance is paid or put on a payment plan**, so you never dun a patient who has already settled up. Batch-print the final-notice letters as one PDF, and export the agency-eligible accounts when you place them with a collections partner. (The dunning ladder is gated behind the **collections.dunning** flag and agency export behind **collections.agency_export** in the Control Center.)

Capped Rentals. Track CMS 13- and 36-month capped-rental cycles and rotate the KH/KI/KX modifiers automatically as each cycle advances.

ADR / Audit Response. When a payer or contractor sends an Additional Documentation Request, every open ADR lands here ranked by response deadline, with a per-request checklist of what to gather and a record of the win/loss outcome so you can see how the practice fares on audits. Gated behind the **billing.adr_queue** flag.

Audit Packet. Assemble a complete, audit-ready CPAP/PAP documentation packet — the Standard Written Order, CMN, sleep study, compliance summary, proof of delivery, and supporting notes — into a single PDF you can send to the payer in one click, instead of hunting the chart for each page.

■ Tools & configuration

The clearinghouse, payment posting, manual entry, and the payer rules behind it all.

ERA Files. Upload an 835 remittance and the system auto-posts the payer's adjudications against the matching claims, flagging denials for the worklist.

Office Ally. Your clearinghouse cockpit: 837P submissions, acknowledgements (999/277CA), and live transmission status. (Office Ally has a stub mode for preview environments.)

Manual Claim. Key a corrected, void/replacement, or paper-backup claim by hand when an automated path doesn't fit.

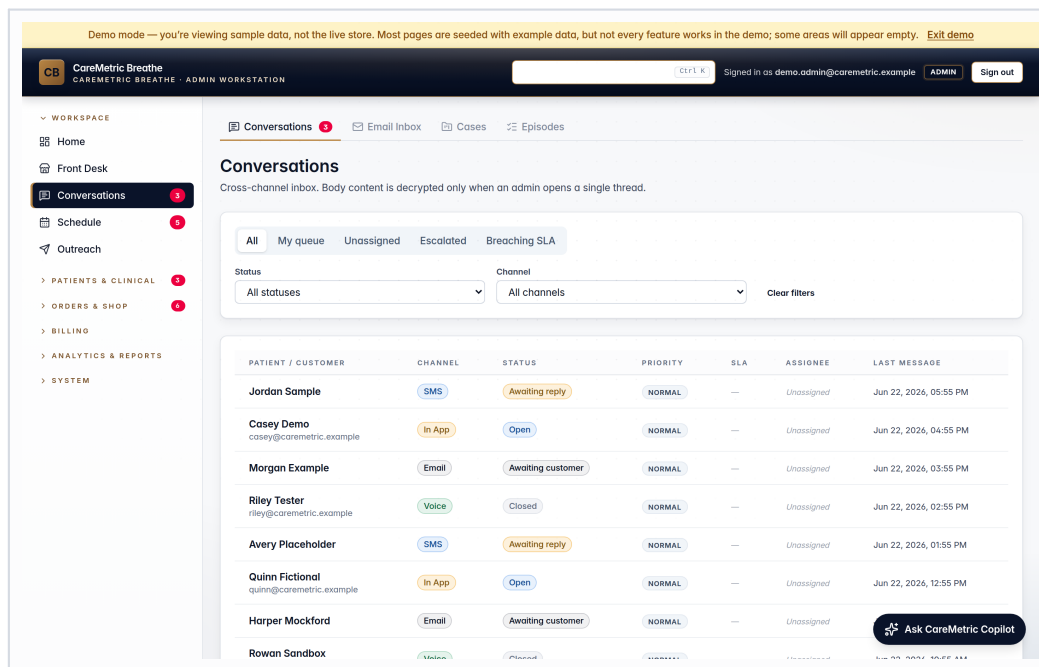
Billing Config. The rule store the whole engine reads: payer profiles, fee schedules (with CMS import), modifier rules, denial-code mappings, claim templates, and HCPCS coverage diagnoses.

Package & Usage. Your CareMetric Breathe subscription plan, optional add-ons, and the current month's usage — useful when planning spend.

CUSTOMER SERVICE REP (CSR)

Customer Service Rep

The patient's first point of contact — messages, orders, scheduling, and day-to-day service.



Conversations — the unified SMS/MMS/email inbox a CSR works all day.

■ Resupply: history, eligibility and outreach

Use the patient record and resupply calendar together: recorded history explains the replacement-rule estimate, while the patient's response starts the next review.

Patient supplies and history. Open a patient record's resupply view or **Orders & eligibility** from the calendar. **Supplies & next eligibility** lists supply name/SKU, HCPCS mapping, last ordered date, prescription expiry, replacement-rule result, quantity available and scheduled resupply. **Order history** shows recorded supply lines newest first, including quantity, status and shipment/delivery evidence; use **Older** and **Newer** for 25-line pages. **CSR orders & signatures** separately shows links from the latest 50 resupply drafts. A signed request can also appear later in fulfillment history; these are not two separate dispenses.

Eligibility is not a shipment authorization. Read the reason beside **Eligible by replacement rule**, **Quantity limit reached** or the interval-opening date. Related mapped supplies and recorded quantities affect the available allowance, so do not treat another SKU name as a new allowance. **Needs eligibility review** means the app cannot establish the rule, including missing HCPCS mapping; it does not mean eligible. A scheduled due date is distinct from interval and quantity eligibility. Verify active insurance, prescription validity, actual patient need and required paperwork before fulfillment.

Calendar and overdue work. Open **Workspace - Schedule - Resupply calendar**. Use the month view, previous/next month and a day, or **Due now & overdue**. Search patients or supplies; each patient row groups its due supplies and shows contact details/preferences. Dates follow the practice display timezone, including month boundaries. Future scheduled patients may be reviewed but are not yet ready for outreach. Search filters the displayed patients; review the selected date scope before interpreting the count as a worklist.

Individual and selected-batch outreach. From a due patient's supply review, or after selecting calendar patients, choose **Email**, **SMS** or **Automated call**. **Select visible patients (up to 50)** is a bounded selection, not every patient in the database. Read the confirmation's patient names and chosen channel, then **Queue outreach**. It asks whether supplies are still used and a refill is wanted; it does not place an order. Automated call is the configured voice-agent outreach, not a manual dialer or proof that a person answered.

Read the per-patient result. The result shows queued, skipped and failed patients with reasons. Contact details, channel consent, opt-outs, recent contact, active cycles and local SMS/call contact hours are checked. Duplicate requests and multiple due items do not authorize repeated cross-channel contact. Queued means accepted for processing; the worker may still defer or skip it. Review delivery and replies in **Conversations**. Correct a stated missing detail or wait for the allowed contact time; do not switch channels repeatedly to evade a skip or ambiguous provider response.

Confirmation and fulfillment stay separate. The configured reminder channels support patient replies and signed response links. A response that supplies are wanted proceeds to order review; it is not proof of delivery and does not bypass price, prescription or address holds. The recent-contact guard generally avoids another request within 48 hours. Existing automation depends on the tenant's enabled features, channel setup and valid rules. No patient subscription purchase or unconditional auto-ship is created by these tools.

■ Pricing review in the CSR workflow

CSRs can review relevant item costs and margins, submit new-item information and request approval. Supplier verification, business rules, owner models, actual-event writes and publishing remain management actions.

Review the exact patient order. Open **Orders & Leads - Pricing & Profitability - Item review**, or the embedded review on an order/resupply draft. Confirm patient, canonical SKU, size/model, quantity, supplier, stock/dropship method, destination and service. Choose the matching verified collection evidence when available; your own estimate remains an estimate. Insurance billed amounts and expected collectible revenue are different. Internal self-pay mode is a simulation and cannot attach to an insurance patient order.

Save or ask for help. Calculate after every change. A complete within-policy result can **Save review** as approved without a separate manager action. Use **Request manager approval** for a permitted exception, or **Save draft review** while correcting missing evidence. A hard floor cannot be overridden by an ordinary approval. Select the approved review for the exact patient/items and attach it to the order. Changing lines or quantities needs another matching review. When reviews are Required, an unreviewed order is blocked; when Optional, legacy ordering does not imply financial approval.

Check resupply drafts in a batch. Open **Patient batch review**, select up to 50 open drafts and choose **Evaluate selected drafts**. Each patient's newest matching approval is checked independently for items, quantities, collection and delivery assumptions. Results identify ready, stale, missing or unavailable reviews. If the newest matching review is stale, open the patient individually to choose another valid one. This check sends no messages and creates no orders. The screen exposes up to 200 open drafts; work additional groups deliberately.

Signed lines and holds. An approved review binds to one order and its signature deadline. Resending cannot extend expired pricing evidence; create a fresh review/request when expired. Signing queues the exact lines together; required exact-SKU prescriptions and other holds must be resolved before release. Stocked items decrement tracked inventory once; supplier dropships do not decrement owned inventory. After an address change, ask a manager for delivery re-review at the original accepted amounts. Clearing a general address flag alone is insufficient. After prerequisites are corrected, retry the signed order's fulfillment action.

■ Paperless paperwork — e-signature, nothing printed

Everything a patient or provider needs to sign is signed on a screen. No printing, no scanning, no faxing, no lost forms, no delays.

Sign on a phone, file automatically. Staff stage a document or a packet; the patient gets a link and e-signs on their own device with a typed name and explicit ESIGN consent — image-free and ESIGN-Act compliant. The signed PDF files itself to the chart the moment it's done, and any related **bill hold** lifts automatically so the claim can go out. Nothing is printed, nothing gets lost, and nothing waits on the mail.

Included document templates. Ready-to-send templates ship with the system:

- Standard Written Order (SWO) — PAP device & supplies
- PAP Certificate of Medical Necessity (CMN)
- Structured CMN forms — CMS-484 (oxygen), CMS-846 (compression), CMS-848 (TENS)
- DWO / CMN renewal forms by HCPCS family (PAP, RAD, oxygen, ...)
- ABN (CMS-R-131) with Options 1–3
- Assignment of Benefits & financial responsibility
- DMEPOS Supplier Standards notice
- Proof of Delivery
- Refill Confirmation
- New-patient setup packet (ABN + Supplier Standards + AOB, sent as one)

Plus free-form manual documents (prescription, agreement, delivery ticket, cover letter) and documentation packets (prior-auth support, appeal support, accreditation audit, medical-records request).

■ The provider portal — providers e-sign and see their patients

A secure portal that ends the fax-and-chase cycle with referring physicians.

Providers e-sign — no more faxing. Invite a referring provider (by NPI, verified against NPPES) into a secure, MFA-protected portal. Stage their CMNs, DWOs, prescription packets, and claims for signature, and the provider signs on their own device — instead of receiving a fax, printing it, signing, and faxing it back (and you chasing what never comes). Every signature is captured in a tamper-evident, hash-chained audit trail with a printable, ESIGN-compliant certificate you can hand a payer.

Providers see their patients. In the same portal a provider can view their patients' active orders and prescriptions and read their therapy data and reports (read-only) — so they can confirm adherence and close the loop without a phone call to your office.

Lifecycle & security. Each document moves pending → signed → ready-to-print / returned-signed / attached-to-chart / released, so staff always know where it stands. Multi-factor authentication is mandatory for every provider, and the portal is off by default — an Owner enables it per tenant in the Control Center.

■ Your daily workspace

Where a CSR lives all day — the inbox and the work queues.

Home. The shared worklist and live counters. As a CSR you'll watch conversations awaiting reply, overdue follow-ups, and returns to action.

Conversations. The unified inbox for inbound SMS, MMS, and email across every patient. Triage threads, claim one for yourself, reply (optionally with a canned reply), tag, snooze, and escalate to a case. Smart routing and required-skills keep threads with the right people.

Email Inbox. Inbound patient email split into “needs response” and “already answered.” When the AI email auto-reply feature is on, high-confidence answers can be drafted or sent automatically; anything account-, order-, or clinically-specific falls through to a human.

Cases. Multi-channel tickets that bind together the conversations, orders, faxes, and documents belonging to one issue, so a complex problem stays tracked end to end.

Episodes. Dated service promises — “we'll call Tuesday,” “awaiting the sleep study” — that keep open commitments from slipping.

■ Schedule & outreach

Plan the day and reach patients at scale or one at a time.

Company Calendar. The shared team schedule of patient appointments — fittings, equipment setups, follow-ups, and video visits — with office hours and closures respected.

Video Visits. Run telehealth visits for setups and mask troubleshooting; the system generates a secure join link and sends it by SMS/email.

Follow-ups. Today's callback queue across customers and patients, with overdue items surfaced so nothing is forgotten.

Bulk Campaigns. A separate management-authorized campaign workflow: filter the audience, check recipients and review the message before sending. Standard CSR resupply outreach uses the calendar and conversation permission; it does not grant campaign-send authority.

Alert Library & Reminders. Send a curated one-off alert (SMS/email/call) to an individual patient, and manage the automated resupply reminder schedule.

Playbooks, Canned Replies & Automated Messages. Reusable situation-based outreach cadences, saved snippets you drop into manual replies, and the editable copy behind system-sent messages (order confirmations, tracking).

■ Patients & paperwork

The patient record and the documents that surround care.

Patients. Search the roster and open a record for the 360° timeline — every order, message, document, therapy reading, and billing event in one place. Edit demographics and notes.

Duplicate Review. Find likely-duplicate patient records and merge them so a patient's history isn't split across two charts.

Documents & Document Packets. Draft a CMN, prescription, agreement, or fax cover by hand, then send and track an e-signature packet for new-patient onboarding.

Awaiting Signatures & Inbound Faxes. Track documents out for provider signature and scan returned faxes to file them; triage the inbound-fax queue (sleep studies, Rx renewals, chart notes).

Referral Reviewer & Sources. Review AI-extracted intake from faxed/uploaded referral packets and accept them into a new patient record; the Sources scorecard ranks referring physicians by volume and revenue.

■ Orders, catalog & leads

Fulfillment, stock, and the acquisition funnel.

Fit Requests. The queue a finished mask fitting lands in. The patient never files their own insurance order — they send their details or ask to be called, and a CSR verifies the benefit and places the order. Closing a row records what actually happened; only **Fulfilled** — the patient has the mask — marks the fitting as dispensed for the outcomes dashboard.

Orders. Work insurance orders — fulfill, look up, and track — from a single queue.

Shipping Labels. This XPS label worklist serves historical cash-pay shop orders. Current insurance resupplies use the PacWare and fulfillment workflow. A shipping estimate in Item review is not a purchased label, a supplier order or proof of shipment.

Catalog. The SKU registry records what you dispense and what is on the shelf. Stock moves through recorded movements with reasons, so every change has history. Blank stock means **untracked**, not zero. Low stock uses a configured threshold, or five units when unset; an explicit zero is preserved. Priced signed orders require canonical item mappings and valid prescriptions. A queued stock dispense is not shipment evidence, and supplier dropship fulfillment does not consume owned stock.

Backorders. Mark a SKU out of stock and set the substitution rules that apply while it is. This is read by the insurance fulfillment path, not a retail shelf notice, so an uncleared backorder keeps routing patients away from that SKU. Recording a receipt on Catalog clears it automatically.

Insurance Leads. Work new benefit-verification requests that came in from the storefront.

Fitter Invites & Prospects. Invite a patient to the AI mask fitter and review the returned mask & size recommendation; the Prospects view tracks the fitter conversion funnel.

RESPIRATORY THERAPIST (RT)

Respiratory Therapist

Watch therapy, keep patients adherent, and document the clinical care that backs every claim.

Demo mode — you're viewing sample data, not the live store. Most pages are seeded with example data, but not every feature works in the demo; some areas will appear empty. [Exit demo](#)

CareMetric Breathe
CAREMETRIC BREATHE · ADMIN WORKSTATION

RT Overview Therapy Fleet Setup Adherence Resupply Opportunities RT outcomes

RT overview

Daily clinical board for tracked patients across ResMed AirView, Philips Care Orchestrator, and React Health. Alerting rows sort to the top.

7 days 14 days 30 days 90 days [Export CSV](#) [Refresh](#)

ACTIVE
4
 with x1 night in last 7d

ALERTING
2
 undismitted smart-trigger events

STALE
1
 linked but no recent night

Patients

4 tracked · window 7 days

Q Search name or pacware id... Alerting only Stale only

SOURCE: philips_care_orchestrator react_health resmed_airview

PATIENT	STATUS	NIGHTS	LAST	AHI	LEAK	USE (H)	SOURCES
Sample, Avery PW-10004	⚠ High leak	6	2026-06-21	6.2	38.0	3.6	resmed_airview
Patient, Demo PW-13007	⚠ High AHI	5	2026-06-21	12.6	19.0	3.1	philips_care_orchestrator
Fixture, Jordan PW-13008	⚠ Stale 21d	0	2026-06-01	—	—	—	react_health
Mockton, Quinn PW-12005	✅ OK	7	2026-06-21	3.0	11.0	6.5	resmed_c

[Ask CareMetric Copilot](#)

RT Overview — the therapy board with per-patient AHI, leak, and usage alerts.

Therapy data from every cloud, one login

Manage every patient from one screen no matter whose machine they sleep on — no juggling three vendor portals.

Three manufacturers, one screen. A nightly sync pulls device data from **ResMed AirView**, **Philips Respironics Care Orchestrator**, and **3B Medical React Health** (Luna / iCode). Whatever brand a patient uses, their therapy lands in the same boards and the same patient chart — so an RT works one worklist, not three logins.

What it pulls. For each linked patient the sync brings back device settings (model, serial, therapy mode, pressure min/max, ramp, humidifier, mask type); a compliance summary (days with data, days ≥ 4 hours, average usage and AHI, and the CMS 90/30 flag); recent nights (usage minutes, AHI, leak rate, P95 pressure); and supply items with last-replaced and next-eligible dates that drive resupply timing.

How the sync works. An automated nightly job walks every active therapy link (least-recently-synced first, rate-limited so it's gentle on the vendor APIs), normalizes each vendor's quirks into one common shape, and stores a snapshot. The RT boards read that cache instantly; you can also force a manual refresh for one patient or one source from the patient chart.

■ Alerts, and how compliance is ensured

The platform watches therapy for you and surfaces the patients who need a human.

Clinical alerts to the RT. The Therapy Fleet worklist ranks patients by reason so you work the highest risk first: **setup-adherence risk**, **no recent data**, **high AHI**, **high leak**, and **usage decline**. Mask-fit feedback and open interventions feed the same queue, and clinical outreach is frequency-capped (a minimum gap between touches) so patients aren't over-contacted.

Ensuring CMS 90-day compliance. Setup Adherence tracks every new PAP patient's first 90 days against the Medicare standard — ≥ 4 hours on ≥ 21 nights within any rolling 30-day window — and classifies each as qualified, on-track, or at-risk, with the best rolling count, nights still needed, and days remaining. At-risk patients surface early so you can coach them before the window closes and the rental fails to convert.

Alerts and messages to the customer. The Alert Library sends curated one-off email, SMS, or voice alerts to a patient (with safe variable substitution), and resupply reminders nudge them on cadence. Optional enforcement can hold a too-soon or coverage-blocked reorder and route it to a CSR as an alert instead of letting it ship.

■ Therapy monitoring

Population boards that surface who needs attention, fed by the therapy-cloud integrations.

RT Overview. The at-a-glance therapy board: per-patient alerts with AHI, mask leak, and usage metrics, so you can spot a struggling patient fast.

Therapy Fleet. Population compliance cohorts and the clinical outreach worklist — slice the patient base by how they're doing and act on the cohort that needs you.

Setup Adherence. The CMS 90-day adherence tracker for new Medicare setups (the ≥ 4 hours on $\geq 70\%$ of nights compliance window), so a setup never quietly fails its window.

Resupply Opportunities. Device-reported supplies that are due for replacement, which feed resupply orders.

RT Outcomes. Per-therapist activity — encounters logged, patients touched, interventions opened — for visibility into clinical workload and impact.

■ Clinical work

Document care and drive non-adherent patients back on track.

Clinical Encounters. The clinical documentation store: record an encounter with reason, assessment, intervention, and plan, or just a free-text note. The patient's clinical timeline is built from these.

Interventions. A structured worklist for non-adherent patients — capture the cause, the plan, and the outcome, and track it to resolution.

Mask-fit Feedback. Triage patients who report a leaking or uncomfortable fit and route them to a follow-up or refit.

Clinical Outreach. Send supportive check-ins to patients with an open intervention. Consent- and do-not-disturb-aware so outreach stays appropriate.

Adherence Coaching. Build outreach plans for patients whose CPAP use is slipping, before they fall out of compliance.

Video Library. Curate the short education videos shown to patients on the storefront's learn pages.

■ Providers, devices & reporting

The registry, equipment safety, and provider-ready output.

Providers. The central physician / nurse-practitioner registry, backed by NPPES lookups, used across prescriptions and referrals.

Recalls. The manufacturer recall registry, scanned against your dispensed device serials to flag at-risk patients.

Asset Recovery. Recover machines from discontinued patients so they can be refurbished and redeployed.

Therapy Report. A provider-ready, print-quality adherence snapshot you can generate by provider, patient, or device manufacturer — ideal for closing the loop with referring physicians.

Patient clinical timeline. Inside the patient 360° view, the clinical tab gathers therapy readings, prescriptions, encounters, and resupply history in one chronological story.

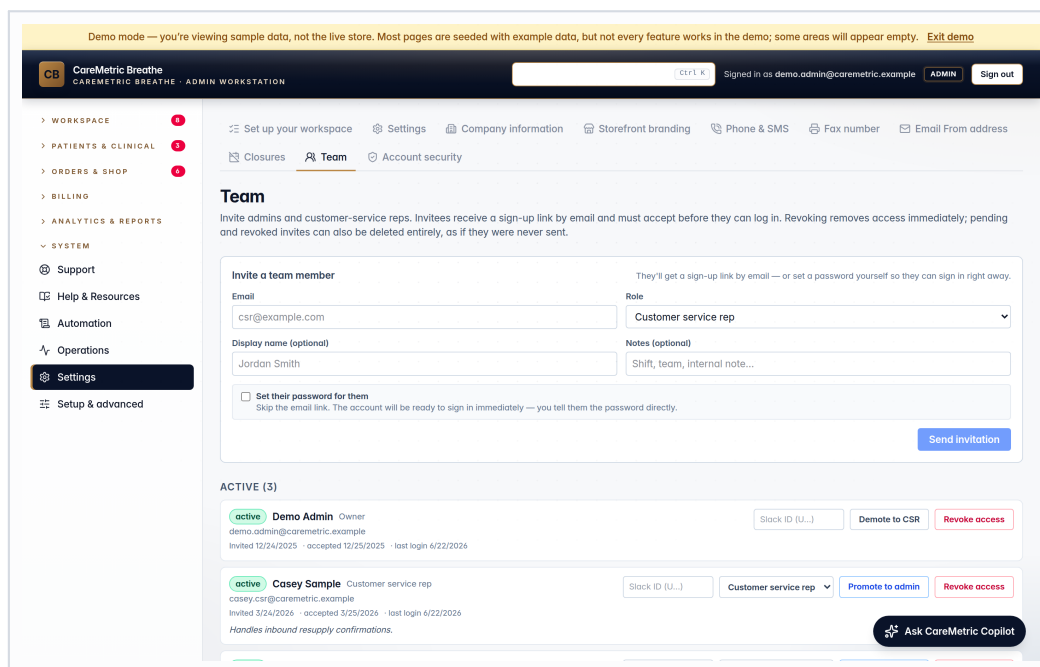
Part 3 — Job Aides by Role

Step-by-step walkthroughs for the highest-value, most common tasks in each role. Navigation paths are shown as **Group** → **Section** → **Page**.

ADMINISTRATOR (OWNER & ADMIN)

Administrator

Job aides — Run the whole practice



The Team page — invite staff and assign a role (Owner, Admin, CSR, Biller, RT).

Start the day with Owner overview

Read activity, current work and financial coverage in their correct periods.

- 1 Open **Analytics & Reports - Owner overview** and choose a preset or **Custom dates**, then **Apply dates**.
- 2 Read the UTC period, equal-duration comparison and update time. Use **Refresh** when needed.
- 3 Review activity and current priorities, then open the relevant order, claims, resupply or service queue. Detail pages have their own filters.
- 4 Check shipping evidence and stock coverage; do not interpret prepared units or untracked stock as delivered or available.
- 5 Read date-quality and incomplete-review counts before comparing period financial activity with separate lifetime completed-review contribution.
- 6 Choose **Download overview CSV** for the returned snapshot. An unavailable source is labeled, and refunds, credits and losses remain numeric.

Tip Claims paid to date, period financial events and lifetime contribution measure different things. Do not add them together as receipts or company net profit.

Set up a reviewable pricing policy

Supply the business's own economics before requiring reviews.

- 1 Open **Orders & Leads - Pricing & Profitability**. In **Supplier costs**, map exact SKUs, enter cost per canonical selling unit and supplier pack/quantity rules, verify evidence and expiry, and identify all freight/fee inclusions.
- 2 In **Collection evidence**, verify the patient's exact items, allowed amount and expected total collections. Reconcile any component shares once.
- 3 In **Pricing policy**, enter target, floor, optional minimum dollars, basis and any dated scoped rules; choose **Save draft policy**.
- 4 For an initial policy, leave **Require an approved insurance review before creating patient orders** unchecked, then choose **Publish this policy** and confirm. Check that **Patient order review** remains **Optional**; Item review needs a published policy to evaluate.
- 5 Evaluate representative patient, supplier and delivery cases in **Item review**, resolving blocked or missing evidence.
- 6 When the workflow is ready, select **Require an approved insurance review before creating patient orders**, choose **Publish this policy** again and confirm the requirement.
- 7 Verify the current policy and **Patient order review** cards. Review availability and follow-up after enabling.

Tip Neither a default margin nor free freight is invented. Publishing prices does not create a patient checkout or collect money.

Compare suppliers for an unpriced item

Save a provisional cost comparison without promising a customer price.

- 1 Open **New items** and describe the exact manufacturer, model, size, source and terms.
- 2 Enter the same requested quantity, destination and service for up to five supplier options.
- 3 Record pack cost, units per pack, minimum packs, applicable fee categories, evidence and expiry. Mark genuine inclusions; leave unknown costs blank.
- 4 Compare purchase outlay, purchased/surplus units and delivered-cost completeness, as well as availability, lead time and clinical suitability.
- 5 Save the proposal, verify the chosen supplier evidence and link the canonical SKU before preparing an authoritative order review.

Tip The least goods cost is not necessarily the least delivered cost. An estimate is not a verified supplier commitment.

Choose an owner profit model

Answer a business question using explicit assumptions.

- 1 Evaluate an item and choose **Use in owner models**, or open **Owner models** and choose a saved scenario; use **Next saved scenarios** for more.

- 2 Choose **Pricing strategies** for a unit-price decision; **Monthly break-even & profit** for required orders; **Cost & collection stress** for cost/collection changes; **Price versus volume** for demand cases; **Repeat orders & acquisition** for finite repeat orders; **Working capital** for a cash-funding estimate.
- 3 Enter only the card's needed assumptions. Keep blank unknown inputs blank, and enter fixed costs once for the stated period.
- 4 For multi-item price strategies, choose the one line allowed to change. Insurance collection assumptions do not rise with billed prices.
- 5 Calculate each needed section, read its status and original-policy checks, then compare results. Edited assumptions require recalculation.
- 6 Use **Download scenario report** before leaving if a planning record is needed; it includes each returned source/version and assumptions.

Tip Refreshing catalog assumptions clears old results and does not renew expired manual evidence. No model publishes a price, approves a patient order or proves demand.

Publish a selected portfolio change

Update future prices while preserving unselected contexts.

- 1 Open **Bulk prices**. Filter the pricing portfolio by item, exact category or supplier, then select the intended complete scenarios.
- 2 Refresh current catalog evidence and review full bundles. Narrow a filter if the 1,000-item scan limit is reached.
- 3 Enter proposed unit amounts or explicitly request recommendations, name the price list and create the frozen preview.
- 4 Review selected changes and retained prices, old/new amounts, current-assumption margins, exclusions and every issue. The complete list must fit the 100-scenario limit.
- 5 Resolve retained-context problems before proceeding. If another publication changed the covered contexts or retained amounts, make a fresh preview.
- 6 Activate the validated version, or set **Activate later (your local time)**. Check schedule/history afterward and re-review changed dependencies.

Tip Returning to an eligible earlier version changes future reviews only. It cannot overwrite accepted orders or silently remove other published prices.

Record and reconcile actual financial events

Preserve source evidence and avoid duplicate money after a network error.

- 1 Open **Reviews & actuals** and select the bound order review.
- 2 Choose the actual source/effect, amount, source document reference, unique economic event reference and optional item allocation.
- 3 Enter **Economic occurrence date and time (UTC)** from evidence. Change the default current minute for a historical invoice or payment, then **Record actual event**.
- 4 If the response is lost, use unchanged **Retry**. If edited details conflict, choose **Refresh actual event history** and investigate before recording a correction; keep stable references.
- 5 Page through history as needed. Compare original quote, current forecast and actuals separately; totals include all pages.

- 6 Confirm cost and collection completeness separately with the reason. Refresh and review any revision conflict; new events reopen completeness.

Tip *A refund reduces revenue; a supplier credit reduces cost. Correct recorded history with a documented new economic event, not a replacement identifier for the same expense.*

Review delivery after a signed-order address change

Release the right delivery while keeping accepted customer terms.

- 1 Open the held signed order and its delivery review action. Confirm the current patient address and exact original items/amounts.
- 2 Obtain new delivery evidence for the correct service and fulfillment source. Review current supplier costs and any expired fee/reserve evidence.
- 3 Compare original and revised contribution at the accepted amounts and fixed collectible revenue; record the reason and any permitted exception acknowledgement.
- 4 Approve the exact valid review revision. A hard financial floor or unresolved prescription requirement still blocks release.
- 5 Retry fulfillment after prerequisites are resolved, and confirm the released SKU quantities before shipping or a manual PacWare export.

Tip *A general address-alert resolution is not delivery approval. The review cannot silently raise accepted amounts, extend an expired signature request or change already shipped history.*

Create your CareMetric Breathe workspace (self-serve sign-up)

How a brand-new practice gets its own workspace and first Owner login.

- 1 Go to the public site **/breathe** and choose **Create your account (/breathe/signup)**.
- 2 Enter your **company name**, a **work email**, a **password** (at least 12 characters), and pick a **plan** (Virtual Mask Fitter, Launch, Growth, or Scale — Enterprise routes to sales).
- 3 Submit. We provision your organization, copy in your plan's feature-flag bundle, and create your first **Owner** login.
- 4 Open the **verification email** we send and click the link to activate the account (sign-in is blocked until it's verified).
- 5 Sign in at **/admin/sign-in**, confirm payment, then work the setup checklist below.

Tip *Only the first Owner self-signs-up; every teammate after that is invited from the Team page. (Operators can also pre-provision a workspace from the command line.)*

Sign in and turn on multi-factor authentication

Do this first — every admin account should have MFA.

- 1 Go to **/admin/sign-in** and sign in with your email and password.
- 2 Open **System** → **Account Security**.
- 3 Choose **Add authenticator** and scan the QR code with your phone's authenticator app (Google Authenticator, Authy, 1Password, etc.).
- 4 Enter the 6-digit code to confirm, then save your recovery codes somewhere safe.
- 5 Sign out and back in to confirm the code is requested.

Tip If you lose your authenticator, a recovery code or another Owner can restore access — store recovery codes offline.

Finish the workspace setup checklist

Make the platform run under your own brand, numbers, and payments.

- 1 Open **System** → **Set Up Your Workspace** (or click the “finish setting up” banner on Home).
- 2 Set your brand and logo under **Storefront Branding**, and your legal details under **Company Information**.
- 3 Connect your numbers under **Phone & SMS** and **Fax Number**, and your sender under **Email From Address** (authenticate the sending domain for deliverability).
- 4 Review the practice's CareMetric subscription under **Package & usage**. Configure the organization's billing identity separately for insurance claims; subscription billing does not enable patient card checkout.
- 5 Work down the checklist until every essential step is green.

Tip A NULL/unset From address falls back to the platform default — patient email still sends, but your own authenticated domain lands in the inbox instead of spam.

Invite a team member and assign a role

Add staff and scope their access to exactly what they need.

- 1 Open **System** → **Team** (Owner only).
- 2 In the invite card, enter the person's email and pick a role: **Owner**, **Admin**, **Customer service rep**, **Biller**, or **Respiratory Therapist**.
- 3 Optionally add a display name, a home location, and notes, then send the invite.
- 4 The invitee receives a sign-up link by email and sets their own password before first login.
- 5 To change someone later, use the role selector on their row, or **Revoke** to remove access immediately.

Tip Pick **Biller** for revenue-cycle staff — it unlocks the whole Billing area without exposing CSR tools, clinical notes, or system settings.

Turn a feature on or off

The Control Center is the master switch panel.

- 1 Open **System** → **Setup & Advanced** → **Control Center**.
- 2 Find the feature — voice agent, SMS/email campaigns, claim auto-submit, AI billing, the storefront chatbot, the admin assistant, multi-location, etc.
- 3 Flip the toggle; the change takes effect immediately for everyone.
- 4 For automation that sends messages (e.g. auto-submit), confirm the matching schedule/credentials are configured too.

Tip Use the Bot Playground to rehearse the chatbots before enabling them for real patients.

Connect a therapy-cloud integration

Pull device adherence data from ResMed, Philips, or 3B.

- 1 Open **System** → **Setup & Advanced** → **System Configuration** (Owner only) and enter the vendor credentials.
- 2 Open **System** → **Operations** → **Integrations** to confirm the connection reports *available*.
- 3 Trigger a sync (or wait for the nightly job) and watch the sync status.
- 4 Patient therapy data now flows into the RT Overview and Therapy Fleet boards.

Tip If an integration shows unavailable, a credential is missing — re-check System Configuration; the badge never reveals which secret is unset.

Send team alerts to Slack

Bring real-time CS alerts and operator digests into your team's Slack.

- 1 In Slack, create an app/bot and note its **bot token**, **signing secret**, and the **channel** for alerts.
- 2 Open **System** → **Setup & Advanced** → **System Configuration** (Owner only) and enter those values.
- 3 In the **Control Center**, confirm `slack.notifications`, `slack.interactivity`, and `slack.digests` are on (they ship on but stay inert until the credentials exist).
- 4 On the **Team** page, link each teammate's Slack handle so Escalate actions are attributed correctly.
- 5 Post a test event and confirm the alert (with its Escalate button and deep link) lands in your channel.

Tip Slack messages are deliberately non-PHI — a reference, a status, and a link back into the console — never message bodies, phone numbers, or clinical detail.

Check that the plumbing is healthy

A two-minute daily glance at jobs and message delivery.

- 1 Open **System** → **Operations** and confirm the background worker and nightly sync ran without errors.
- 2 Open **Outbound Messages** to scan recent SMS/email delivery results.
- 3 Open **Delivery Failures** and retry or resolve any bounces or shipping exceptions.
- 4 If a partner webhook failed, re-queue it from **Webhook Deliveries**.

Set business goals and KPI alerts

Tell the platform what “good” looks like and let it warn you.

- 1 Open **Analytics & Reports** → **Performance & Goals** → **Goals & Targets** and set a target per KPI and period.
- 2 Open **KPI Alerts** and define the thresholds that should trigger a warning (revenue, denial rate, churn).
- 3 Watch pace-to-goal on the dashboard and act on alerts as they fire.

Ask CareMetric Copilot for help

The fastest way to find a page or learn how something works.

- 1 Click **Ask CareMetric Copilot** on any admin page.
- 2 Type a plain-language question — “where do I turn features on or off?” or “walk me through processing a claim.”
- 3 Follow the clickable links it returns straight to the right page.
- 4 Have an idea for a missing feature? Tell it — after you confirm, it emails a structured suggestion to the owners.

Tip The assistant only explains the app and (with your OK) forwards ideas. It never changes data or shows patient PHI.

Get billing ready for go-live

Stand up the revenue cycle before the first claim.

- 1 Enter clearinghouse credentials (Office Ally; and Da Vinci PAS if used) under **System Configuration**.
- 2 Under **Billing** → **Config**, add your payer profiles, import the CMS DMEPOS fee schedule, and set modifier + denial-code rules.
- 3 Decide automation posture in the Control Center: eligibility pre-check, auto-re-verify, claim auto-submit, AI billing — start manual, then enable as you trust it.
- 4 Validate against Office Ally's test (T) cycle before flipping line-level changes like the ordering-provider loop on live claims.

Tip Run claims through the manual staged-approval path first; turn on the auto-submit cron once a few clean batches have gone out.

Connect PacWare (billing/warehouse) sync

Bridge to a legacy PacWare system without an API.

- 1 Open **System** → **Operations** → **PacWare**.
- 2 Import the patient roster CSV — existing patients only have BLANK fields filled, never overwritten.
- 3 Export the patient roster or resupply-due worklist with the verify step (preview the count + sample first).
- 4 Nothing is ever pushed automatically; the opt-in “ready to sync” notice tells staff when an export is worth running.

Run and export a report for the books

Pull the numbers your accountant needs in a couple of clicks.

- 1 Open **Analytics & Reports** → **Reports**.
- 2 Pick the report — revenue summary, patient payments, insurance claims, refunds journal, or the all-financial bundle.
- 3 Set the date range (up to 90 days).
- 4 Export as CSV, PDF, or QuickBooks (Desktop .iif or Online .qbo.csv).

Tip Patient payments and insurance claims are kept separate so cash isn't double-counted across the two.

See who accessed a patient's record

Answer a “who looked at this chart?” question.

- 1 Open **Analytics & Reports** → **Audit Trail** (admins only).
- 2 Filter by employee, by patient, and by time frame to narrow the access events.
- 3 Read the list of who accessed which patient's information, and when.

Tip The Audit Trail is kept out of the CSR and clinician sidebars; only full admins can open it.

Invite a provider to the e-sign portal

End the fax-and-chase cycle with a referring physician.

- 1 Confirm the provider exists under **Patients & Clinical** → **Providers & Recalls** → **Providers** (add them by NPI; NPPES verifies).
- 2 Make sure the provider portal is enabled in the Control Center.
- 3 Invite the provider from the E-signature Portal — they get a set-password link and must enroll MFA.
- 4 Stage their documents for signature; they sign on their device and you track each through to released/attached.

Tip Providers can also view their patients' therapy and reports in the portal — read-only, no PHI leaves your control.

BILLER

Biller

Job aides — Own the revenue cycle

Demo mode — you're viewing sample data, not the live store. Most pages are seeded with example data, but not every feature works in the demo; some areas will appear empty. [Exit demo](#)

CB CareMetric Breathe
CAREMETRIC BREATHE · ADMIN WORKSTATION

Signed in as demo.admin@caremetric.example ADMIN Sign out

WORKSPACE
PATIENTS & CLINICAL
ORDERS & SHOP
BILLING
Dashboards
Worklists
A/R & collections
Tools
ANALYTICS & REPORTS
SYSTEM

Verify insurance Insurance discovery Eligibility Re-verification **Prior auths** CMN / DIF worklist Bill hold
ADR / audit response Audit readiness Collections Auto-submit AI queue Denials worklist

Prior auths

PA Medicaid MCOs run a 7-day SLA starting 2026-01-01. The sweep job tags each PA every 6 hours; this view shows the rows that need a human now. The expiring bucket is your best defence against the "we got the order but no PA" billing gap.

MISSING SLA	AT-RISK SLA	AWAITING	EXPIRING SOON	DRAFTS
1 Past target, no decision	1 ≤ 2 days remaining	1 Submitted, no decision	1 Approved, lapsing ≤ 30d	1 Captured, not yet submitted

Expiring window

Expiring within

14d 30d 60d 90d

Missed SLA
Past target without a decision — call the MCO portal 1

PAYER	HCPCS	STATUS	AUTH #	TARGET	PATIENT
Demo Advantage	E0470	SUBMITTED	—	6/19/2026 Set post.	Open

At-risk SLA
≤ 2 days remaining on the 7-day Medicaid MCO clock

[Ask CareMetric Copilot](#)

*The Prior Auths worklist — at-risk SLAs and auths expiring soon.***Verify a patient's insurance (270/271)**

Confirm coverage before you bill.

- 1 Open **Billing** → **Worklists** → **Verify Insurance**.
- 2 Enter the patient and plan details (or pick an existing patient).
- 3 Run the check; the 271 response shows active/inactive status and plan details.
- 4 If coverage is unknown, use **Insurance Discovery** to find it from demographics instead.

Tip** Work the system-wide **Eligibility** worklist regularly — rejected and inactive coverage floats to the top.Submit a prior authorization**

Secure the auth before the claim goes out.

- 1 Open **Billing** → **Worklists** → **Prior Auths**.
- 2 Pick a draft (or a claim flagged as needing auth) and open the request form.
- 3 Complete the clinical justification and attach the supporting documentation.
- 4 Submit electronically via Da Vinci PAS where the payer supports it; otherwise follow the payer's channel.
- 5 Track the status and watch the "expiring soon" list so an auth never lapses mid-therapy.

Submit claims through the clearinghouse

Get clean claims out the door.

- 1 Open **Billing** → **Worklists** → **Auto-submit** to see claims that are preflight-clean with active eligibility.
- 2 Review the batch and approve it to transmit — or let the scheduled cron send it automatically.
- 3 Open **Billing** → **Tools** → **Office Ally** to watch 837P submissions and acknowledgements (999/277CA).
- 4 For a one-off or corrected claim, use **Manual Claim** instead.

Tip The auto-submit cron only fires when BOTH the schedule and the Control Center flag are on — ask an admin if it's quiet.

Post an ERA (835 remittance)

Auto-post payer payments and surface denials.

- 1 Open **Billing** → **Tools** → **ERA Files**.
- 2 Upload the 835 file from the payer/clearinghouse.
- 3 The system matches it to claims and posts the adjudications automatically.
- 4 Denied lines flow to the Denials worklist for follow-up.

Work denials and file an appeal

Recover the dollars worth chasing first.

- 1 Open **Billing** → **Worklists** → **Denials Worklist** — it's ranked by recoverable dollars × win-probability.
- 2 Open the top denial; the AI Queue may already suggest the corrected code or edit.
- 3 Fix and resubmit, or start an appeal with the supporting documentation.
- 4 Track the appeal to resolution.

*Tip Check **Denials & DSO** periodically to spot a payer whose denial rate is trending up.*

Work A/R aging and timely filing

Don't let claims age out or miss the filing window.

- 1 Open **Billing** → **A/R & collections** → **A/R Aging** and work the oldest buckets and worst payers first.
- 2 Open **Filing Deadlines** and clear anything close to its timely-filing cutoff immediately.
- 3 Roll any primary balances to secondary payers from **Secondary Claims**.

Send a patient statement

Collect the patient-responsibility balance.

- 1 Open **Billing** → **A/R & collections** → **Statement Send**.
- 2 Review the patient-responsibility amounts due.
- 3 Send by email or SMS — the system respects consent and quiet hours automatically.
- 4 Use the billing workflow to review and record any permitted patient responsibility; internal pricing simulations do not create a card checkout.

Configure a payer profile and fee schedule

Set the rules the claim engine reads.

- 1 Open **Billing** → **Tools** → **Config**.
- 2 Add or edit the payer profile (IDs, addresses, claim format).
- 3 Import or enter the fee schedule (CMS import is available), modifier rules, and denial-code mappings.
- 4 Map HCPCS coverage diagnoses so claims build with the right codes.

Tip Fee-schedule and clearinghouse setup is usually a one-time job with an admin — once it's right, the worklists run smoothly.

Quick-verify a patient's coverage from their chart

The one-click check you'll use most.

- 1 Open the patient (**Patients & Clinical** → **Patients**) or their billing tab.
- 2 Click **Verify insurance** in the quick-actions card.
- 3 Read the 271 result inline — active/inactive, plan, copay/deductible — or the “queued” note if the payer answers by deferred file.
- 4 The result is saved to the patient's eligibility history for the next biller.

Tip Same button lives on the eligibility worklist and a standalone cross-patient verifier (Billing → Verify Insurance) for bulk checks.

Find unknown coverage with Insurance Discovery

Turn “we don't know who covers them” into a billable claim.

- 1 Open **Billing** → **Worklists** → **Insurance Discovery**.
- 2 Enter the patient's demographics (name, DOB, ZIP; optional SSN / member-id hint).
- 3 Run the search; review the active coverages the payer network returns.
- 4 Attach the right coverage to the patient and proceed to verify/bill.

Tip Insurance Discovery is a paid clearinghouse add-on — if it's greyed out, an Owner enables it in the Control Center for your tenant.

Work the AI Queue

Let the assistant tee up the highest-payoff claims.

- 1 Open **Billing** → **Worklists** → **AI Queue**.
- 2 **Blocking** drafts need your input — open, fix the flagged issue, re-scrub.
- 3 **Fixable** drafts come with suggested patches — review and apply with one click.
- 4 **Needs analysis** denials — run the denial analyzer to get root cause + fix steps.
- 5 **Auto-resubmit ready** — the fix is safe and high-confidence; approve the one-click resubmit.

Tip The AI never changes a claim without you approving the patch; PHI is minimized before anything reaches the model.

Advance a capped-rental cycle

Keep Medicare CPAP rentals billing on schedule with the right modifiers.

- 1 Open **Billing** → **A/R & collections** → **Capped Rentals**.
- 2 Review the cycles due for their next rental month.
- 3 Confirm the auto-generated monthly claim — the modifier is already rotated (KH months 1–3, KI from month 4, plus KX once 90-day compliance is met).
- 4 Submit it with the rest of your batch.

Tip Compliance (≥ 4 hrs on $\geq 70\%$ of nights) is read from the therapy-cloud sync, so the KX modifier is applied only when the data supports it.

Generate and submit a secondary (COB) claim

Collect the balance the primary left behind.

- 1 After a primary pays (via ERA), open **Billing** → **A/R & collections** → **Secondary Claims**.
- 2 Open the auto-drafted secondary — the COB amounts (paid, contractual, patient responsibility) are already snapshotted from the primary's 835.
- 3 Review the secondary payer and line amounts, then submit.
- 4 If auto-draft is off for your tenant, use the COB-eligible worklist to create it.

Tip Secondary claims are never auto-submitted — they always wait for a biller to review.

Clear a bill hold

Release a claim once its paperwork is back.

- 1 A claim with outstanding REQUIRED paperwork (Rx, SWO/DWO, CMN, AOB, ABN, proof of delivery) is held from submission.
- 2 Open the claim or **Billing** → **Worklists** → **Bill Hold** to see what's missing.
- 3 Mark the requirement satisfied — or it clears itself when the patient e-signs, you upload the doc, or an inbound fax auto-matches it.
- 4 The hold lifts automatically when the last requirement is met; the claim joins the next batch.

Work the patient collections ladder

Escalate unpaid balances without dunning anyone who has already paid.

- 1 Open **Billing** → **A/R & collections** → **Collections**.
- 2 Each unpaid balance sits on a dunning rung — statement → reminder → second notice → final notice → agency — and advances on its cadence automatically.
- 3 Batch-print the final-notice letters as one PDF for the accounts that have reached that rung.
- 4 For accounts that reach the agency step, export the agency-eligible list (a reviewed, deliberate action) to place with your collections partner.

Tip The ladder pauses the instant a balance is paid or the patient goes on a payment plan, so settled patients are never dunned. Requires the `collections.dunning_flag` (and `collections.agency_export` for the hand-off) — an Owner enables them in the Control Center.

Respond to a Medicare ADR (audit request)

Answer an Additional Documentation Request before its deadline.

- 1 Open **Billing** → **A/R & collections** → **ADR / Audit Response** — open requests are ranked by response deadline.
- 2 Open the request and work its checklist of the documents the payer/contractor wants.
- 3 Use **Build audit packet** to assemble the SWO, CMN, sleep study, compliance summary, proof of delivery, and notes into one PDF, then submit it.
- 4 Record the outcome (won/lost) so your audit win-rate is tracked.

Tip Requires the `billing.adr_queue` flag (an Owner enables it in the Control Center); a nightly sweep surfaces at-risk and overdue deadlines.

CUSTOMER SERVICE REP (CSR)

Customer Service Rep

Job aides — The patient's first point of contact

Demo mode — you're viewing sample data, not the live store. Most pages are seeded with example data, but not every feature works in the demo; some areas will appear empty. [Exit demo](#)

CareMetric Breathe
ADMIN WORKSTATION

Signed in as demo.admin@caremetric.example ADMIN Sign out

WORKSPACE PATIENTS & CLINICAL ORDERS & LEADS Catalog Backorders Orders Leads BILLING ANALYTICS & REPORTS SYSTEM

Insurance Leads Filter Invites **Fit Requests** Filter Prospects

Fit requests

Patients who finished the mask fitter and asked you to take it from there — either by sending their details or by asking for a call. Nothing here has been ordered or billed. Oldest first, because the confirmation email promises a reply within one business day.

NEW 4 CONTACTED 2 IN PROGRESS 1 CLOSED 1

Show all (8) All request types Refresh Showing 4 request(s)

Patient	Fitting	Insurance	Waiting	Status	CSR note
Riley Tester riley.test@caremetric.example +12155550113 · DOB 1964-11-30 Prefers: Email	DETAILS SENT ResMed AirFit F30i Size M	Medicare Member 1EG4-TE5-MIC72 Rx by Dr. Alex Rivera	14h ago	NEW New	Add a note... Save note
Morgan Example morgan.example@caremetric.example +12155550112 · DOB 2018-06-02 Prefers: Text · Evening	DETAILS SENT PEDIATRIC ResMed Pxi Pediatric Size one-size "Fitting was for a child — pediatric service line."	Highmark Member HMK-441920 Group HM-B8 Rx by Dr. Sam Patel	8h ago	NEW New	Add a note... Save note
Casey Demo casey.demo@caremetric.example +12155550111 Prefers: Call · Morning	CALLBACK ResMed AirFit P10 Size S	Not provided — verify	5h ago	NEW New	Add a note... Save note
Jordan Sample jordan.sample@caremetric.example +12155550110 · DOB 1978-04-12 Prefers: Call · Afternoon	DETAILS SENT ResMed AirFit N20 Size M "Sleeps on their side; asked about a quieter"	Aetna Member W2840173355 Group PA-00271 Rx by Dr. Alex Rivera	2h ago	NEW New	Add a note... Save note

Ask CareMetric Copilot

*Fit Requests — finished fittings waiting for an order.***Check what was ordered and when supplies are due**

Use history and replacement rules before contacting the patient.

- 1 Open the patient's resupply view, or select **Orders & eligibility** on the **Resupply calendar**.
- 2 Read the exact supply, HCPCS mapping, last ordered date, prescription expiry, interval/quantity result and separate scheduled due date.
- 3 Check **Order history** status, quantity and shipment evidence. Use **Older/Newer** to inspect beyond the first 25 lines.
- 4 Review **CSR orders & signatures** separately so a request and its later fulfillment are not mistaken for two supplies.
- 5 Resolve missing eligibility information, then verify coverage, valid prescription and actual patient need before ordering.

Tip Due on the calendar does not mean coverage approved. Missing mapping or history needs review; it is not an unlimited allowance.

Ask one patient or a batch about new supplies

Queue the right channel deliberately and inspect every result.

- 1 Open **Workspace - Schedule - Resupply calendar**. Select a month/day or **Due now & overdue**, and search if needed.
- 2 Open **Orders & eligibility** to review a patient. Use that due patient's action or select intended rows, up to 50, in the calendar.
- 3 Choose **Email, SMS or Automated call**. Review the named recipients and confirm **Queue outreach**.
- 4 Read queued, skipped and failed results per patient. Correct missing details or respect the stated timing, consent and recent-contact restriction.
- 5 Follow processing, delivery and replies in **Conversations**. Confirm supplies are wanted, then complete the normal order and financial review.

Tip Queued is not delivered or ordered. Do not repeatedly switch channels after a skip or uncertain send; check the conversation first.

Attach a valid pricing review to an order

Use exact patient/item economics without managing business policy.

- 1 Open **Pricing & Profitability - Item review** or the embedded order review. Select the patient, exact SKU/quantity, supplier, fulfillment method, delivery and insurance evidence.
- 2 Calculate and resolve missing or blocked inputs. Use **Save review** for a verified within-policy result or **Request manager approval** for a permitted exception.
- 3 After approval, choose that matching review in the order form. Recalculate for changed items/quantities; another patient's approval cannot be reused.
- 4 Send the normal signature request. An expired pricing deadline requires a fresh review/request, not a longer resend.
- 5 After signing, verify exact fulfillment lines and address/prescription holds; request manager delivery re-review when required.

Tip An internal self-pay simulation cannot attach to the insurance patient workflow. The signature packet excludes private cost and margin details.

Check existing approvals for resupply drafts

Review a selected group without sending or ordering.

- 1 Open **Pricing & Profitability - Patient batch review**.
- 2 Select the intended drafts, up to 50, and choose **Evaluate selected drafts**.
- 3 Read each patient's ready, stale, missing-review or unavailable result. Each patient's exact items and delivery assumptions are checked separately.
- 4 Open an individual review to fix mappings/evidence or choose another still-valid approval when the newest matching approval is stale.
- 5 Create and send each authorized order through the normal workflow only after the required checks.

Tip Batch pricing review is separate from bulk resupply outreach. Neither action automatically performs the other.

Handle an inbound message

Triage and reply in the unified inbox.

- 1 Open **Workspace** → **Conversations** and pick a thread awaiting reply (or claim one for yourself).
- 2 Read the patient's history in the side panel for context.
- 3 Reply directly, or insert a **Canned Reply** for a common answer.
- 4 Tag or snooze the thread, or promote it to a **Case** if it spans multiple channels.
- 5 If it needs a specialist, assign it or escalate.

Tip Order-, account-, and clinical-specific questions should go to the right person — use Cases so the full history travels with the issue.

Work a fit request

Turn a finished mask fitting into an order.

- 1 Open **Orders & Leads** → **Fit Requests** — oldest first; the patient was told one business day.
- 2 Read what they asked for: they either sent their details or asked to be called.
- 3 Verify the benefit properly — whatever the patient typed is a starting point, not a verified plan.
- 4 Place the order and move the row through Contacted → In progress.
- 5 Close it with the real outcome. Only **Fulfilled** means the patient has the mask.

Tip Never close a row as Fulfilled to tidy the queue — it inflates your dispense rate and tells the outcomes dashboard a mask arrived that never did.

Look up a patient and read the 360° timeline

Everything about a patient in one place.

- 1 Open **Patients & Clinical** → **Patients** and search by name or phone.
- 2 Open the record to see the unified timeline — orders, messages, documents, therapy, and billing.
- 3 Edit demographics or add a note as needed.
- 4 If you find a duplicate, flag it in **Duplicate Review** to merge the charts.

Send a bulk SMS or email campaign

Reach a filtered audience at once.

- 1 Confirm your role has campaign-send permission, then open **Workspace - Outreach - Bulk Campaigns**. Standard CSRs should use Resupply calendar for due-patient outreach.
- 2 Build the audience with filters and check the resolved recipient count.
- 3 Draft the message (or start from a Playbook).
- 4 Review and send; track delivery under **Outbound Messages**.

Tip Always sanity-check the recipient count before sending — it's the cheapest way to avoid an embarrassing mass message.

Schedule a follow-up or video visit

Put the next touch on the calendar.

- 1 Open **Workspace** → **Schedule** → **Company Calendar**.
- 2 Create an appointment (fitting, setup, follow-up) on the right day, respecting office hours.
- 3 For telehealth, create a **Video Visit** — the join link is sent to the patient by SMS/email.
- 4 Track the callback queue under **Follow-ups**.

Process a return or RMA

Decide a return and close it out.

- 1 Open **Orders & Leads** → **Orders** → **Returns & RMAs**.
- 2 Open the return request and review the reason and comfort-guarantee window.
- 3 Approve or deny; on approval, choose restock and/or refund.
- 4 The patient is notified automatically of the decision.

Fulfill and ship an order

Get product out the door with tracking.

- 1 Review the patient's exact signed order, required documents and any prescription, stock or delivery-address hold before release.
- 2 Use the **PacWare** export preview for the insurance resupply handoff. Check withheld rows and exact items and quantities, then download and import the eligible rows into PacWare.
- 3 Complete the warehouse or supplier shipping workflow. Import the actual shipment report through **PacWare**, review matched, unmatched and ambiguous rows, and record only verified matches.
- 4 Check the patient's **Fulfillments** history for the recorded shipment dates and references. Follow up on missing evidence before telling the patient an order shipped.

***Tip** Exporting or queuing an order does not ship it. Shipping Labels is a separate historical cash-pay shop worklist; an Item review rate estimate does not purchase a label.*

Send an e-signature document packet

Get new-patient paperwork signed.

- 1 Open **Patients & Clinical** → **Documents & e-sign** → **Document Packets**.
- 2 Choose the patient and the packet template (CMN, prescription, agreement).
- 3 Send for e-signature; track progress under **Awaiting Signatures**.
- 4 Returned faxes can be scanned in and filed against the patient.

Triage an inbound fax or referral

Turn a faxed referral into a patient.

- 1 Open **Patients & Clinical** → **Documents & e-sign** → **Inbound Faxes** and review the queue.
- 2 For a referral packet, open **Referral Reviewer** — the AI has already extracted the intake fields.
- 3 Verify the demographics and insurance, then accept it into a new patient record.
- 4 Attribute the referral to its source so the physician scorecard stays accurate.

Invite a patient to the AI mask fitter

Let a patient size their mask from a selfie.

- 1 Open **Orders & Leads** → **Storefront & Leads** → **Fitter Invites**.
- 2 Enter the patient and send the invite link.
- 3 When they finish, review the returned mask and size recommendation.
- 4 Convert it to an order, or follow up from the Prospects funnel.

Reply to a patient email (with AI assist)

Work the email side of the inbox.

- 1 Open **Workspace** → **Email Inbox** and pick a thread in “needs response.”
- 2 If AI auto-reply is on, a high-confidence draft may already be proposed — review and send, or edit first.
- 3 For anything order-, account-, or clinically-specific, write the reply yourself or escalate to a case.
- 4 Sent threads move to “already answered.”

Tip AI auto-reply only sends on its own above a confidence bar; everything else falls to a human by design.

Work an insurance lead

Follow up someone who asked what their plan covers.

- 1 Open **Orders & Leads** → **Insurance Leads**.
- 2 Review what they asked for and their contact details.
- 3 Run the benefit check, then call or message them with what you found.
- 4 Convert them into a patient record when they want to proceed.

Keep stock straight after a count

Make the shelf and the system agree.

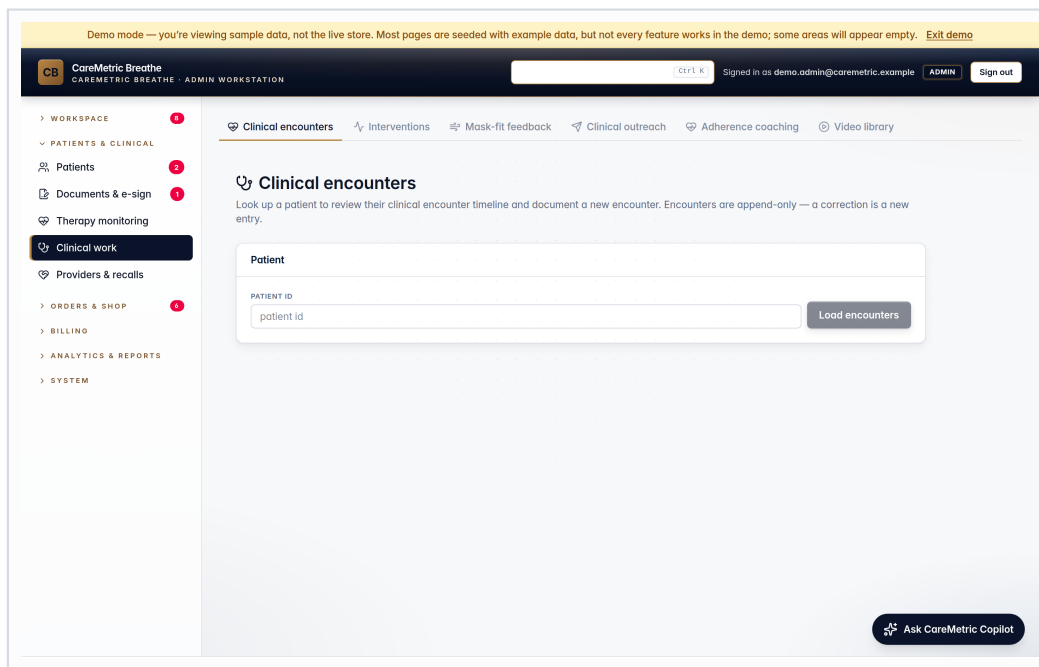
- 1 Open **Orders & Leads** → **Catalog**.
- 2 Find the SKU and record the difference as a **counted** movement — never type over the total.
- 3 Give the movement a reason while you still remember it; that is what explains the variance later.
- 4 Recording a receipt also clears any backorder on that SKU automatically.

Tip A blank count means untracked, not zero — an untracked SKU never warns you that it has run out.

RESPIRATORY THERAPIST (RT)

Respiratory Therapist

Watch therapy, keep patients adherent, and document the clinical care that backs every claim.



Clinical Encounters — document the care you provided.

Review the therapy board and spot at-risk patients

Your daily clinical triage.

- 1 Open **Patients & Clinical** → **Therapy Monitoring** → **RT Overview**.
- 2 Scan the alerts — high AHI, high mask leak, low usage — sorted by severity.
- 3 Open a flagged patient to see their therapy detail.
- 4 Use **Therapy Fleet** to work a whole compliance cohort at once.

Tip Therapy data is only as fresh as the integration sync — if a board looks empty, ask an admin to check Integrations.

Document a clinical encounter

Capture the care you provided.

- 1 Open the patient's record, or **Patients & Clinical** → **Clinical Work** → **Clinical Encounters**.
- 2 Start a new encounter and choose its type (mask fit, troubleshoot, setup education, adherence intervention, phone, other).
- 3 Record the reason, assessment, intervention, and plan — or just a free-text note.
- 4 Set a follow-up date if one is needed, then save.

Tip Encounters build the patient's clinical timeline and back up the medical necessity behind claims — document while it's fresh.

Open and work a non-adherence intervention

Get a slipping patient back on therapy.

- 1 Open **Patients & Clinical** → **Clinical Work** → **Interventions**.
- 2 Open the patient (or create an intervention from an RT Overview alert).
- 3 Record the cause of non-adherence and the plan.
- 4 Reach out via **Clinical Outreach** (consent/DND-aware), then log the outcome.
- 5 Track it to resolution.

Track CMS 90-day setup adherence

Make sure new Medicare setups clear their compliance window.

- 1 Open **Patients & Clinical** → **Therapy Monitoring** → **Setup Adherence**.
- 2 Review each new setup's progress toward ≥ 4 hours on $\geq 70\%$ of nights within the 90-day window.
- 3 For anyone trending short, open an intervention and coach early.
- 4 Confirm passing patients before the window closes.

Work the resupply-due worklist

Catch supplies that need replacing.

- 1 Open **Patients & Clinical** → **Therapy Monitoring** → **Resupply Opportunities**.
- 2 Review device-reported supplies that are due.
- 3 Confirm the patient's needs and hand off to resupply ordering.
- 4 Reorder reminders can carry the rest automatically.

Triage mask-fit feedback

Fix leaks and discomfort before they cause drop-off.

- 1 Open **Patients & Clinical** → **Clinical Work** → **Mask-fit Feedback**.
- 2 Open a patient reporting a leaking or uncomfortable fit.
- 3 Decide on a refit or follow-up and act on it.
- 4 Document the encounter and outcome.

Generate a provider-ready therapy report

Close the loop with the referring physician.

- 1 Open **Analytics & Reports** → **Clinical & Customer** → **Therapy Report**.
- 2 Choose the scope — by provider, patient, or device manufacturer.
- 3 Generate the print-quality adherence snapshot.
- 4 Send or print it for the physician's chart.

Check equipment recalls

Protect patients on recalled devices.

- 1 Open **Patients & Clinical** → **Providers & Recalls** → **Recalls**.
- 2 Review the manufacturer recall registry.
- 3 The system scans recalls against your dispensed serials and flags at-risk patients.
- 4 Reach out to affected patients and arrange a remedy.

Request or renew a prescription

Keep orders billable with current paperwork.

- 1 From the patient chart or **Patients & Clinical** → **Providers & Recalls** → **Providers**, find the ordering provider.
- 2 Start a prescription / renewal request and stage the document.
- 3 Send it for the provider's e-signature (or fax it) and track it under Awaiting Signatures.
- 4 Once signed, it files to the chart and lifts any related bill hold.

Run a population outreach from a compliance cohort

Act on a whole at-risk group at once.

- 1 Open **Patients & Clinical** → **Therapy Monitoring** → **Therapy Fleet**.
- 2 Pick the cohort that needs attention (e.g. low-usage, leak, or slipping adherence).
- 3 Launch consent/DND-aware Clinical Outreach to the cohort.
- 4 Track responses and open interventions for those who need a closer touch.

Frequently Asked Questions

Quick answers, grouped by area. For step-by-step instructions see the job aides in Part 3; for the full detail see Part 2.

Pricing and owner models

Q Which model should I choose?

Use pricing strategies for a selected item's unit amount; monthly break-even for required order counts; sensitivity for explicit cost/freight/collection changes; price and volume for user-entered demand cases; acquisition for finite repeat orders; working capital for a cash-funding estimate. Calculate cards separately from a reviewed scenario and read their assumptions and statuses.

Q Do simulations publish prices or take patient payments?

No. Owner models and internal self-pay calculations are planning only. They do not approve orders, change insurance reimbursement, add patient checkout or replace accounting net profit. Use the normal review and publication controls for operational changes.

Q Why is markup different from margin?

Markup divides profit by costs; margin divides it by revenue. Owner markup applies to all variable costs, including relevant fees, not merchandise alone. The shared calculation applies exact cents and policy boundaries; a rounded displayed percentage is not the approval test.

Q Why does a supplier's low price still fail review?

Pack minimums, freight, dropship/handling, payment costs or incomplete evidence may change delivered cost. Check destination/service coverage and inclusions. Blank cost is unknown, not zero. Availability, clinical suitability and prescription requirements also remain relevant.

Q Can a CSR approve their own pricing exception?

A complete verified within-policy review can save as approved automatically. A permitted exception needs management approval with evidence; a hard floor stays blocked. CSRs cannot verify authoritative supplier costs, publish prices, enter actual financial events or use management owner models under the standard role.

Q Will changing one portfolio item remove other prices?

No. The frozen preview retains other published contexts and validates them at their existing amounts. A retained context with stale evidence requires review. Concurrent changes to contexts or retained amounts block an outdated activation or schedule until a fresh preview is made.

Q Can I resend an expired priced order to get more time?

No. Resend is capped by the approved review deadline. Prepare a fresh review and request after expiry. A signed-order address change uses management delivery re-review at the accepted prices, not silent repricing.

Q What if an actual-event save times out?

Use unchanged Retry: the full body and occurrence date are preserved. Keep the source and economic-event references. If edited details conflict, refresh history and check whether the first event exists before documenting a correction. Do not create another reference for the same expense or payment.

Q Are my model results saved forever?

No. Assumptions survive tabs in the current workspace, but the model is not a durable planning record. Download scenario report before leaving. Editing assumptions or refreshing sources removes outdated results from the current report.

Owner overview and financial dates**Q Why do changing dates not change every number?**

Activity uses the selected UTC period, stages use the current status of records created in that period, and current queues include all open work now. Lifetime completed-review contribution uses a different scope again. The labels distinguish these measures.

Q Is claim paid-to-date the same as cash received in this period?

No. It follows the claims-created cohort and includes later recorded payments. Pricing event activity uses the entered economic occurrence dates. Neither can be added to the other as a second receipt.

Q Which date should I enter for an invoice or payment?

Use Economic occurrence date and time (UTC) from the evidence, not today's entry time. The form accepts historical dates and rejects invalid or future manual dates. Missing, invalid or future imported dates are disclosed and excluded rather than replaced with record creation time.

Q Does an unavailable section mean zero?

No. Business and financial sources load independently, and a failed source is labeled unavailable in the screen and CSV. Retry if both fail. Even an available zero only describes recorded matching data; it does not prove every event has been entered.

Q Can I sum negative amounts in the CSV?

Yes. Refunds, credits and losses are numeric signed USD values, including negative amounts. Text is protected from spreadsheet formulas. Export represents the returned snapshot, not an unapplied date selection or a pending refresh.

Q Why is a product with no threshold marked low stock?

Tracked products use five units when no threshold is configured. A configured zero remains zero. Null stock is untracked and excluded from low-stock totals; the overview lists up to ten items but counts the full matching population.

Getting started & signing up**Q How does a new practice sign up?**

Go to the public CareMetric Breathe site (/breathe) and choose Create your account. Enter your company name, work email, a 12-character password, and a plan; that provisions your workspace and your first Owner login. Verify the emailed link, then sign in at /admin/sign-in and finish setup.

Q Do my staff each sign up too?

No — only the practice's first Owner self-signs-up. Everyone else is invited from the Team page and sets their own password from the invite link. Public self-signup is for patients creating their own account and for new tenants, not staff.

Q Why didn't I get an error when I mistyped my email?

For security the sign-up and sign-in pages never reveal whether an address is on file — they always say “check your email.” If nothing arrives, re-check the address and try again. (Inline checks still catch an obviously malformed email or a password that's too short before you submit.)

Q Can I start texting patients before I have my own phone number?

Yes — until you provision your own number, texts and calls go out on a shared platform number and a patient's reply routes back to you automatically. Provision your own dedicated number under Phone & SMS when you want messages to come from your own line.

Q I don't have a logo yet — what shows on my storefront?

Generate a one-click starter monogram from your storefront name under Storefront Branding; it's created in your browser so nothing shows blank while you source real artwork. A brand-new, unconfigured tenant otherwise shows neutral CareMetric branding, never another tenant's brand.

Q What's the fastest way to turn on the right features for my plan?

The Control Center has a one-click “Apply recommended preset” that sets your feature flags to the bundle for your billing plan. New tenants already start on it; re-apply it after changing plans, then fine-tune any individual flag.

Customer care

Q Where do patient messages go?

SMS, MMS, and email — plus AI call summaries — all land in one unified Conversations inbox. CSRs triage, claim a thread, reply (with canned replies), tag, snooze, and escalate to a Case, so nothing is scattered across separate tools.

Q What can the chatbot handle without a person?

The CareMetric Assistant answers therapy, mask, resupply, and insurance questions 24/7, and for signed-in patients it covers order status, tracking, subscriptions, device info, and returns — handing off to a CSR for anything order-specific, clinical, or actionable.

Q How do we make sure a promise to a patient isn't forgotten?

Use Episodes for dated service promises and Follow-ups for the callback queue; overdue items surface on the Home dashboard.

Q Can we run telehealth visits?

Yes — Video Visits generate a secure join link sent by SMS or email for setups and mask troubleshooting.

Q How do we message many patients at once?

For due supplies, use Resupply calendar and select up to 50 patients for email, SMS or automated calls. General Bulk Campaigns needs separate campaign-send permission. Always review the audience and the per-patient result.

Q How does a patient use the AI mask fitter?

The fitter is invitation-only. Staff send the patient a signed link by text or email from Orders & Leads → Fitter Invites; the patient opens it, runs the camera-based fitting in their browser, and the completed result attaches back to their chart (or waits in a holding area for staff to attach). The public storefront's “get fitted” button routes to an invitation-required explainer rather than the fitter.

Q Can patients buy anything from us directly?

No. The platform is insurance-only: patients are supplied against their plan and are never charged a card, so there is no cart, no checkout, and no membership to buy. Whatever their plan leaves them owing appears on their billing page, and your billing team collects it. Cash-pay memberships and card-on-file were retired.

Q Do post-purchase review requests go out automatically?

They can. The review request can always be sent on demand from the Reviews worklist ("Send due"), and once the automatic sweep is enabled it goes out on its own about two weeks after delivery — one request per order, only to customers who haven't opted out.

Q Can a returned item go back into inventory?

Optionally. When you mark a return received you can choose to restock it, which adds the quantities back to tracked stock. It's off by default because most DME consumables (opened masks and supplies) aren't resaleable — so you opt in only for genuinely resaleable items.

Billing & revenue cycle

Q Is insurance checked before we bill?

Yes. Eligibility runs by real-time 270/271 on a schedule and again in the seconds before a claim transmits; inactive or prior-auth-required coverage is held back so claims don't bounce.

Q How are only clean claims submitted?

Every claim is scrubbed — structurally and by AI — before submission (modifiers for the rental month, quantity vs. LCD limits, HCPCS/diagnosis match, fee-schedule drift). Problems are flagged or fixed first, lifting your first-pass acceptance rate.

Q What happens when a claim is denied?

The AI denial analyzer reads the CARC/RARC codes, finds the root cause, populates the corrective patch, and — when it's safe and high-confidence — resubmits in one click. The worklist is ranked by recoverable dollars.

Q How do payer payments get posted?

Upload the 835 ERA file and the system auto-posts allowed, paid, and patient-responsibility amounts at the claim and line level, flags denials for follow-up, and is idempotent if a file is re-posted.

Q Does it handle capped rentals and secondary claims?

Yes — capped-rental cycles auto-advance each month with the correct KH/KI/KX modifier rotation, and a secondary / COB claim auto-drafts after the primary pays (you review before submitting).

Q What if a patient's insurance is unknown?

Insurance Discovery searches the payer network from the patient's demographics to find active coverage (a paid clearinghouse add-on).

Q Can we bill on day one?

Yes — the platform ships with 100+ fully-configured payer profiles (the whole Pennsylvania DME market plus the national carriers, Medicare, Medicaid, federal, and workers'-comp/auto programs), each with electronic IDs, claim format, timely-filing window, prior-auth rules, and claims address pre-filled. Adding a payer for another region takes a minute under Billing → Config → Payers.

Reporting & analytics

Q What reports can I run?

The Reports catalog contains legacy operational and finance exports with its own date limits. Owner overview separately covers recorded business activity and pricing-review financial coverage over presets or custom UTC dates up to 366 days. Owner models exports explicit scenario assumptions and results. Check each report's definition before comparing totals.

Q Can I export to QuickBooks?

Yes — CSV and printable PDF, plus QuickBooks Desktop (.iif) and QuickBooks Online (.qbo.csv).

Q How do I know if I'm hitting my numbers?

Set a target per KPI in Goals & Targets and let KPI Alerts flag any metric that crosses a threshold. Financial analytics (margin & COGS, payer profitability, LTV:CAC) show where money is made and lost.

Q Is patient PHI in the reports?

PHI is minimized — storefront reports hash customer IDs and the customer-activity report is counts only.

Integrations (device clouds)

Q Which manufacturer clouds connect?

ResMed AirView, Philips Respironics Care Orchestrator, and 3B Medical React Health (Luna / iCode).

Q Do I have to log into each vendor portal?

No — a nightly sync pulls all three into one view, so you manage every patient from a single login regardless of device brand.

Q What data comes back?

Device settings, a compliance summary (days $\geq$ 4 hours, average usage and AHI, the CMS 90/30 flag), recent nights (usage, AHI, leak, pressure), and supply eligibility dates that drive resupply timing.

Q How current is the data?

A nightly job refreshes every active link (rate-limited so it's gentle on the vendor APIs); you can also force a manual refresh per patient or per source.

Re-supply

Q How and when do reorder reminders go out?

An hourly job finds prescriptions due on their cadence — typically the 90- and 30-day windows, tuned per patient and by frequency rules — and sends SMS or email. The AI voice agent can call, too.

Q How does the patient confirm a reorder?

Use the configured reminder response links or patient reply, then complete the normal order review. Confirmation does not bypass prescription, pricing, consent or delivery requirements and is not proof of shipment.

Q Does it avoid pestering patients?

Yes — a quiet-period guard skips anyone you've talked to in the last 48 hours, and only one reminder goes out per patient per cycle even if several items are due.

Q Can I contact just one patient or a due group?

Yes. Open Orders & eligibility for one patient, or select up to 50 patients in Resupply calendar. Choose Email, SMS or Automated call, review names and Queue outreach. Read per-patient results and follow delivery/replies in Conversations.

Q Does scheduled due mean eligible for all supplies?

No. Check each item's replacement-rule interval and remaining quantity, prescription expiry and insurance coverage. Needs eligibility review is unresolved, not approved. Patients must still confirm their need.

Q Is queued outreach already delivered?

No. The worker still checks contact details, channel permissions, local contact hours and recent contact. Queued is processing acceptance. Review skipped/failed reasons and delivery evidence before retrying; do not switch channels to work around a guard.

In-person / counter orders

Q Someone walks in wanting a mask — what now?

Create or open their patient record, verify the benefit from Billing → Verify insurance, and raise the order against their plan. There is no counter sale: everything is billed to insurance, so a walk-in follows the same path as any other patient, just faster.

Q Can I print a receipt and a shipping label?

Yes — print a receipt and, if you're shipping it, a label with the patient's address merged in and tracking auto-filled back onto the order.

Q Who is allowed to take counter orders?

Anyone with the orders.create permission — front-line CSRs and up.

Respiratory therapy (RT)

Q How do I find at-risk patients?

RT Overview and Therapy Fleet rank patients by reason — setup-adherence risk, no recent data, high AHI, high leak, usage decline — so you work the highest risk first.

Q How is CMS 90-day compliance tracked?

Setup Adherence measures each new PAP patient against ≥ 4 hours on ≥ 21 nights in any rolling 30-day window and flags at-risk patients early so you can coach them before the window closes.

Q Where do I document care?

Clinical Encounters — capture reason, assessment, intervention, and plan (or a free-text note); it builds the patient's clinical timeline and backs the medical necessity behind claims.

Q Can I reach a whole at-risk group at once?

Yes — launch consent- and do-not-disturb-aware Clinical Outreach to a Therapy Fleet cohort.

Q Can I give a referring provider a report?

Generate a print-quality Therapy Report by provider, patient, or device manufacturer.

Documents, e-signature & provider portal

Q Can the system read a referral for us?

Yes — the Referral Reviewer reads an uploaded or faxed referral in one AI pass and extracts the patient's demographics, insurance, ordered items (with HCPCS), sleep study, referring provider, and diagnoses. It flags anything a clean claim is missing, lets you verify insurance in one click, and — once you accept — creates the patient, attaches the coverage, and files the split documents to the chart.

Q Can we fax straight from the system?

Yes — send prescriptions, CMNs, orders, renewal requests, and appeal letters by eFax right from the document. No fax machine, no separate eFax login, and a failed fax retries in a click.

Q How does a faxed-back signature get filed?

Every document you send for signature carries a barcode. When the signed page is faxed back, the system reads that barcode, files the document to the right patient's chart, marks it returned and signed, and releases any bill-hold waiting on it — automatically. Anything it can't match goes to a triage queue for a person.

Q How do we keep track of signatures we're waiting on?

The Awaiting Signatures dashboard lists everything out for a provider's signature, who has it, and how long it's been outstanding — re-send the stale ones, mark items hand-delivered, or scan a returned barcode to file it instantly.

Q Do patients have to print and mail forms?

No — they e-sign on their own device (typed name + explicit ESIGN consent) and the signed PDF auto-files to the chart. It's ESIGN-Act compliant — nothing printed, nothing lost, no delay.

Q Which document templates are included?

SWO, PAP CMN, CMS-484/846/848, DWO / renewal forms, ABN (CMS-R-131), Assignment of Benefits, DMEPOS Supplier Standards, Proof of Delivery, Refill Confirmation, and a new-patient setup packet — plus free-form manual documents and documentation packets.

Q Can providers sign instead of faxing?

Yes — the provider portal lets a referring provider e-sign CMNs, DWOs, prescriptions, and claims on their device, captured in a tamper-evident audit trail. No more fax-and-chase.

Q Can providers see their patients' therapy?

Yes — read-only therapy data and reports for their own patients, in the same MFA-protected portal.

Roles, access & setup

Q What staff roles are there?

Owner, Admin, Customer Service Rep, Biller, and Respiratory Therapist — assigned on the Team page. A page above your role simply doesn't appear in your navigation.

Q How do I turn a feature on or off?

The Control Center has an immediate on/off toggle for every major feature (voice agent, campaigns, auto-submit, AI billing, the chatbots, multi-location, and more).

Q How do I add a teammate?

Invite them by email on the Team page and pick a role; they set their own password from the link, and admins enroll multi-factor authentication.

Q What has to be set up before go-live?

Brand and domain, phone/SMS/fax numbers, an authenticated email sender, payments, and — for billing — clearinghouse credentials and payer/fee-schedule config. The Setup Guide walks through it.

Q Can the team get alerts in Slack?

Yes — add a Slack bot token + channel under System Configuration and the platform posts real-time CS alerts and operator digests into your channel, with an Escalate button and a slash command to act on them. Messages are non-PHI (a reference, a status, and a deep link back into the console). Link each teammate's Slack handle on the Team page.

Q Can I see who looked at a patient's record?

Yes — the admins-only Audit Trail report (Analytics & Reports → Audit Trail) shows who accessed which patient's information and when, filterable by employee, patient, and time frame.

The AI assistants

Q What's the difference between the Assistant and the Copilot?

CareMetric Assistant is the customer-facing storefront chatbot; CareMetric Copilot is the staff-facing helper inside the admin console. (The Penn tenant calls them PennBot and PennPilot.)

Q Will the Copilot change my data?

No — it explains the app and, only after you confirm, forwards a feature idea to the owners. It never changes data and never shows patient PHI.

Q What happens if the AI vendor is down?

Both assistants degrade gracefully — Claude first, then OpenAI, then a safe offline reply — so the app never breaks because a key is missing.

Appendix

Role & permission matrix

What each role can reach.

● full ◐ partial / Owner-gated ○ none

Area	Admin	Biller	CSR	RT
Home & Workspace	●	◐	●	◐
Conversations / inbox & cases	●	●	●	○
Patients (read / edit)	●	◐	●	◐
Orders, shop & returns	●	○	●	○
Billing & revenue cycle	●	●	○	○
Therapy monitoring & clinical notes	●	○	◐	●
Analytics & financial reports	●	◐	◐	○
Owner overview & owner models	●	○	○	○
Item pricing evaluation / proposals	●	○	●	○
Cost verification, approval & publishing	●	○	○	○
Team management	◐	○	○	○
System configuration & secrets	◐	○	○	○

Administrator combines the Owner and Admin tiers — Owner-only items (Team management, System Configuration) are marked partial because only the Owner role can use them. **Biller** reaches the whole Billing area plus patient billing context and the shared inbox (revenue-cycle staff work patient-balance and benefit threads), but not clinical notes or settings. **Owner and Admin** have pricing evaluation, management, approval and publication permissions. Owner models require pricing management; Owner overview requires both management metrics and cost reporting. **CSR** pricing evaluation exposes relevant item economics and proposals, not authoritative cost edits, actual-event writes, owner models or price publication. **Biller** cost/reporting access does not grant pricing management or Owner overview. **RT** has clinical reports, not financial pricing access. Calendar reads need patient access; resupply outreach also needs conversation-management access. Available navigation also depends on enabled practice modules.

How it compares

How CareMetric Breathe's (CMB) marquee features line up against the DME/HME platforms a resupply business is most likely to evaluate. A full circle = native, half = partial / add-on / via partner, open = not offered or not core.

CMB = CareMetric Breathe · Brightree = Brightree with its ReSupply module · Niko = NikoHealth · TIMS = TIMS Software.

Patient experience

Feature	CMB	Brightree	Niko	TIMS
AI camera-based mask fitting, in-browser, privacy-first	●	○	○	○
Patient portal: what's due, tracking, statements, e-sign	●	◐	◐	◐
Automated resupply outreach, one-tap confirm (SMS/email)	●	●	◐	◐
Conversational AI voice agent for reorders/check-ins	●	◐	○	○
Patient AI chatbot and sleep coach	●	○	○	○

Clinical & therapy

Feature	CMB	Brightree	Niko	TIMS
Therapy-cloud sync (ResMed, Philips, React Health)	●	●	◐	◐
CMS 90-day setup-adherence tracking	●	●	◐	◐
Clinical intervention, coaching, mask-fit worklists	●	◐	○	○
Inbound fax OCR and document triage	●	●	◐	◐

Revenue cycle

Feature	CMB	Brightree	Niko	TIMS
Clearinghouse claims (837P/835) + real-time eligibility	●	●	●	●
AI claim scrubbing + denial recovery by win-probability	●	◐	◐	○
Electronic prior authorization (Da Vinci PAS)	●	◐	○	○
DME A/R: capped rentals, secondary/COB, timely filing	●	●	●	●
Patient statements, payment plans, payment links	●	●	●	●

Operations & intelligence

Feature	CMB	Brightree	Niko	TIMS
Unified omnichannel inbox (SMS, MMS, email, chat)	●	◐	◐	○
Provider e-signature and e-ordering collaboration	●	◐	◐	○
Analytics: margin, LTV/CAC, payer profitability	●	◐	◐	◐
In-app AI staff assistant + no-code automation rules	●	◐	◐	○

CareMetric Breathe entries reflect the shipped platform described in this manual. Competitor entries summarize publicly available product information and may be delivered via partners or paid add-ons; verify with each vendor before relying on this comparison.

Glossary

270 / 271	The electronic eligibility request (270) and the payer's response (271) that confirm a patient's coverage.
837P	The electronic professional claim file sent to a payer (via the clearinghouse) to request payment.
835 / ERA	The Electronic Remittance Advice — the payer's machine-readable explanation of what it paid, used to auto-post payments.
999 / 277CA	Acknowledgements from the clearinghouse/payer confirming a claim file was received and accepted (or rejected).
AHI	Apnea-Hypopnea Index — events per hour of sleep; a core measure of how well therapy is controlling apnea.
Capped rental	A Medicare payment model where a device is rented over a capped number of months (13 or 36) rather than purchased outright.
CMN / DIF	Certificate of Medical Necessity / DME Information Form — documentation a payer requires to justify the equipment.
COGS	Cost of Goods Sold — the product cost used to compute gross margin.

DSO	Days Sales Outstanding — the average number of days it takes to collect after billing.
HCPCS	The procedure/supply codes used on claims (e.g. the codes for a mask, machine, or filter).
LTV : CAC	Lifetime Value to Customer-Acquisition-Cost ratio — a measure of marketing efficiency.
MFA	Multi-Factor Authentication — a second login factor (authenticator-app code) on top of your password.
NPS	Net Promoter Score — a post-delivery satisfaction measure from patient survey responses.
Prior auth (PAS)	Prior Authorization — payer approval secured before dispensing; Da Vinci PAS is the electronic FHIR standard for it.
Resupply	The recurring replacement of CPAP consumables (masks, cushions, filters, tubing) on a cadence.
Allowed amount	The payer-contract allowance, distinct from the billed charge; relevant payer and patient shares are portions of it, not additions to it.
Expected collectible	The supported total the practice expects to collect. It is an assumption until receipts are recorded and reconciled.
Delivered / landed cost	Goods plus applicable acquisition, dropship, freight, packing and other fulfillment costs, with inclusions counted only once.
Purchase pack	The supplier's purchasing unit. New-item comparisons calculate whole-pack outlay and surplus. Firm reviews require whole allowed packs and use the recorded cost per canonical selling unit.
Contribution	Net revenue less modeled or recorded variable costs. It is before separately allocated fixed overhead and is not company net income.
Margin versus markup	Margin divides contribution by revenue; markup divides it by costs. Equal percentages do not produce equal prices.
Hard floor	A required pricing boundary. A routine exception approval cannot override a failed floor or other hard rule.
Pricing context	The complete item/quantity, revenue and delivery assumptions to which a published price applies, including a full bundle where selected.
Frozen preview	A saved price-list proposal whose selected changes, retained prices, evidence and comparisons cannot be changed by later filter edits.

Bound review	An approved pricing snapshot committed to a specific patient order; later cost changes do not rewrite its accepted amounts.
Economic occurrence date	When the invoice, receipt, refund or credit happened. Recorded as UTC and used for financial activity periods, independently of entry time.
Economic event reference	A stable identifier used to recognize the same expense or receipt on retry and prevent duplicate financial entries.
Completed-review contribution	Lifetime recorded net revenue minus net costs for bound reviews with costs and collections complete and reliable event dates.
Created cohort	Records created in a selected period, shown with their current status or paid-to-date values. Later changes can affect an earlier cohort.
Current snapshot	Work queues or balances as recorded now, including older records; it is not a reconstructed historical balance.
Break-even orders	Whole orders needed to cover entered fixed costs at the assumed contribution per order; a positive requirement needs positive contribution.
Acquisition payback	Orders needed to recover entered acquisition cost using contribution after retention cost; it does not allocate all company expenses.
Working-capital estimate	An explicit cash-outlay projection over inventory, collection and vendor-payment timing; not a forecast of every cash flow.
Queued / delivered	Queued means accepted for processing. Delivery needs recorded provider evidence; neither status is a patient order confirmation.
UTC	The time standard used for owner-report periods and financial occurrence dates. A scheduling field labeled local time follows that separate label.

Need more? Ask CareMetric Copilot in the app, or reach your practice administrator. Owners can find deployment and operator runbooks in the project documentation.